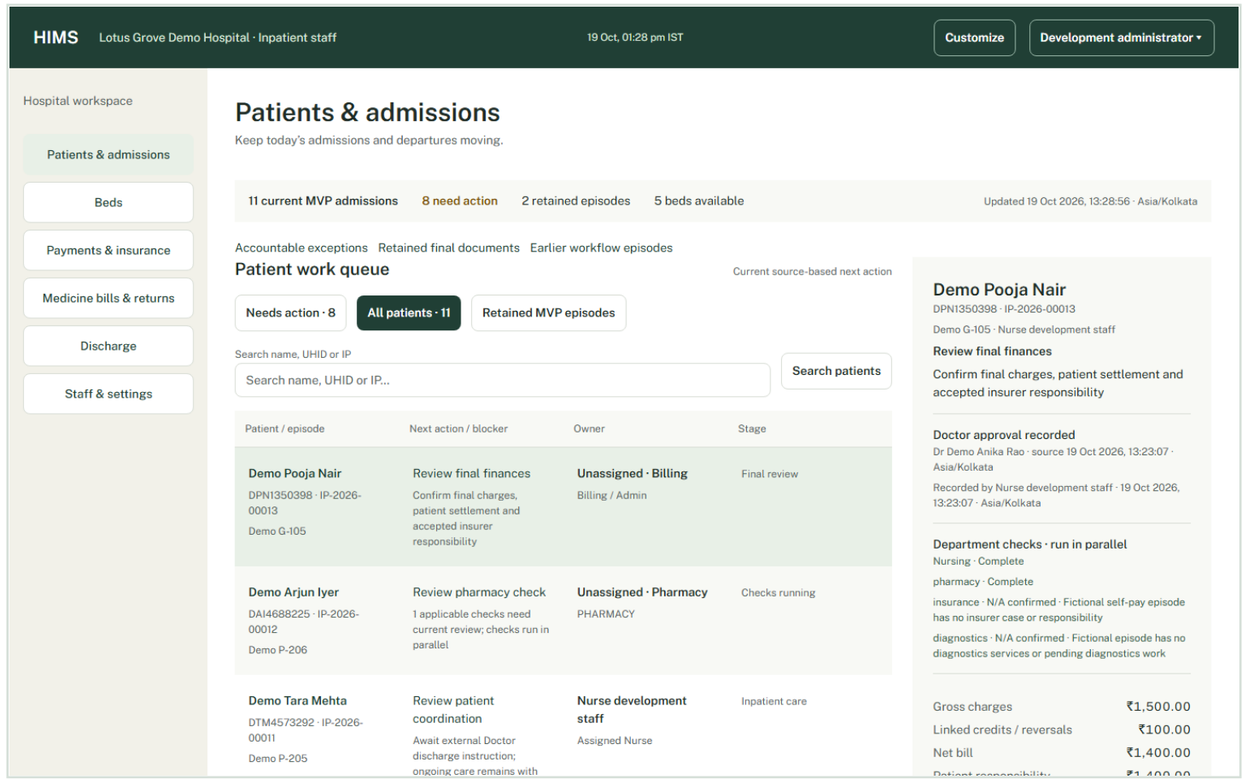
Patient admission to discharge

Illustrated staff guide | Lotus Grove Demo Hospital | 19 October 2026

1. Start at http://localhost:3000/login. Use your existing staff account; passwords are intentionally omitted from this guide. Sign out through Profile before switching roles, or use separate browser profiles.
2. Part 1 follows Demo Rohan Desai, age 45, male, insured, IP-2026-00008 / UHID DRD8354056. His completed journey shows who acts, which exact control to use and what happens next.
3. Part 2 explains every sidebar section across Admission, Admin, Nurse, Billing, Pharmacy and Insurance, with the important tabs/forms inside them.
4. Screenshots are real local application pages. Journey images show stages already completed for Rohan; current reference screens also use Ananya, Kabir, Tara, Arjun and Pooja. Read the patient name before using a form.
5. This is a fictional training hospital. No patient transactions, assignments or configuration changes were submitted to create this document. The source-backed optional forms are marked as inspected rather than completed.



Current section / form reference | upper workspace; scroll for more

How the patient moves between desks

Follow responsibility, not the sidebar order

1. 1. Admission: find/register identity > reserve a bed > record actual deposit > record actual arrival. Admin assigns Nurse responsibility; Nurse begins the assigned care shift.
2. 2. During care: Pharmacy records medicine charges/accepted returns; Billing records performed services and money. Insurance verifies policy and external payer information in parallel.
3. 3. Doctor decides externally. Nurse records the named instruction and physical checklist. Applicable Nursing, Pharmacy, Diagnostics and Insurance checks run in parallel; Admin confirms permitted optional N/A with a reason.
4. 4. Nurse records physical bed vacancy and waiting-care handover. Final room use is fixed, but the bed stays held. Billing/Admin reviews patient settlement, refunds and accepted insurer responsibility.
5. 5. Authorized staff generate/deliver the exact papers. Assigned Nurse confirms actual hospital exit. Admin records authorized cleaning/turnover before the bed becomes Ready.
6. 6. An accepted insurer receivable can remain at ordinary exit. Later actual insurer cash settles the receivable without changing the original exit or its final PDF.

Note: For self-pay, an insurer receipt is unnecessary. Optional checks require appropriate N/A confirmation; Doctor approval, Nursing readiness/exit and final financial authority remain mandatory.

1 ADMISSION

Identity, deposit, actual arrival

2 CARE TEAMS

Nurse, Pharmacy, Billing, Insurance

3 EXTERNAL DOCTOR

Instruction recorded by assigned Nurse

4 PARALLEL CHECKS

Nursing, Pharmacy, Diagnostics, Insurance

5 SETTLEMENT & PAPERS

Vacancy, final finance, exact paper delivery

6 ACTUAL EXIT & TURNOVER

Nurse exit, then Admin bed release

Find the page you need

Clickable contents and PDF bookmarks

Patient journey 4

Admission dashboard 26

Admin dashboard 32

Nurse dashboard 43

Billing dashboard 48

Pharmacy dashboard 54

Insurance dashboard 57

Practice patients 63

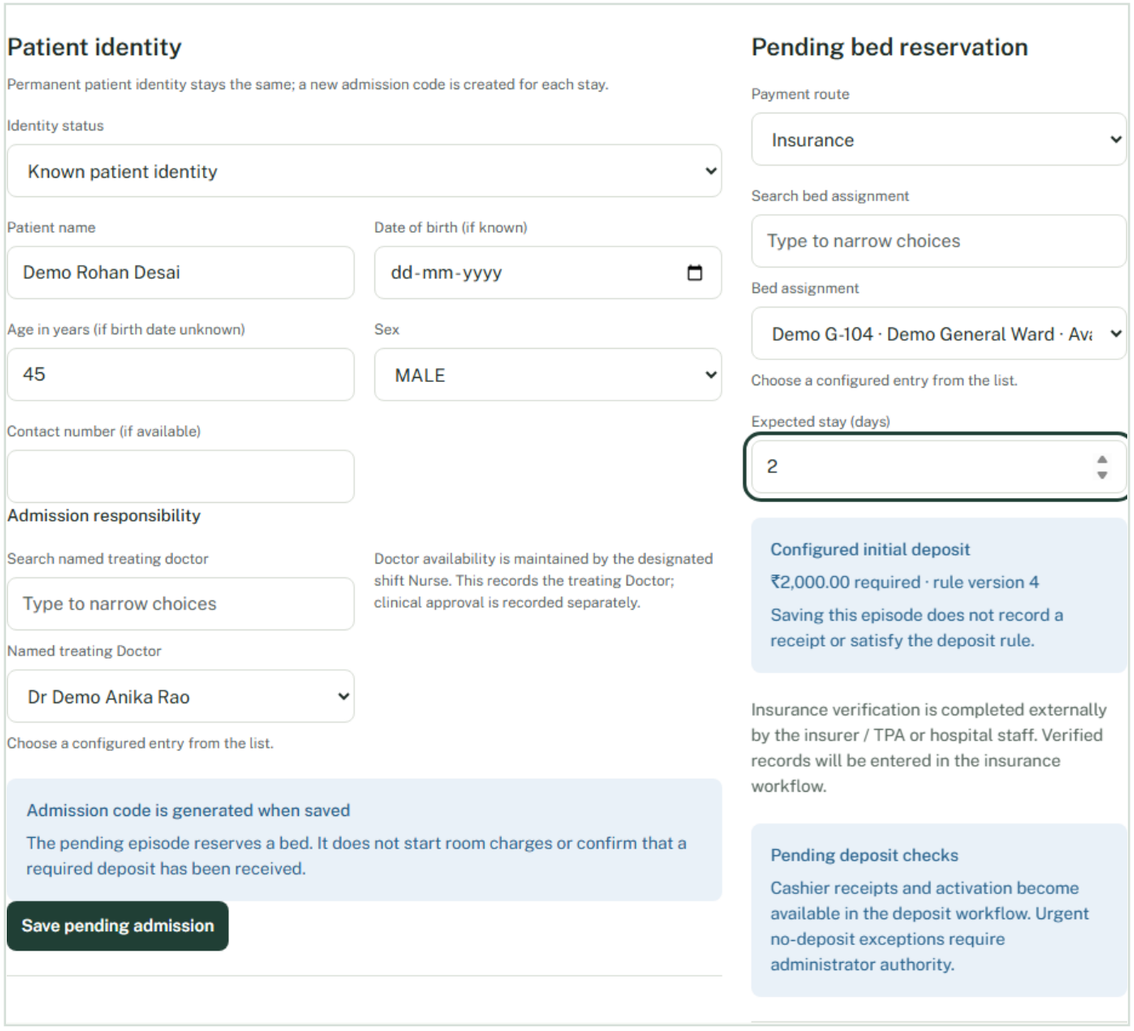
Limits and source notes 64

Exact button names are bold in the instructions where captured. A triangle beside a heading opens a form; it does not save anything. A save/record button submits the form. Always check the patient name, role and current state first.

01 - Register Rohan and reserve his bed

Admissions > Patient identity & intake · /admissions?view=intake

1. Fictional walkthrough: Demo Rohan Desai, age 45, MALE, Insurance, Dr Demo Anika Rao, Demo G-104 and expected stay 2 days.
2. Search for an existing patient before creating an identity. Existing patients keep their UHID; each stay receives a separate IP code.
3. Click **Save pending admission**, then Open admission record. Rohan receives UHID DRD8354056 and IP-2026-00008.
4. Result: the episode is PENDING and the bed is reserved. Saving intake neither receives a deposit nor begins occupied-room charges.

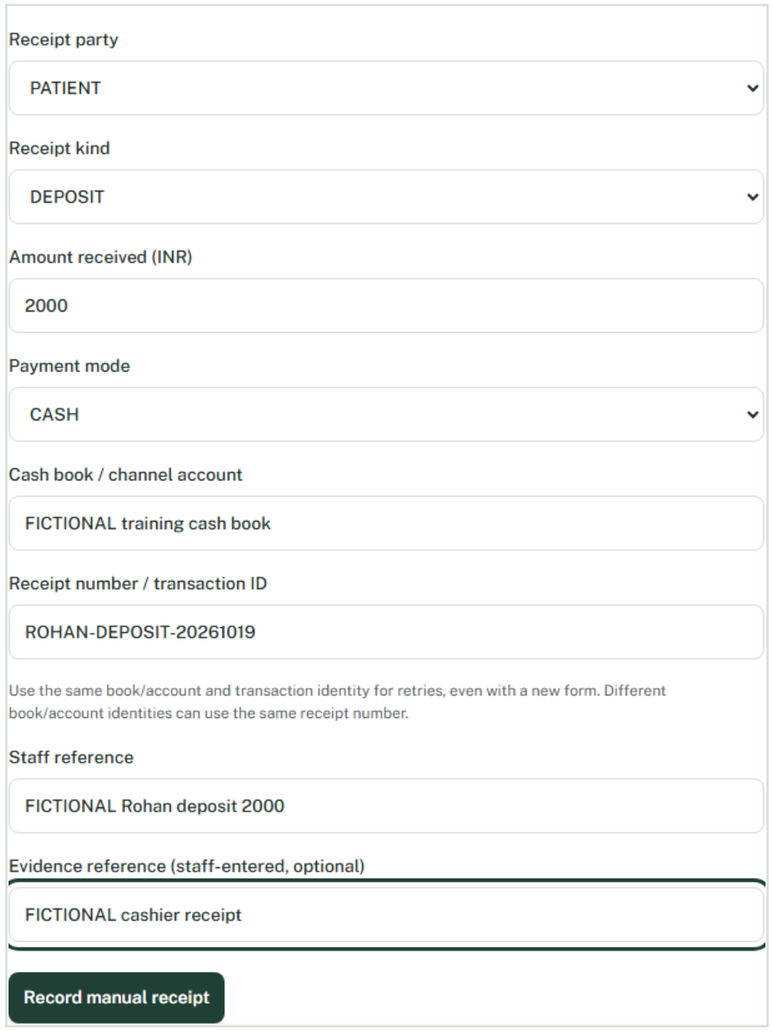


Journey capture - Rohan

02 - Record the deposit actually received

Rohan's admission record > Patient deposit

1. Select **Patient deposit**. Enter Amount 2000, the actual hospital channel, transaction/book reference, receipt record reference and evidence.
2. This exercise uses FICTIONAL Rohan deposit 2000 and reference ROHAN-DEPOSIT-20261019. No real transfer occurred.
3. Click **Record manual receipt**. The recorded ₹2,000 becomes held patient money; recording it does not itself allocate it against a bill.
4. The episode remains pending until actual arrival is recorded. Do not enter the same original deposit again in Billing.

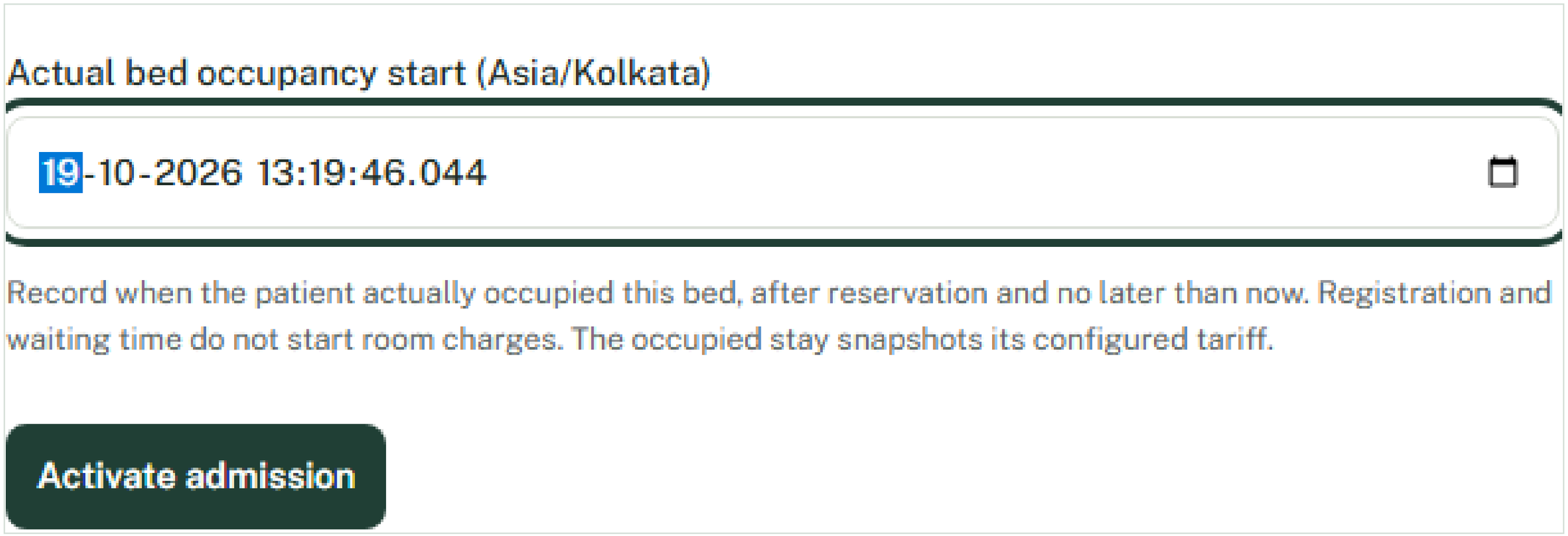


Journey capture - Rohan

03 - Activate actual arrival

Rohan's admission record > Actual arrival

1. After checking the configured deposit requirement and the patient's real arrival, select **Actual arrival**.
2. Enter the actual arrival time and click **Activate admission**. A reservation time must not be substituted for physical arrival.
3. Result: Rohan becomes ADMITTED, Demo G-104 is occupied and chargeable room use begins at the recorded boundary.
4. Registration, receipt and activation are separate facts. The same arrival checks apply to insured and self-pay admissions.

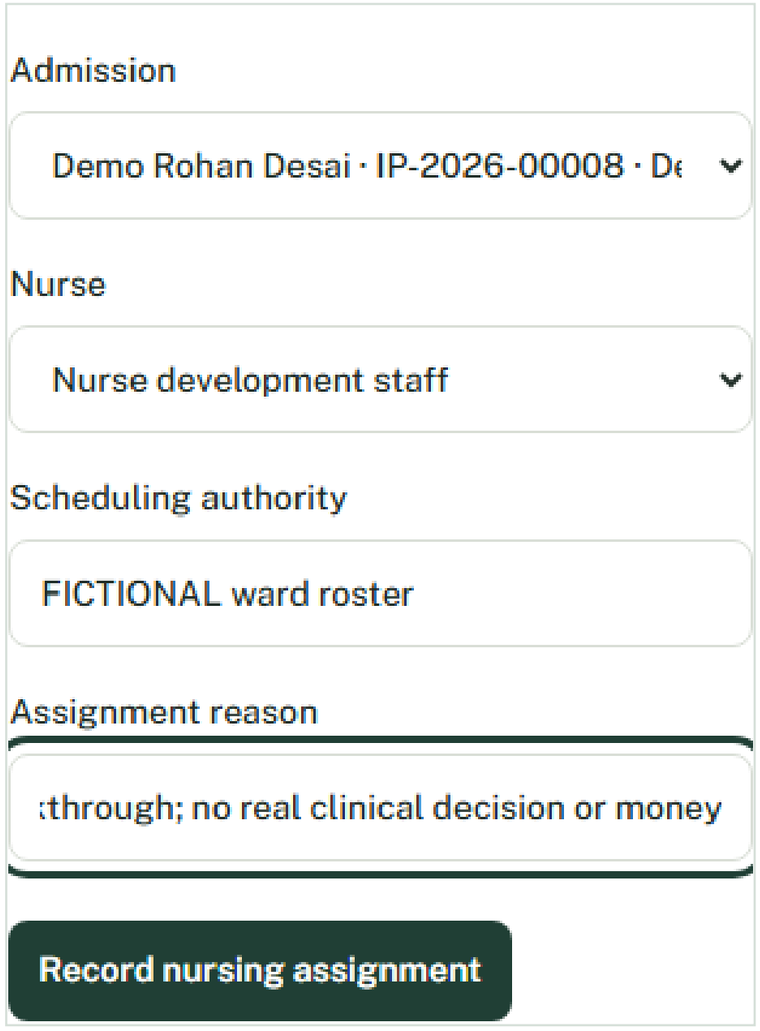


Journey capture - Rohan

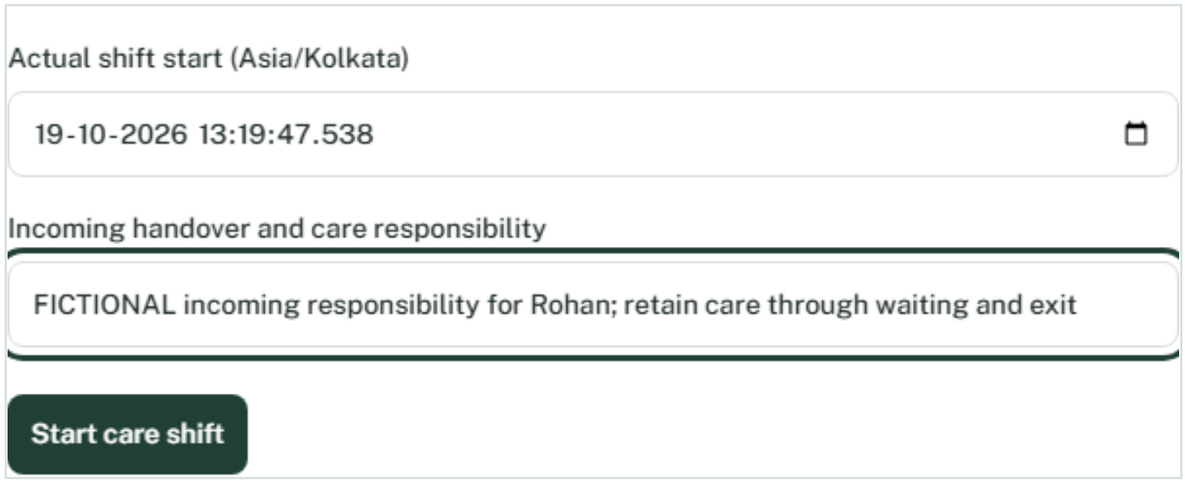
04 - Assign responsibility and begin the care shift

Admin > Staff & settings > nursing responsibility · /admin/care; Nurse > Rohan > Readiness & checks

1. Admin selects Rohan and Nurse development staff, enters roster authority and assignment reason, then clicks **Record nursing assignment**.
2. The assigned Nurse opens Rohan, expands Start assigned care shift, and enters Actual shift start plus Incoming handover and care responsibility.
3. Click **Start care shift**. The system records this Nurse's actual attendance and care responsibility separately from the assignment.
4. Only an assigned Nurse can perform the patient's Nursing work. Creating a new staff account alone does not assign that person.



Journey capture - Rohan

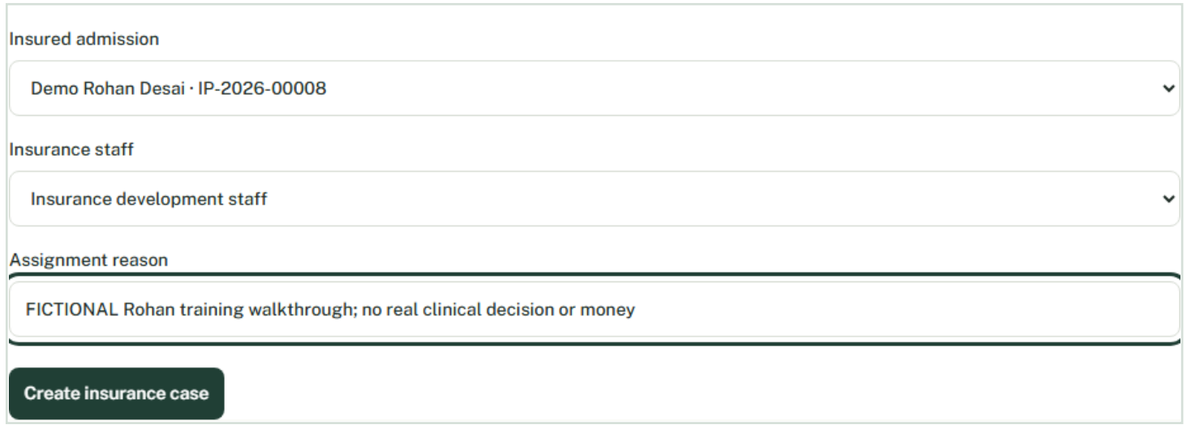


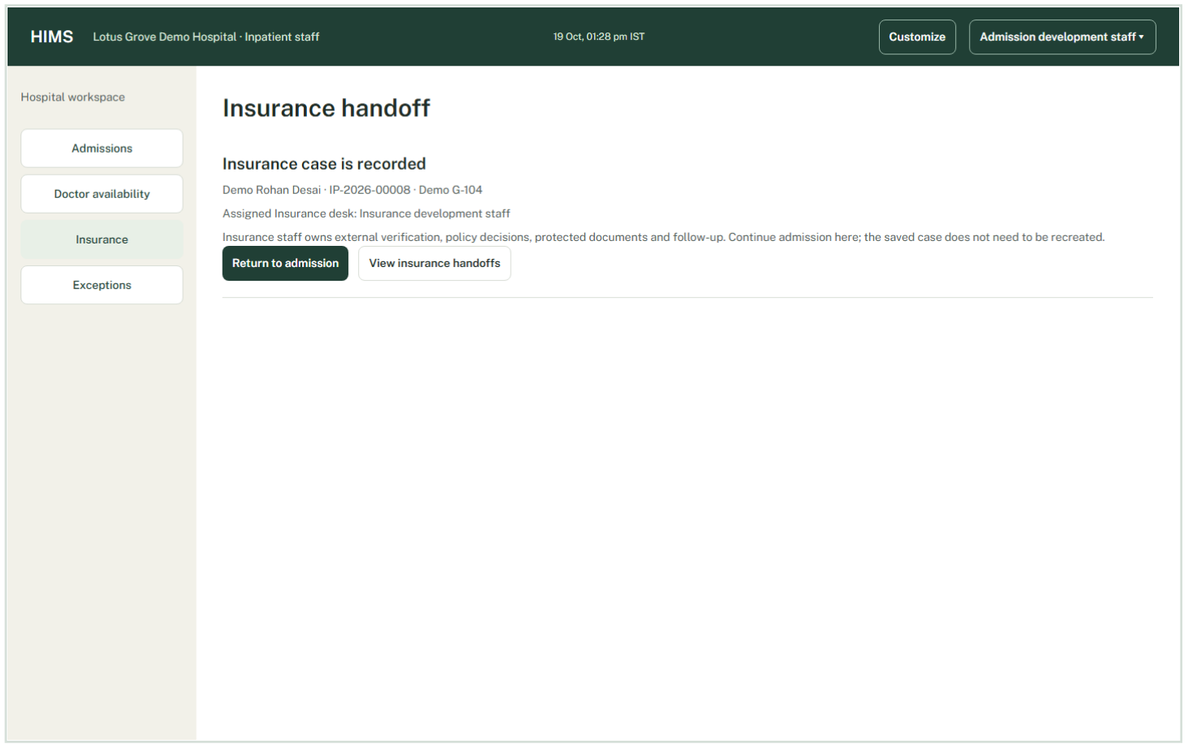
Journey capture - Rohan

05 - Create the Insurance handoff

Admission > Insurance > Create and assign an insured case

1. Select Rohan's insured admission and Insurance development staff. Enter Assignment reason, then click Create insurance case.
2. Result: Insurance case is recorded shows the patient, IP code, bed and assigned desk. Continue through Return to admission or View insurance handoffs.
3. The assigned Insurance user opens the exact case from Cases. Policy verification, decisions, protected files and follow-up belong to that desk.
4. Creating the case neither verifies coverage nor contacts an insurer. Do not recreate an already saved case.





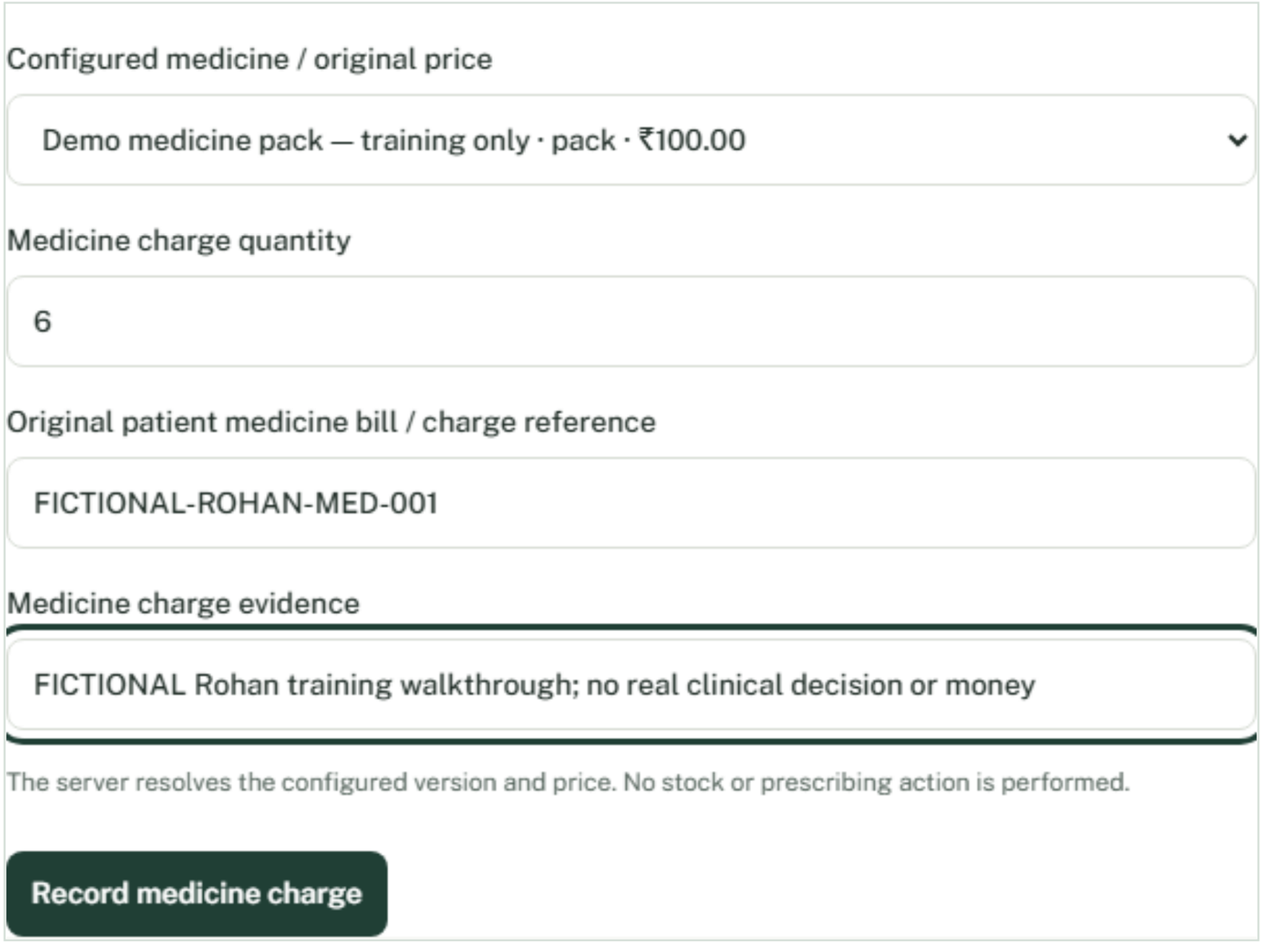
Journey capture - Rohan

Current section / form reference | upper workspace; scroll for more

06 - Record the original patient medicine bill

Medicine bills & returns > Rohan > Medicine bills & returns

1. Expand Record patient medicine charge. Select Demo medicine pack - training only at its configured ₹100 per pack.
2. Enter Medicine charge quantity 6, original source FICTIONAL-ROHAN-MED-001 and Medicine charge evidence.
3. Click **Record medicine charge**. Result: the original charge is six packs × ₹100 = ₹600, linked to this admission.
4. The server uses the configured price version. This records a patient charge; prescribing and stock inventory are outside this desk's MVP scope.

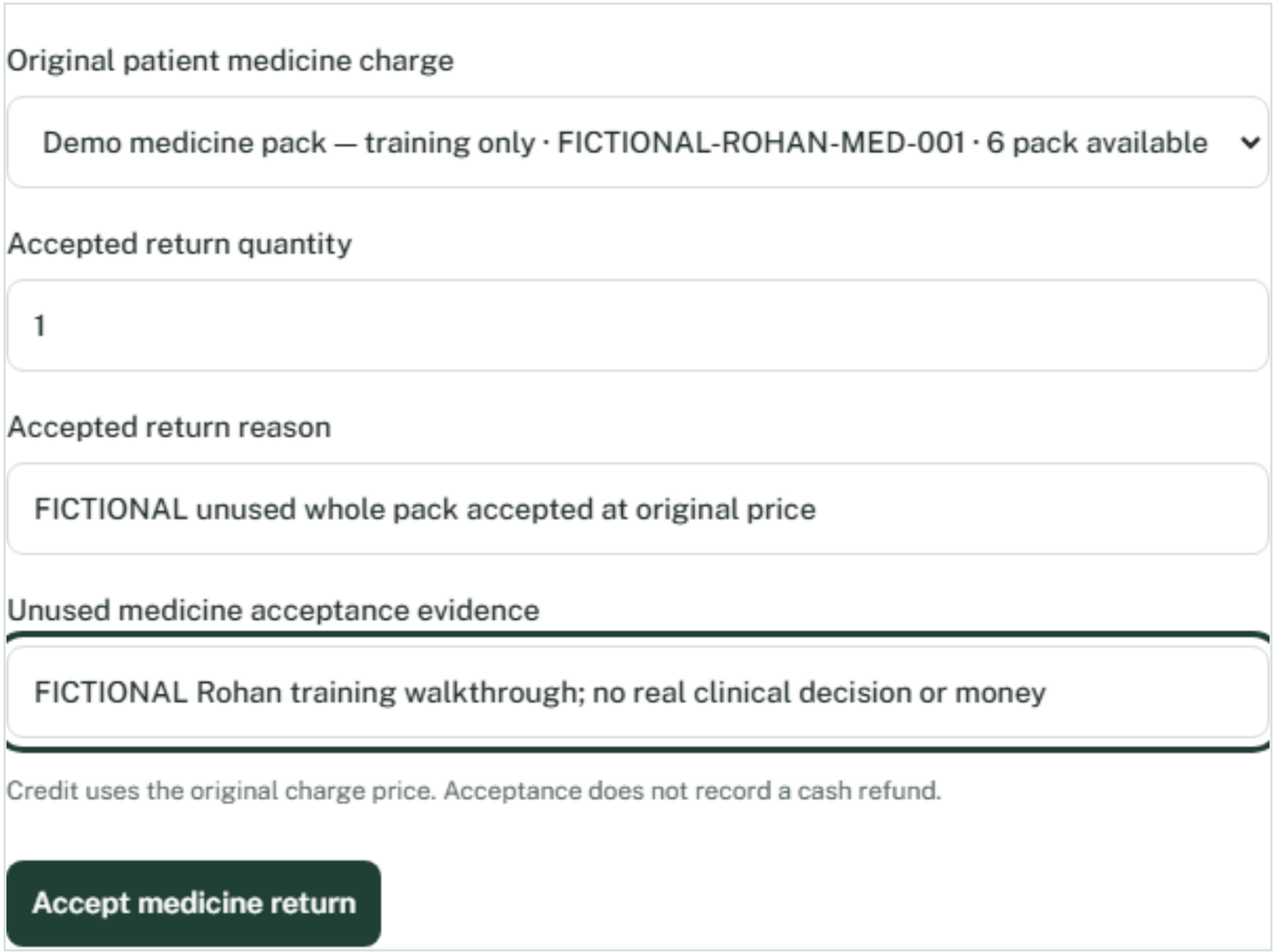


Journey capture - Rohan

07 - Accept an unused return against the original charge

Rohan > Medicine bills & returns > Accept unused medicine return

1. Choose the original FICTIONAL-ROHAN-MED-001 charge. Enter Accepted return quantity 1, Accepted return reason and Unused medicine acceptance evidence.
2. Click **Accept medicine return**. The credit uses the original ₹100 unit price: ₹600 − ₹100 = ₹500 net medicine charge.
3. Medicine charge / return history retains both sources, original quantity/price, accepted returns and remaining returnable quantity.
4. An accepted return creates bill credit. Billing separately records any actual cash refund through the existing hospital channel.

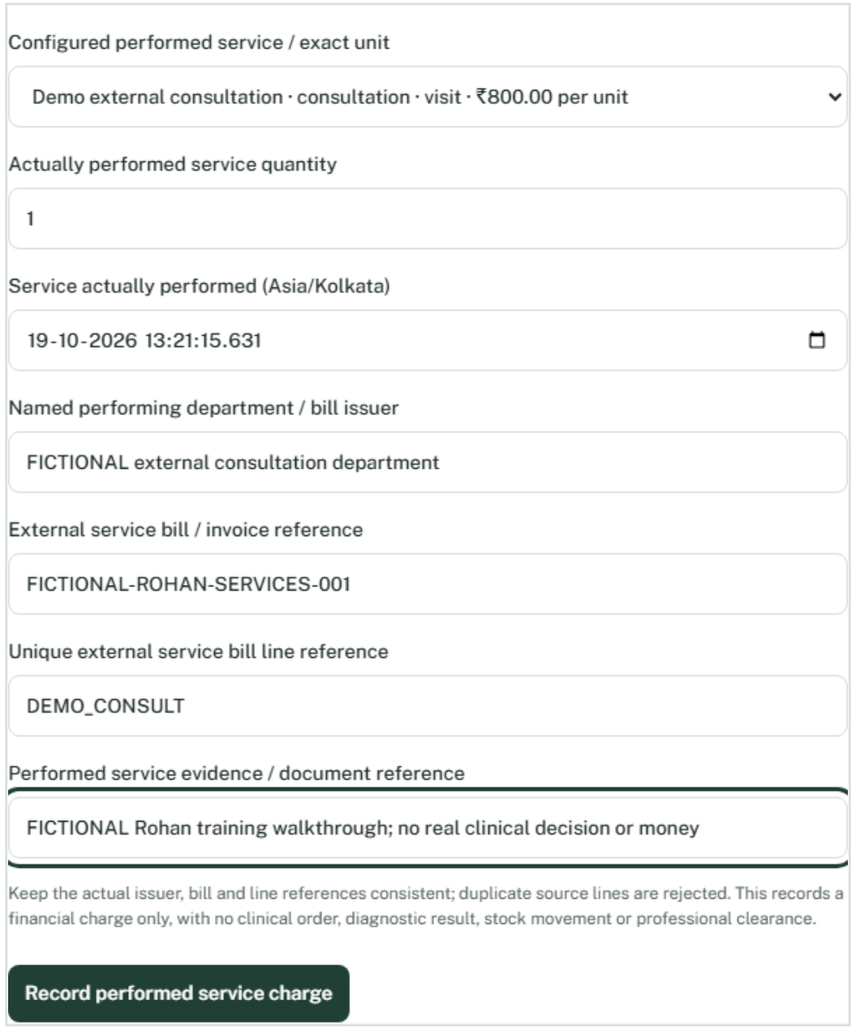


Journey capture - Rohan

08 - Record externally performed consultation and blood count

Rohan > Money & insurance > Record a performed service charge

1. Select Demo external consultation, quantity 1; enter the actual service time, named issuer, external bill reference, unique line DEMO_CONSULT and evidence.
2. Click **Record performed service charge**. Repeat for Demo blood count, quantity 1, with its own unique line DEMO_CBC.
3. The configured prices add consultation ₹800 and blood count ₹400. Duplicate external bill lines are rejected.
4. These are financial records of work performed outside the app. Recording a charge does not place a clinical order, produce a result or supply diagnostic clearance.

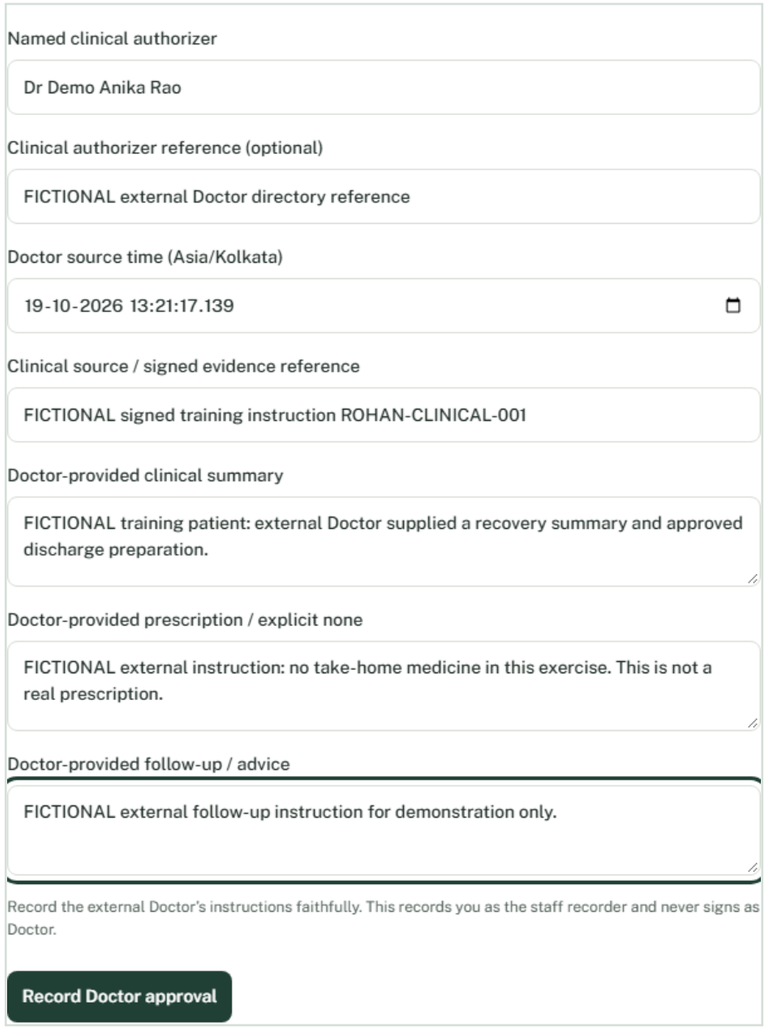


Journey capture - Rohan

09 - Record the external Doctor's discharge instruction

Rohan > Readiness & checks > Record external Doctor approval

1. Enter Named clinical authorizer Dr Demo Anika Rao, optional authorizer reference, Doctor source time and Clinical source / signed evidence reference.
2. Copy the Doctor-provided clinical summary, prescription or explicit none, and follow-up/advice faithfully from the external instruction.
3. Click **Record Doctor approval**. The app retains the named external authority and the separate Nurse recorder and time.
4. There is no routine Doctor dashboard. Nurse records the instruction; the Nurse does not sign as Doctor or invent clinical advice.

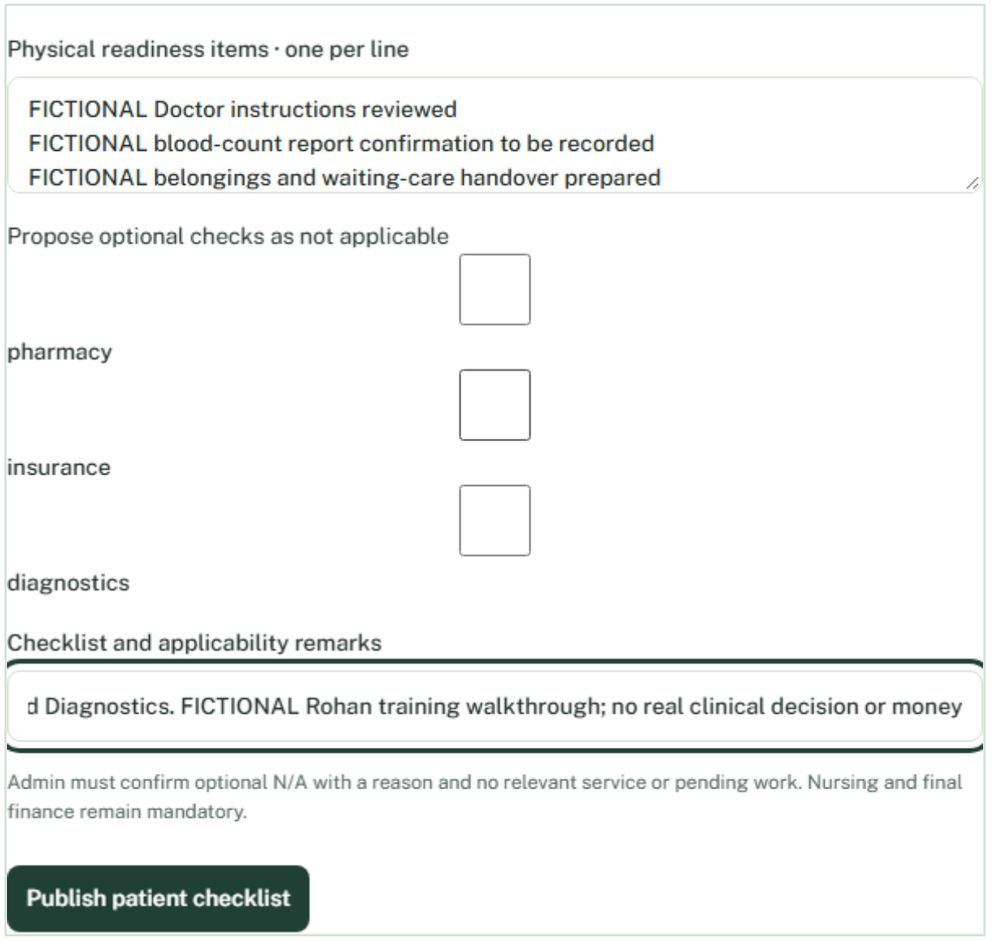


Journey capture - Rohan

10 - Prepare clinical papers and publish the checklist

Rohan > Vacancy, papers & exit; then Readiness & checks

1. After Doctor approval, expand Generate an authorized paper version, choose Clinical papers · after Doctor approval and click Generate authorized paper. Review the saved CLINICAL version.
2. In Readiness & checks, expand Publish physical patient checklist. Enter Physical readiness items · one per line and Checklist and applicability remarks.
3. Click **Publish patient checklist**. In Rohan's example all four checks apply: Nursing, Pharmacy, Insurance and Diagnostics; these checks can proceed in parallel.
4. For genuinely optional work, Nurse may propose N/A. Admin must Confirm proposed optional N/A with a reason and no relevant work. Nursing and final finance remain mandatory.

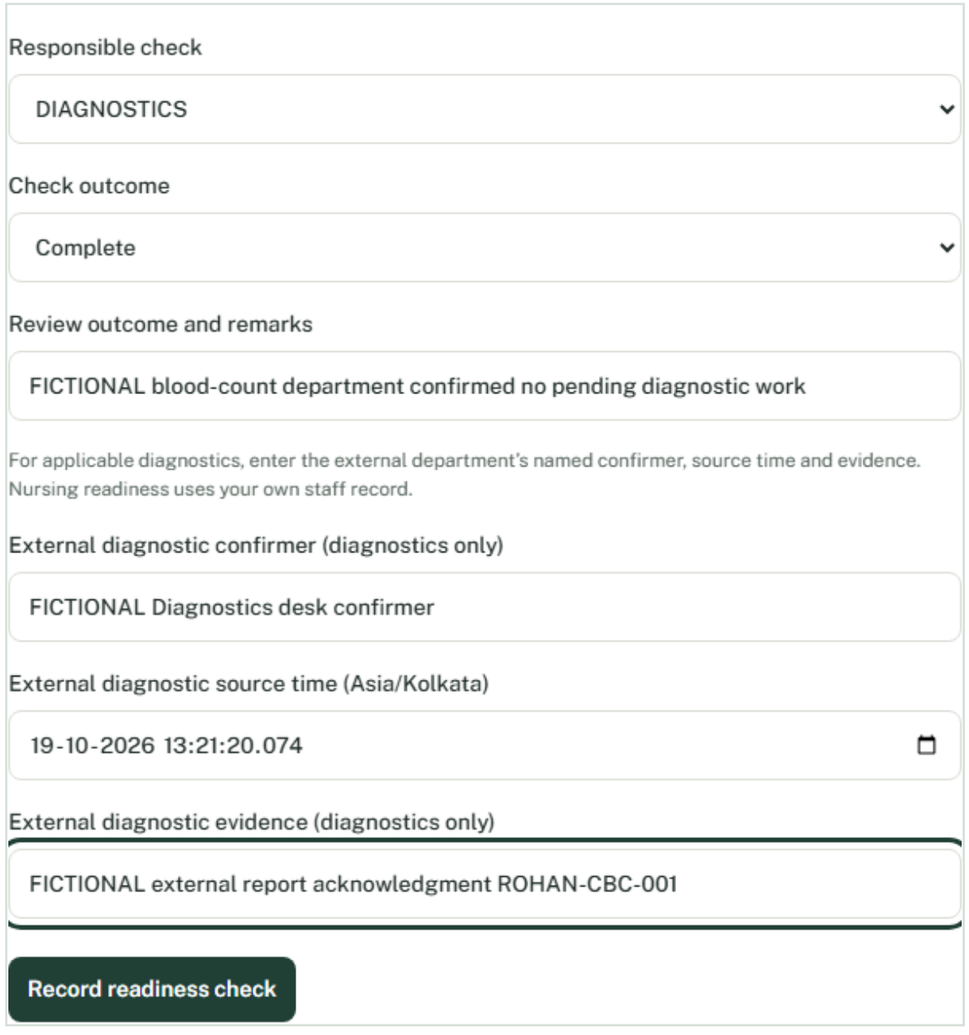


Journey capture - Rohan

11 - Complete Nursing and named external Diagnostics checks

Rohan > Readiness & checks > Record your readiness check

1. For Nursing, choose Responsible check NURSING, Check outcome Complete and enter Review outcome and remarks. Click **Record readiness check**.
2. Repeat for DIAGNOSTICS after actual external confirmation. Enter the named External diagnostic confirmer, source time and evidence as well as remarks.
3. Click **Record readiness check** for that separate check. Rohan's fictional evidence confirms no pending blood-count work.
4. Nursing confirmation does not supply Pharmacy, Insurance or financial authority. Pending / further work remains available when the work is unfinished.

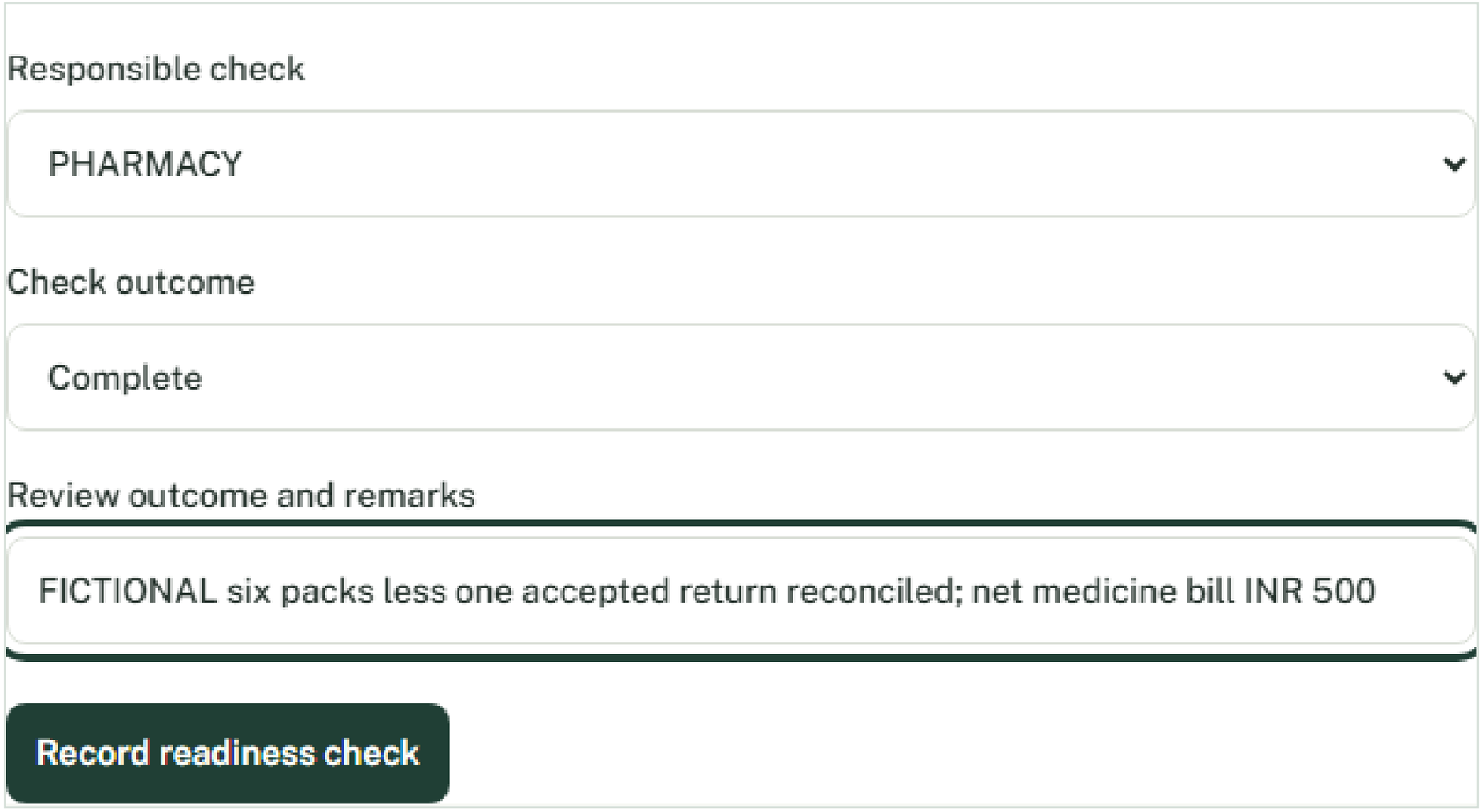


Journey capture - Rohan

12 - Pharmacy confirms its own reconciliation

Pharmacy > Discharge > Rohan > Readiness & checks

1. Open the patient's Complete Pharmacy readiness check task and expand Record your readiness check.
2. Choose Responsible check PHARMACY and Check outcome Complete. Enter the reconciliation: six packs charged, one accepted return, ₹500 net medicine bill.
3. Click **Record readiness check**. Result: Pharmacy's current check is recorded against the patient's sources.
4. A later material source change can require affected checks again. Pharmacy reconciliation remains separate from final finance and authorized paper delivery.

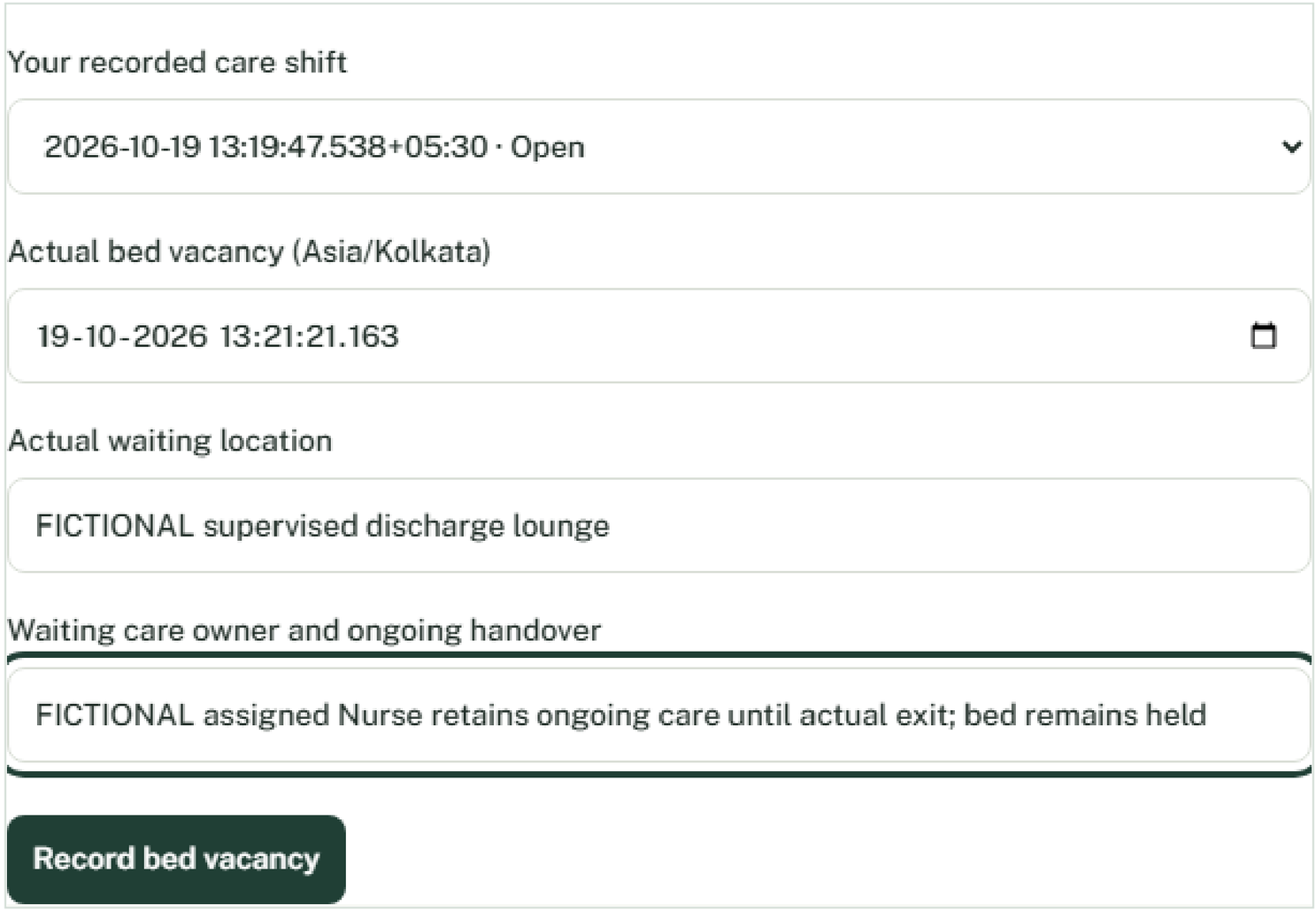


Journey capture - Rohan

13 - Record physical vacancy and continuing waiting care

Rohan > Vacancy, papers & exit > Record actual bed vacancy

1. Enter Actual bed vacancy, Actual waiting location and Waiting care owner and ongoing handover under the responsible care shift.
2. Rohan moves to the FICTIONAL supervised discharge lounge; the assigned Nurse retains care until actual exit. Click Record bed vacancy.
3. Result: occupied room use ends and final room charges are posted. The screen shows Bed vacant · held until exit & turnover.
4. Physical vacancy does not mean hospital exit or a reusable bed. The bed stays held while the patient waits and until exit and cleaning are recorded.

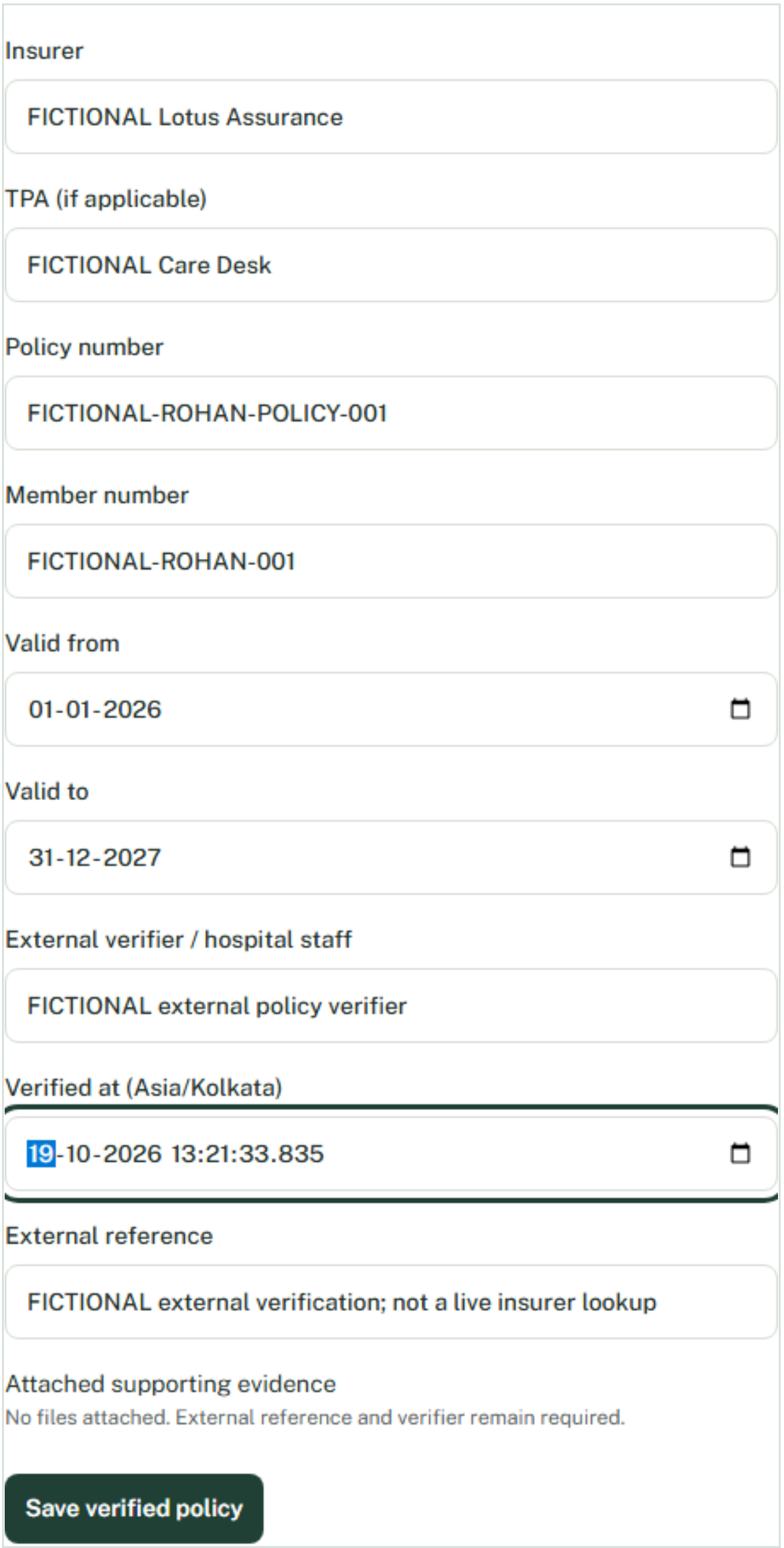


Journey capture - Rohan

14 - Save externally verified policy information

Cases > Rohan's exact case > Decision > Record externally verified policy

1. Enter Insurer, optional TPA, Policy number, Member number, Valid from and Valid to. Rohan uses FICTIONAL Lotus Assurance and FICTIONAL Care Desk.
2. Record External verifier / hospital staff, Verified at and External reference. Select relevant Attached supporting evidence when available.
3. Click **Save verified policy**. Prior versions remain retained; the app records what staff verified outside this software.
4. Documents contains protected evidence attachments. Policy entry is not a live insurer lookup, coverage approval or receipt of money.

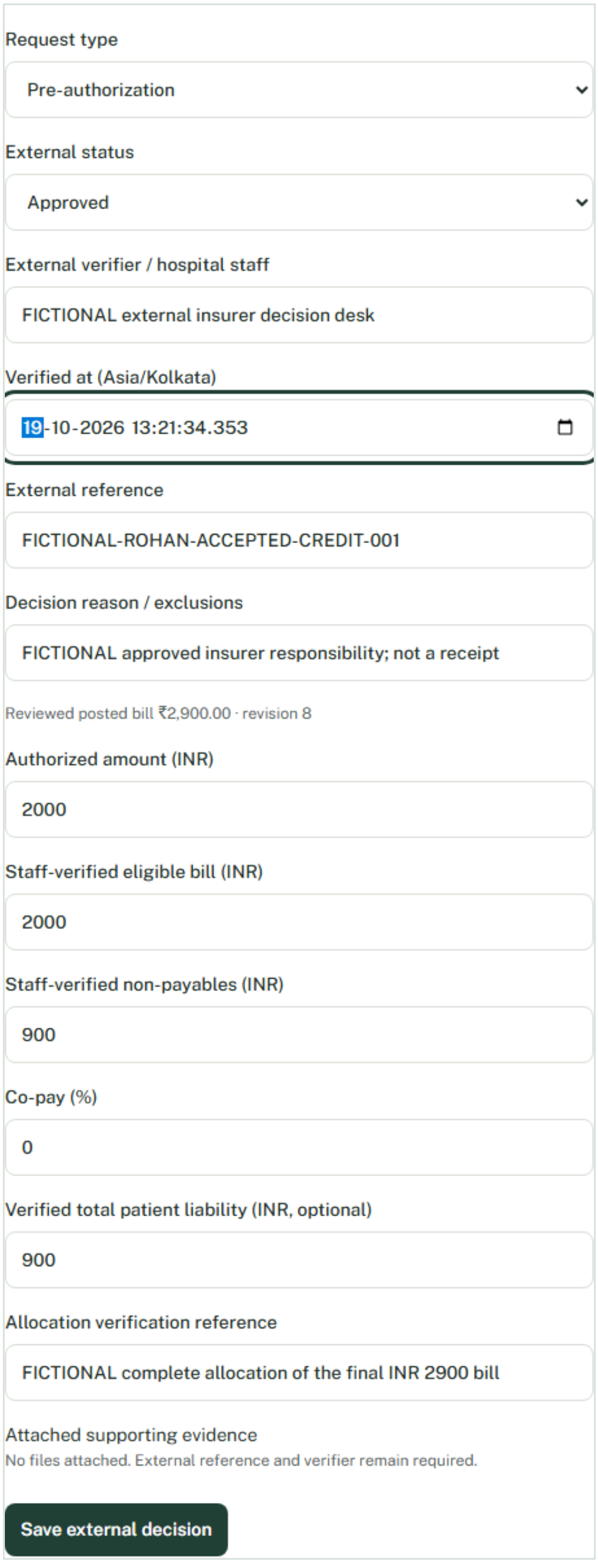


Journey capture - Rohan

15 - Record the payer allocation and Insurance readiness

Rohan's case > Decision; then Review Insurance discharge check

1. In **Record payer decision and allocation**, record the real request type, Approved status, external verifier/time/reference and Decision reason / exclusions.
2. Rohan's final bill is ₹1,200 room + ₹800 consultation + ₹400 blood count + ₹600 medicines − ₹100 return = ₹2,900.
3. Enter Authorized amount 2000, eligible bill 2000, non-payables 900, co-pay 0, total patient liability 900 and allocation reference. Click **Save external decision**.
4. Use Review Insurance discharge check > Record your readiness check; choose INSURANCE, Complete and remarks, then **Record readiness check**. Accepted ₹2,000 remains receivable until actual receipt.

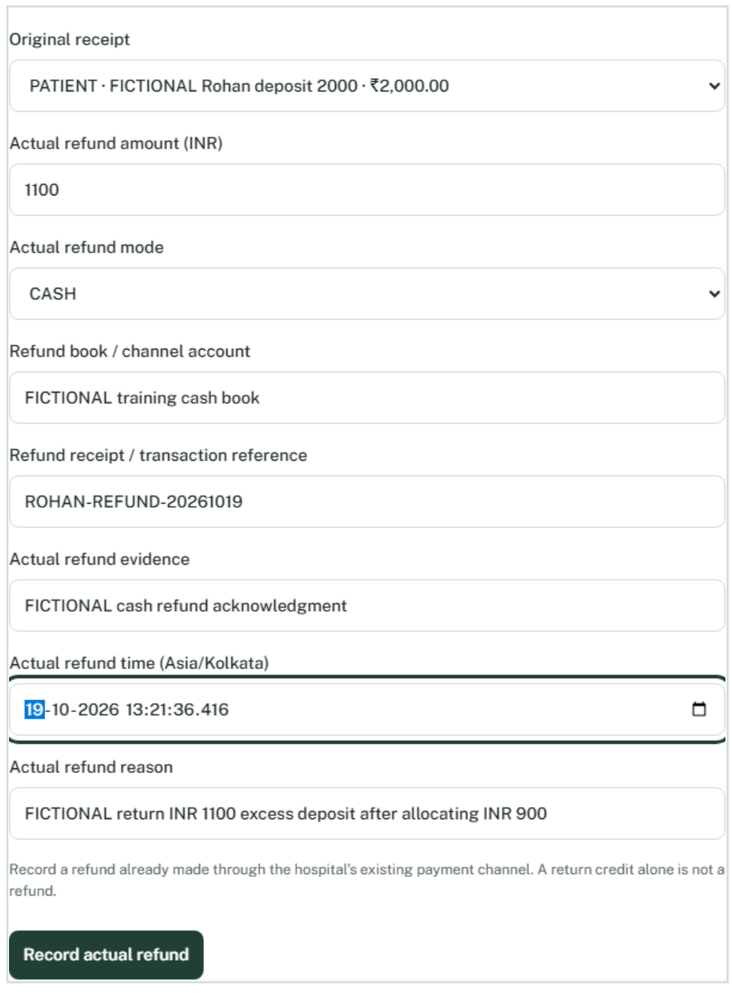


Journey capture - Rohan

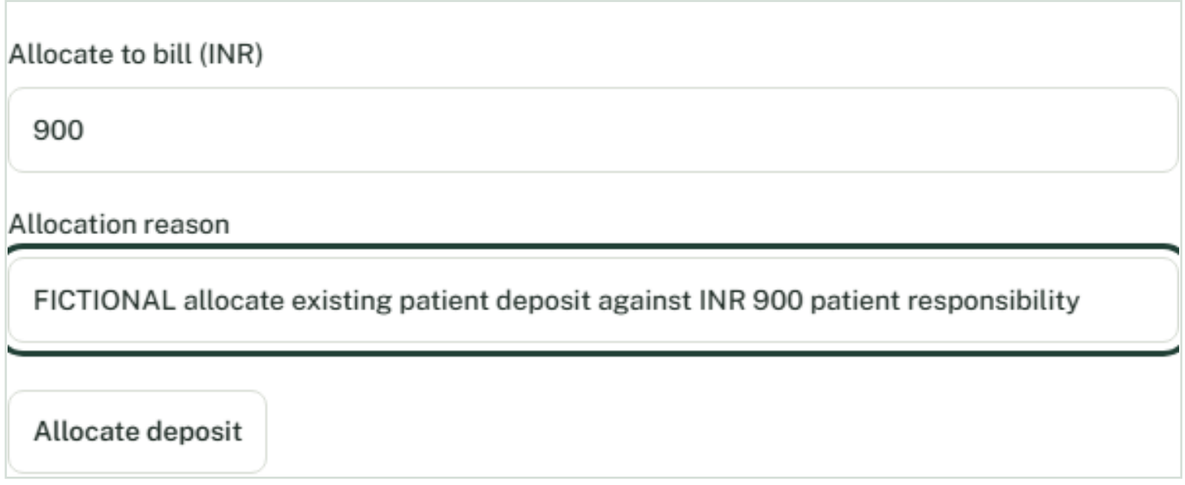
16 - Allocate ₹900 and record the actual ₹1,100 refund

Rohan > Money & insurance

1. Expand Allocate original deposit · FICTIONAL Rohan deposit 2000. Enter amount 900 and reason; click Allocate deposit against the ₹900 patient responsibility.
2. The held deposit has ₹2,000 − ₹900 = ₹1,100 excess. Refund it through the existing hospital channel before recording that actual event.
3. Expand Record an actual linked refund. Choose the original receipt; enter 1100, actual mode, channel, transaction reference, evidence, time and reason. Click **Record actual refund**.
4. Result: the patient share is settled and the excess is actually refunded. Accepted insurer ₹2,000 remains a separate receivable; no second deposit is entered.



Journey capture - Rohan

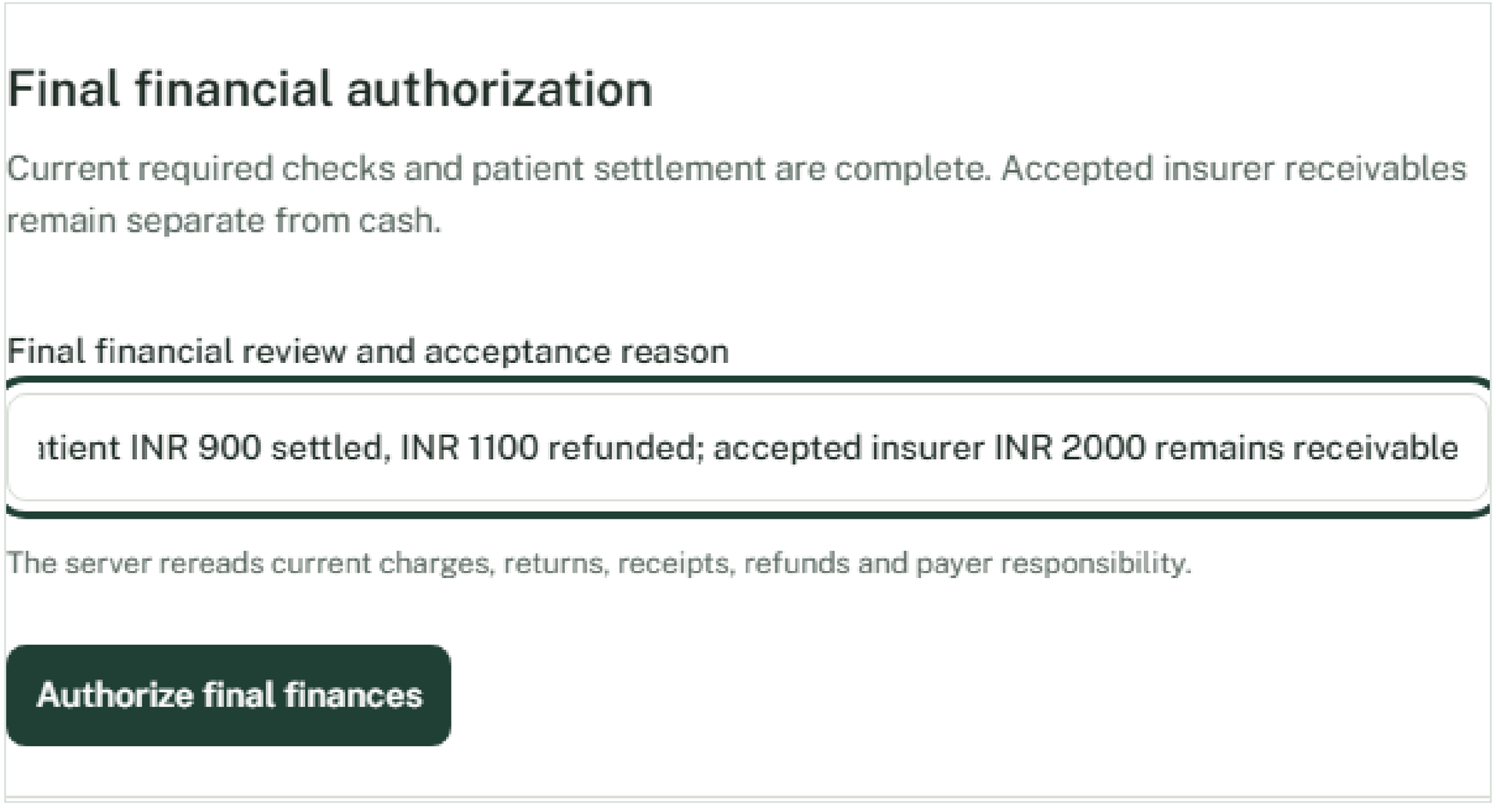


Journey capture - Rohan

17 - Authorize final finances with the receivable visible

Rohan > Vacancy, papers & exit > Final financial authorization

1. Check the final itemized bill, all applicable checks, accepted responsibility, deposit allocation and actual refund. Resolve every displayed blocker.
2. When the screen states Current required checks and patient settlement are complete, enter Final financial review and acceptance reason.
3. Click **Authorize final finances**. The server rereads current charges, returns, receipts, refunds and payer responsibility before accepting it.
4. Rohan: ₹2,900 bill = ₹900 patient settled + ₹2,000 accepted insurer receivable. Insurer cash is still ₹0; accepted credit can remain receivable at ordinary exit.

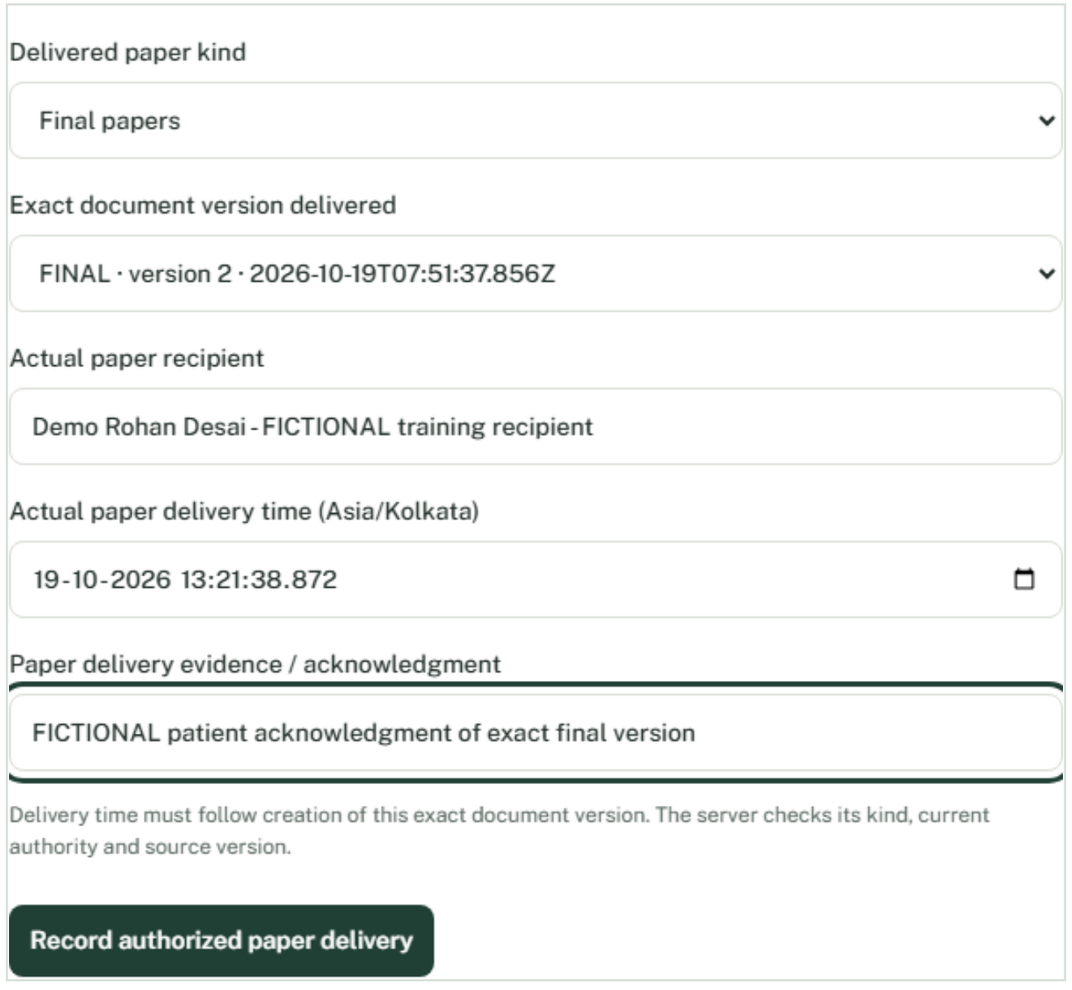


Journey capture - Rohan

18 - Generate and deliver the exact final paper version

Rohan > Vacancy, papers & exit

1. After final finance, expand Generate an authorized paper version. Select Final papers · after financial authorization and click Generate authorized paper.
2. Open the saved FINAL · version link and review that protected version before giving the papers to the recipient.
3. Expand Deliver authorized patient papers. Select Final papers and Exact document version delivered; enter actual recipient, delivery time and acknowledgment/evidence.
4. Click Record authorized paper delivery. Generation and delivery are separate records, and delivery must follow creation of that exact version.

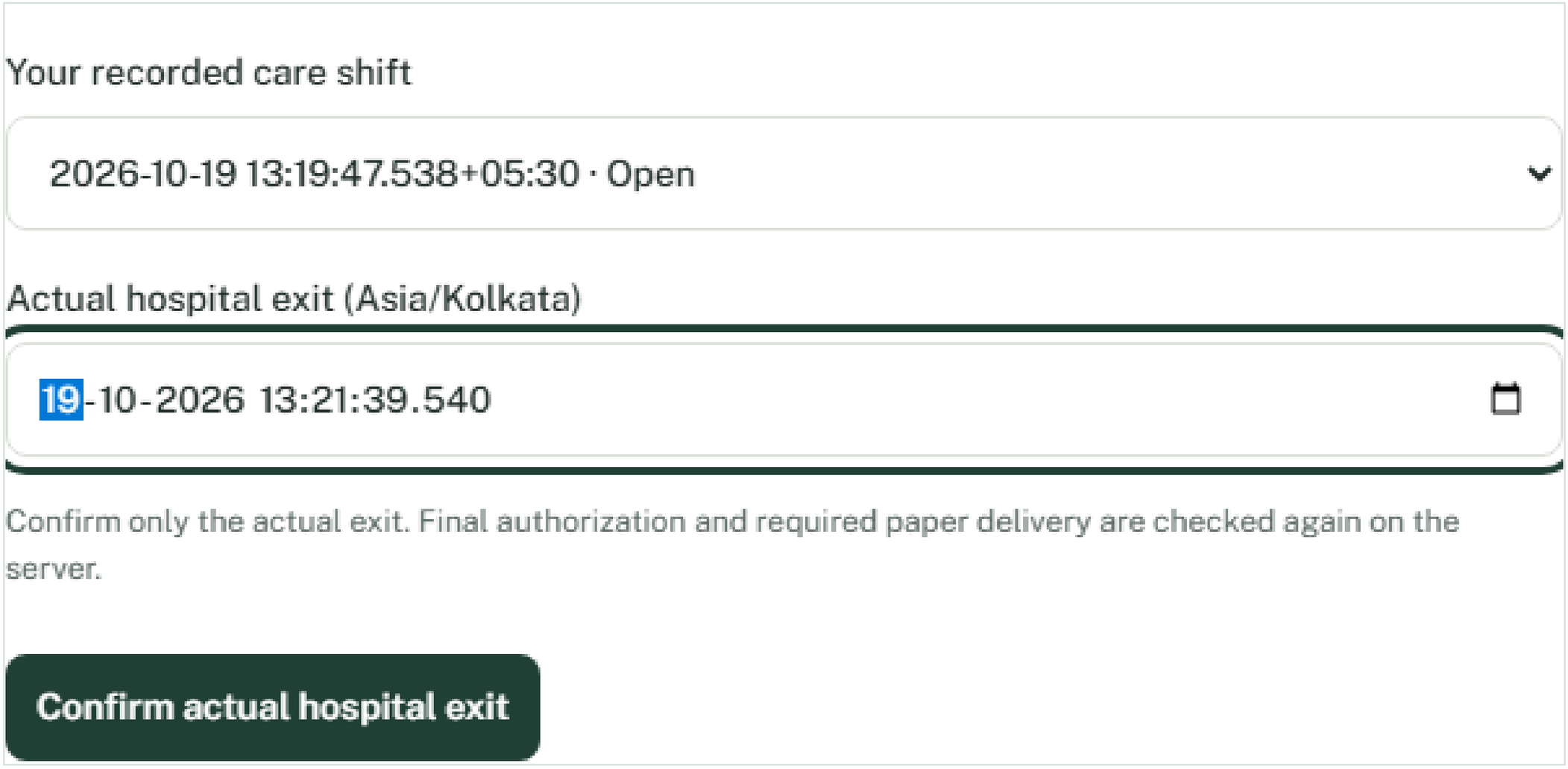


Journey capture - Rohan

19 - Confirm actual hospital exit

Rohan > Vacancy, papers & exit > Confirm actual hospital exit

1. After required authorizations and paper delivery, choose the responsible Nurse care shift and enter the actual hospital exit time.
2. Click **Confirm actual hospital exit** only when the patient really leaves. The server checks final authority and required paper delivery again.
3. Result: Rohan is DISCHARGED and the historical exit is retained. This is separate from the earlier move into waiting care.
4. The bed still needs authorized cleaning/turnover before reuse. Ending a Nurse shift alone does not confirm hospital exit.

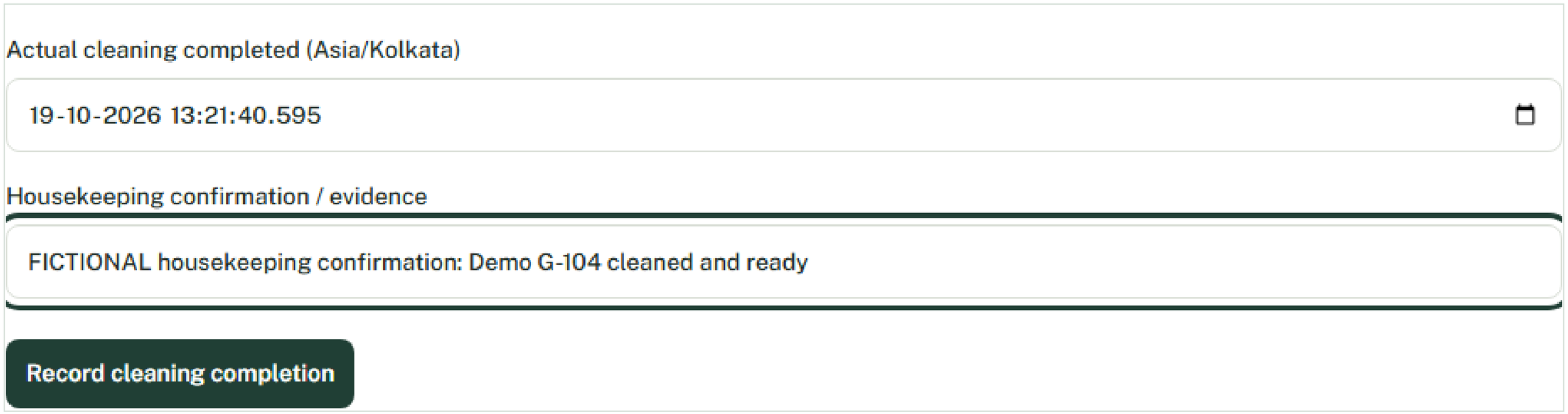


Journey capture - Rohan

20 - Record housekeeping completion and release the bed

Discharge > Retained MVP episodes > Cleaning & turnover before bed reuse; then Beds

1. After actual exit, open Demo G-104 · cleaning pending in the Cleaning & turnover before bed reuse panel.
2. Enter Actual cleaning completed and Housekeeping confirmation / evidence from the real cleaning team. Click Record cleaning completion.
3. Return to Beds. Result: Demo G-104 is Ready and can be reserved for a later patient.
4. Keep the sequence distinct: physical vacancy ends room use; actual exit closes the patient's departure; confirmed cleaning/turnover makes the bed reusable.

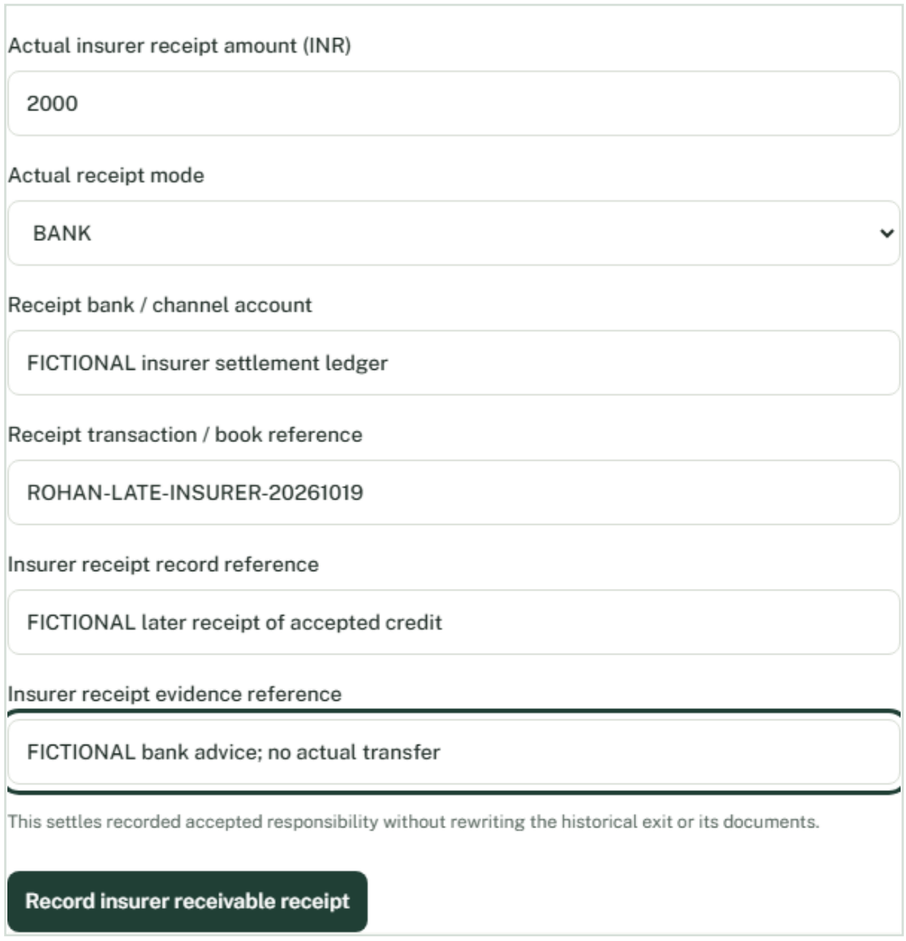


Journey capture - Rohan

21 - Settle the later insurer receipt without changing exit

Retained Rohan episode > Money & insurance > Record actual insurer receipt against receivable

1. When the insurer really pays through the hospital's existing channel, enter Actual insurer receipt amount 2000 and Actual receipt mode BANK.
2. Record Receipt bank / channel account, transaction/book reference, Insurer receipt record reference and evidence reference. Click Record insurer receivable receipt.
3. Result: accepted insurer ₹2,000 − actual insurer ₹2,000 = ₹0 remaining receivable. Patient cash retained is ₹900 after the ₹1,100 refund.
4. Rohan stays DISCHARGED. The recorded exit, delivered-paper status and original final PDF remain unchanged; matching PDF hashes were verified in the completed exercise.



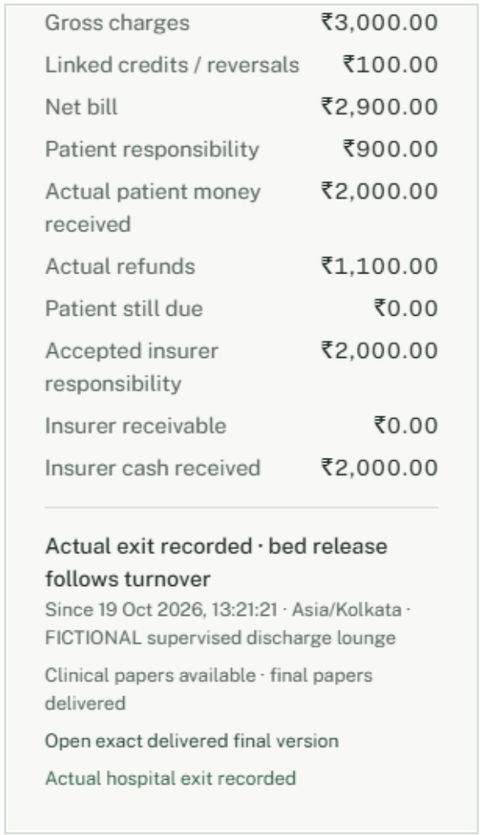
Journey capture - Rohan

22 - Sign the shift handover and review retained history

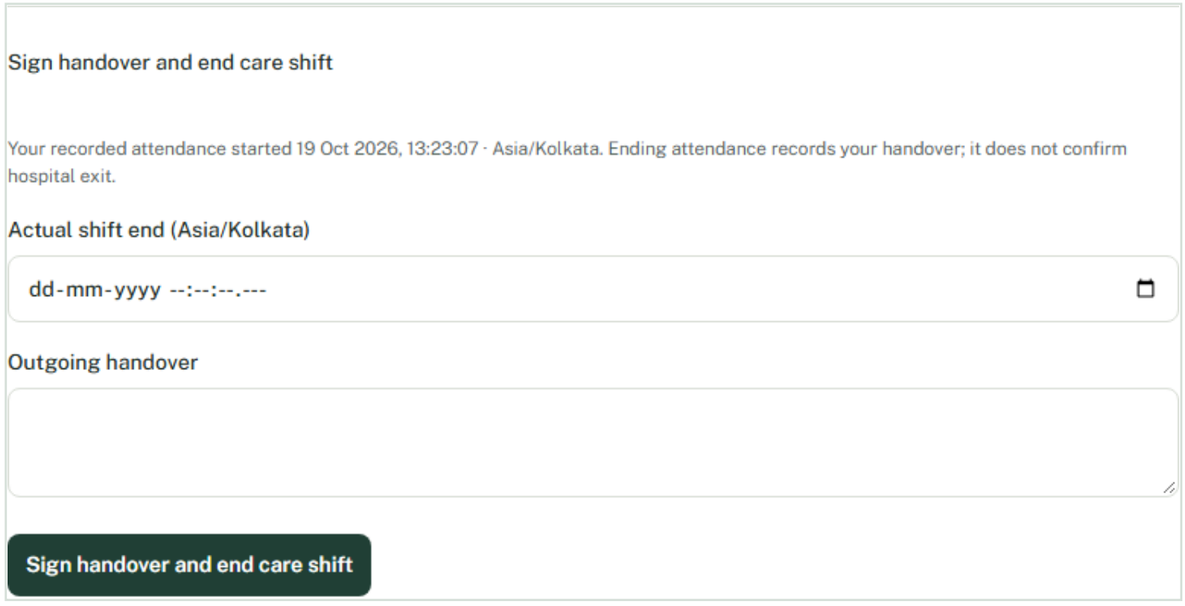
Rohan > Readiness & checks; role queue > Retained MVP episodes / Retained MVP cases

1. For an open recorded shift, expand **Sign handover and end care shift**. Enter Actual shift end and Outgoing handover, then click **Sign handover and end care shift**.
2. This was completed for Rohan after the journey, using his actual exit time. A post-departure recorded shift end must be no later than actual hospital exit.
3. Use Retained MVP episodes, or Insurance's Retained MVP cases, then Search patients by Rohan's name, UHID or IP code to inspect the completed admission.
4. Retained records in the sidebar opens earlier workflow history. It is separate from the current MVP queue's latest 50 closed episodes; role and assignment access still apply.

Note: The shift-end form shown here belongs to still-admitted Kabir; it is a reference only. Rohan's shift handover was already recorded at his actual exit time. Do not submit this form for another patient while following Rohan's history.



Current section / form reference

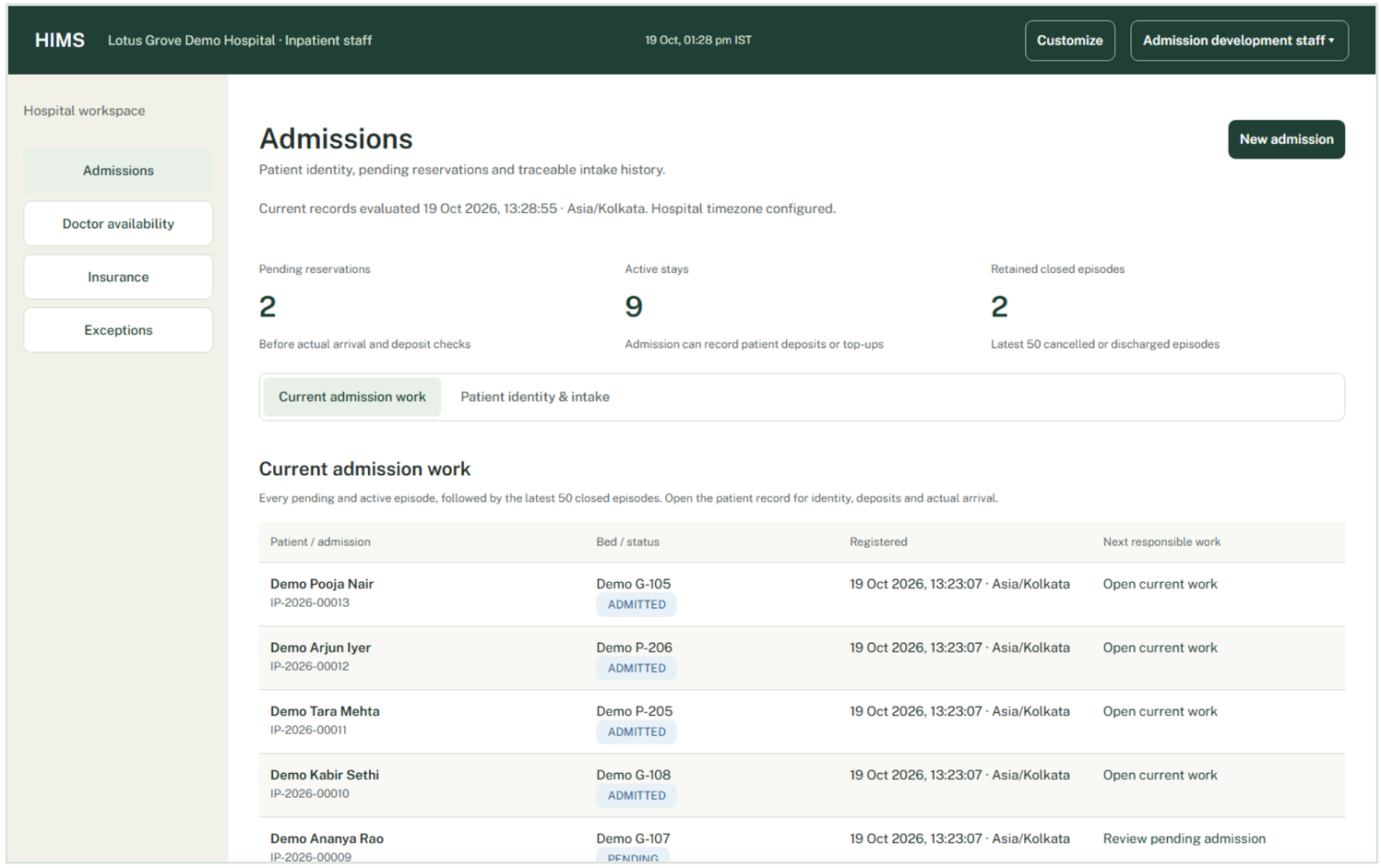


Current section / form reference

Admissions - current work

Admissions > Current admission work

1. Use **Current admission work** to see pending reservations, active stays and the latest 50 closed episodes. The UHID identifies the person; the IP code identifies this stay.
2. Review pending admission opens deposits and arrival for a reservation. Open current work opens the active admission record; Review retained history opens a closed record.
3. **New admission** switches to **Patient identity & intake**. Saving intake reserves a bed; it does not start occupancy or room charges.
4. Create limited insurance case opens the Insurance handoff. Urgent authority and identity follow-up opens the relevant exception work.

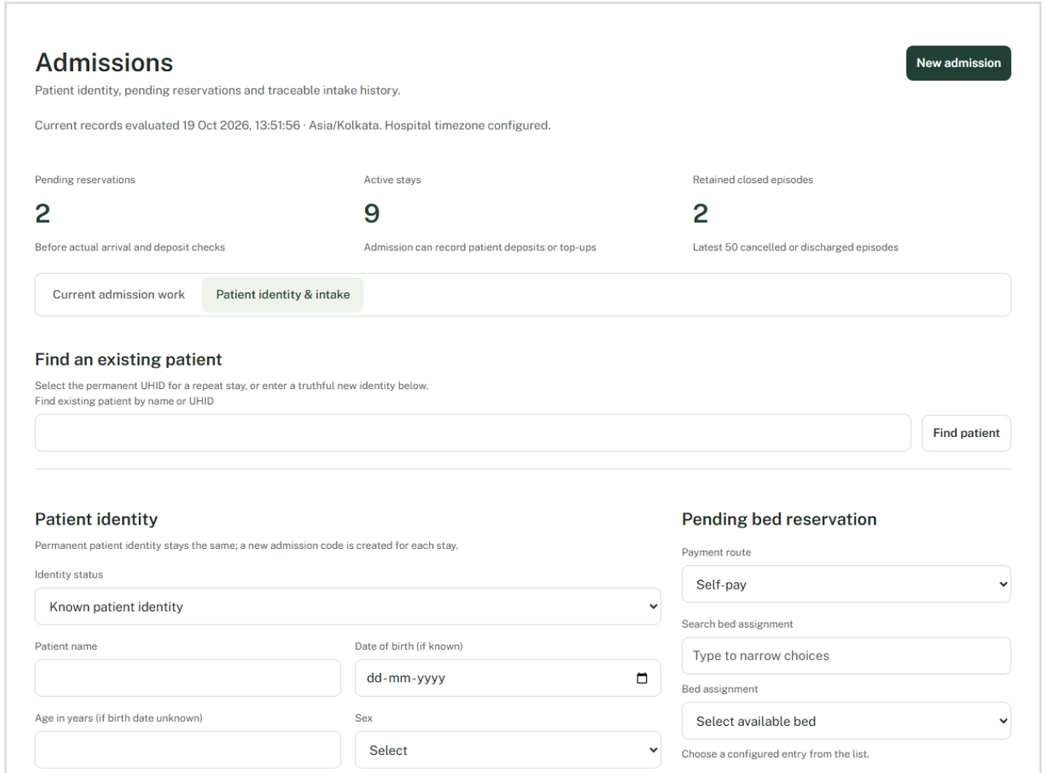


Current section / form reference | upper workspace; scroll for more

Patient identity & intake - create the reservation

Admissions > Patient identity & intake

1. Search Find existing patient by name or UHID, then select **Find patient**. Choose the matching permanent identity for a repeat stay; Register a different patient clears that selection.
2. For a new identity, enter Patient name, Date of birth (if known) or Age in years (if birth date unknown), Sex and an available Contact number (if available). Urgent temporary identity / facts unknown requires truthful provenance and later Admin authority.
3. Select Named treating Doctor, Payment route, Bed assignment and Expected stay (days). Doctor and bed choices are searchable; the current form still requires a configured Doctor.
4. **Save pending admission** creates a separate IP code and reserves the selected bed. The displayed Configured initial deposit is a rule, not a receipt.
5. After saving, Open admission record continues the same episode; Start another admission opens a fresh intake. Never create another copy merely to continue this stay.

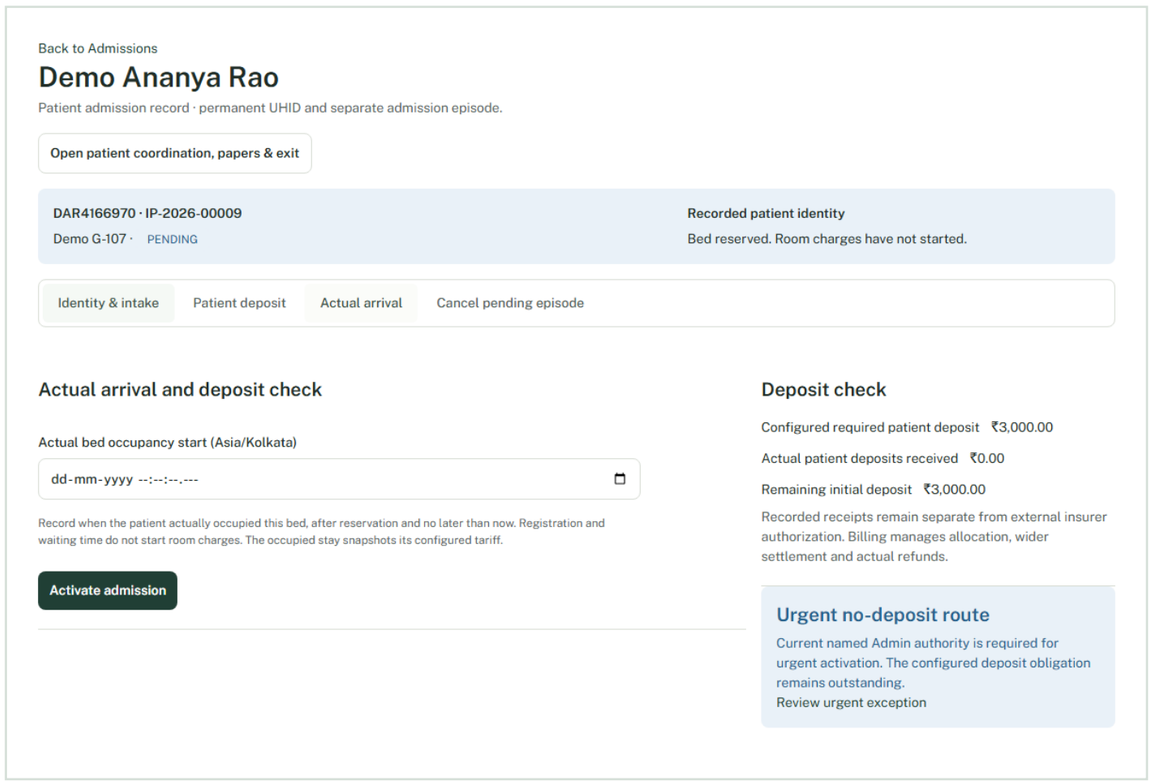


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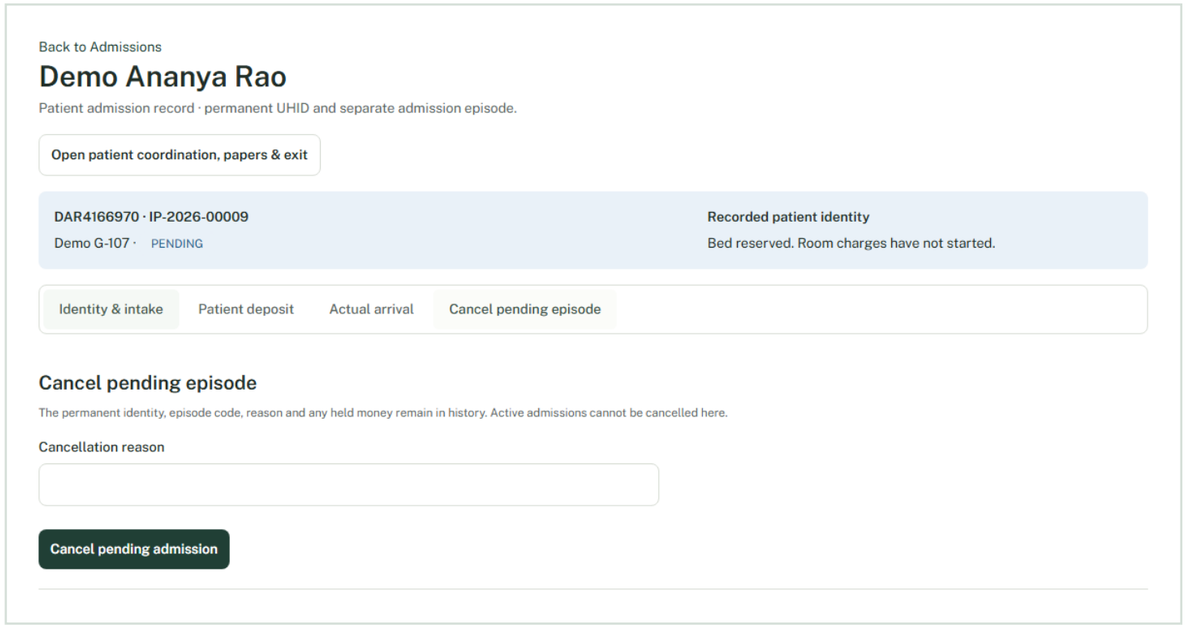
Patient record - deposits, arrival and cancellation

Open a pending patient > Identity & intake / Patient deposit / Actual arrival / Cancel pending episode

1. **Identity & intake** shows the permanent UHID, episode IP, reserved bed and actual admission status. **Open patient coordination, papers & exit** opens the episode's wider workflow.
2. **Patient deposit** records actual deposits or top-ups. Enter Amount received (INR), Payment mode, Cash book / channel account, Receipt number / transaction ID and Staff reference, then **Record manual receipt**. Use actual hospital evidence.
3. **Actual arrival** shows the Deposit check beside Actual bed occupancy start (Asia/Kolkata). **Activate admission** starts the actual occupied stay using the configured tariff; registration and waiting time do not start room charges.
4. **Review urgent exception** opens the named Admin authority route when ordinary activation requirements cannot be met. It does not erase the deposit obligation.
5. **Cancel pending episode** requires Cancellation reason, then **Cancel pending admission**. This retains the identity, IP and held money; an active admission cannot be cancelled here.



Current section / form reference

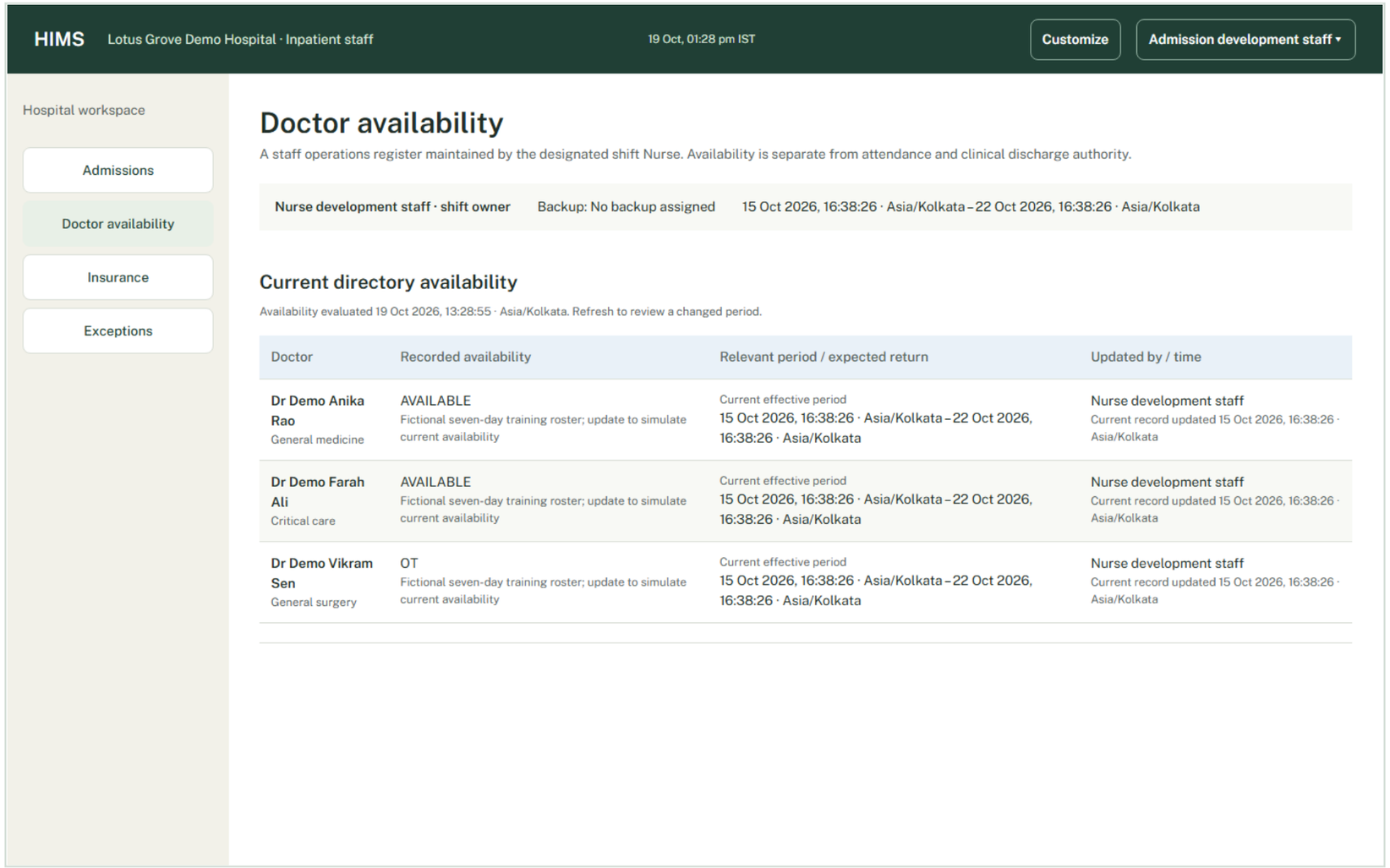


Current section / form reference

Doctor availability - read the maintained directory

Doctor availability

1. Current directory availability shows each configured Doctor’s specialty, recorded status, effective period and staff update time.
2. Check the current period and expected return, not only the last recorded status. Refresh the page when staff change the register.
3. The designated shift Nurse or backup maintains availability. Admission views this register; Admin assigns the operational responsibility.
4. Availability is separate from attendance and clinical discharge approval. A directory Doctor is an external clinical identity, without a routine dashboard login.

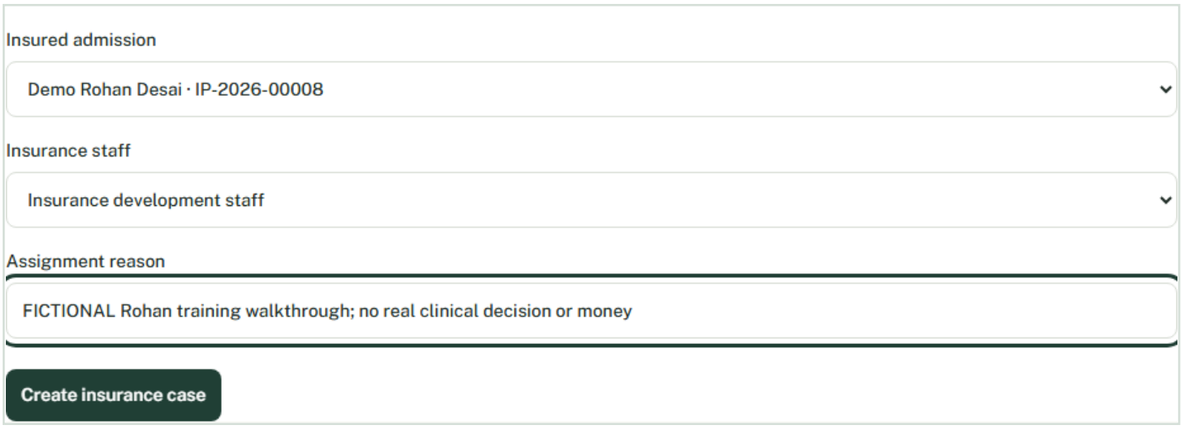


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Insurance - hand the insured admission to its owner

Insurance / patient record > Create limited insurance case

1. Choose an insurance case lists current handoffs and their Assigned desk. Admission sees the handoff status; Insurance staff owns policy, decisions and protected files.
2. When an insured admission has no case, open Create and assign an insured case, choose Insured admission and Insurance staff, enter Assignment reason, then Create insurance case.
3. Insurance case is recorded confirms that the handoff already exists and names its owner. Return to admission continues the same patient; View insurance handoffs returns to the list.
4. Do not recreate a saved case to continue admission. The insurer / TPA or hospital staff verify coverage externally; creating a local case approves no coverage or payment.



Journey capture - Rohan

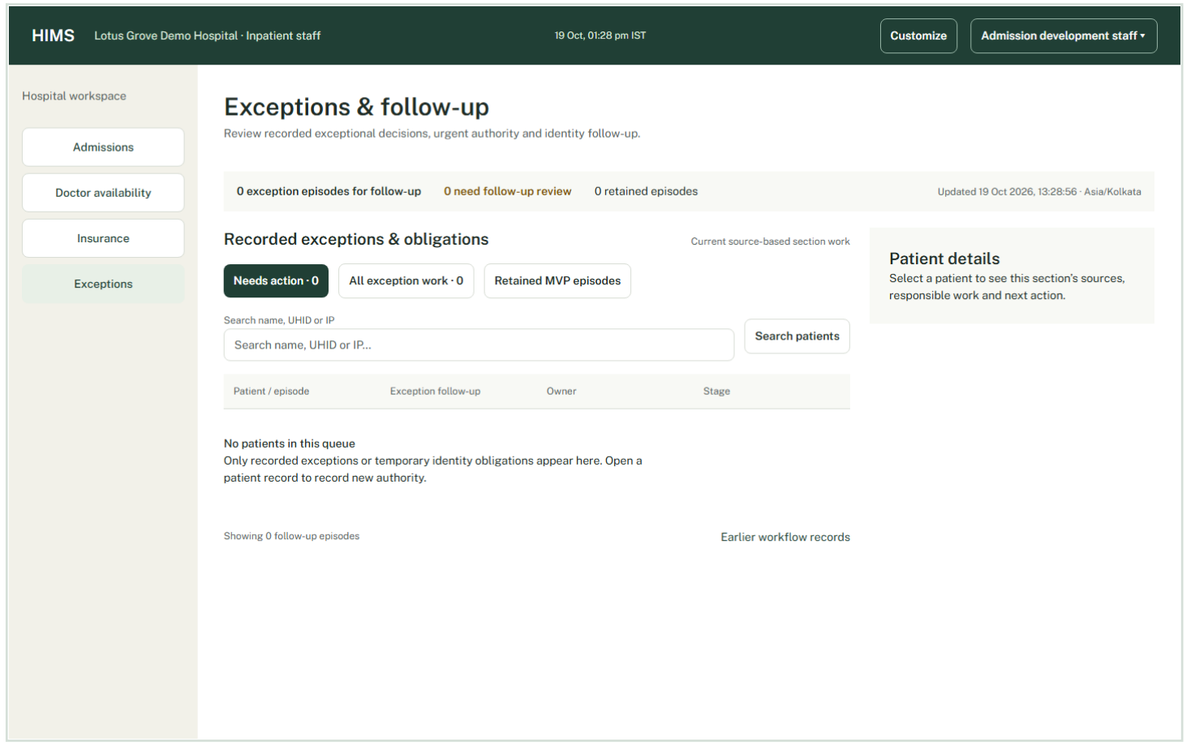


Journey capture - Rohan

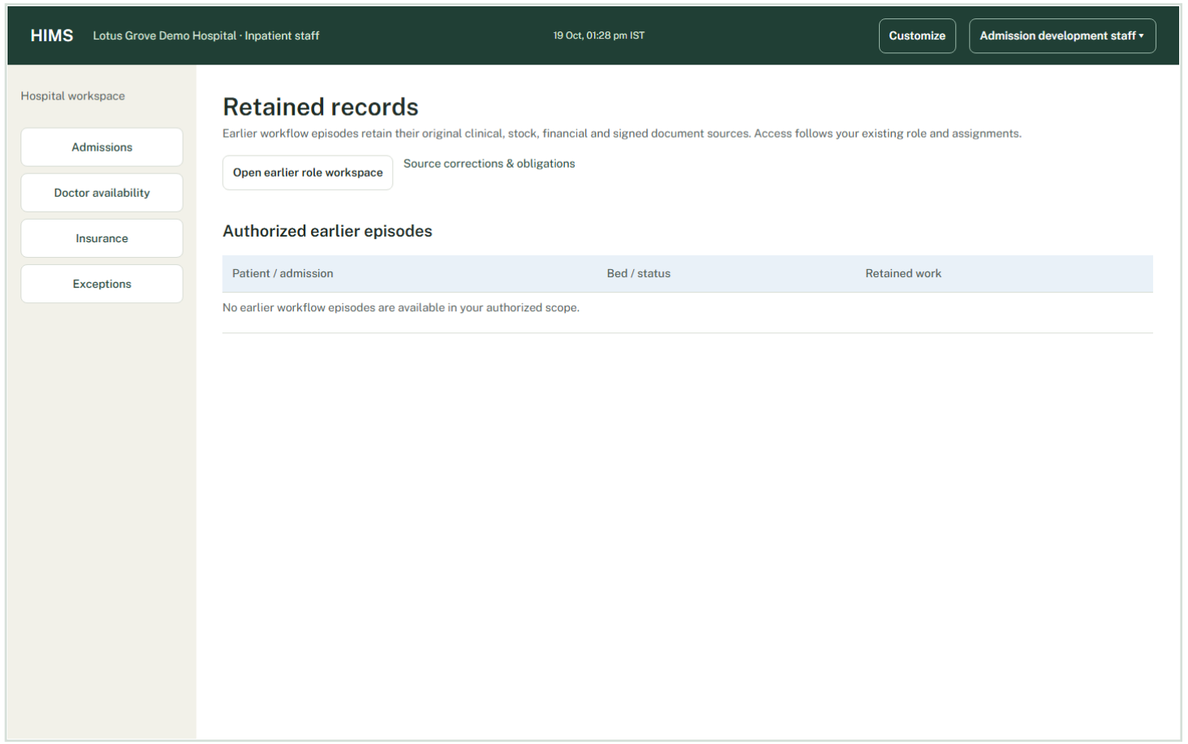
Exceptions and retained records - follow up without losing history

Exceptions / Retained records

1. Exceptions lists recorded intake authority and temporary identity follow-up. Needs action and All exception work filter the queue; Search patients accepts name, UHID or IP.
2. No patients in this queue is valid when no relevant exception is recorded. Start new authority from the relevant patient record, not an unrelated clinical or financial queue.
3. Retained MVP episodes shows closed new-workflow episodes within the section. The current implementation retains the latest 50 closed episodes in these queues.
4. Retained records opens earlier-workflow history. Open earlier role workspace and Source corrections & obligations preserve their authorized source paths; an empty earlier-workflow list does not mean the new MVP history was deleted.



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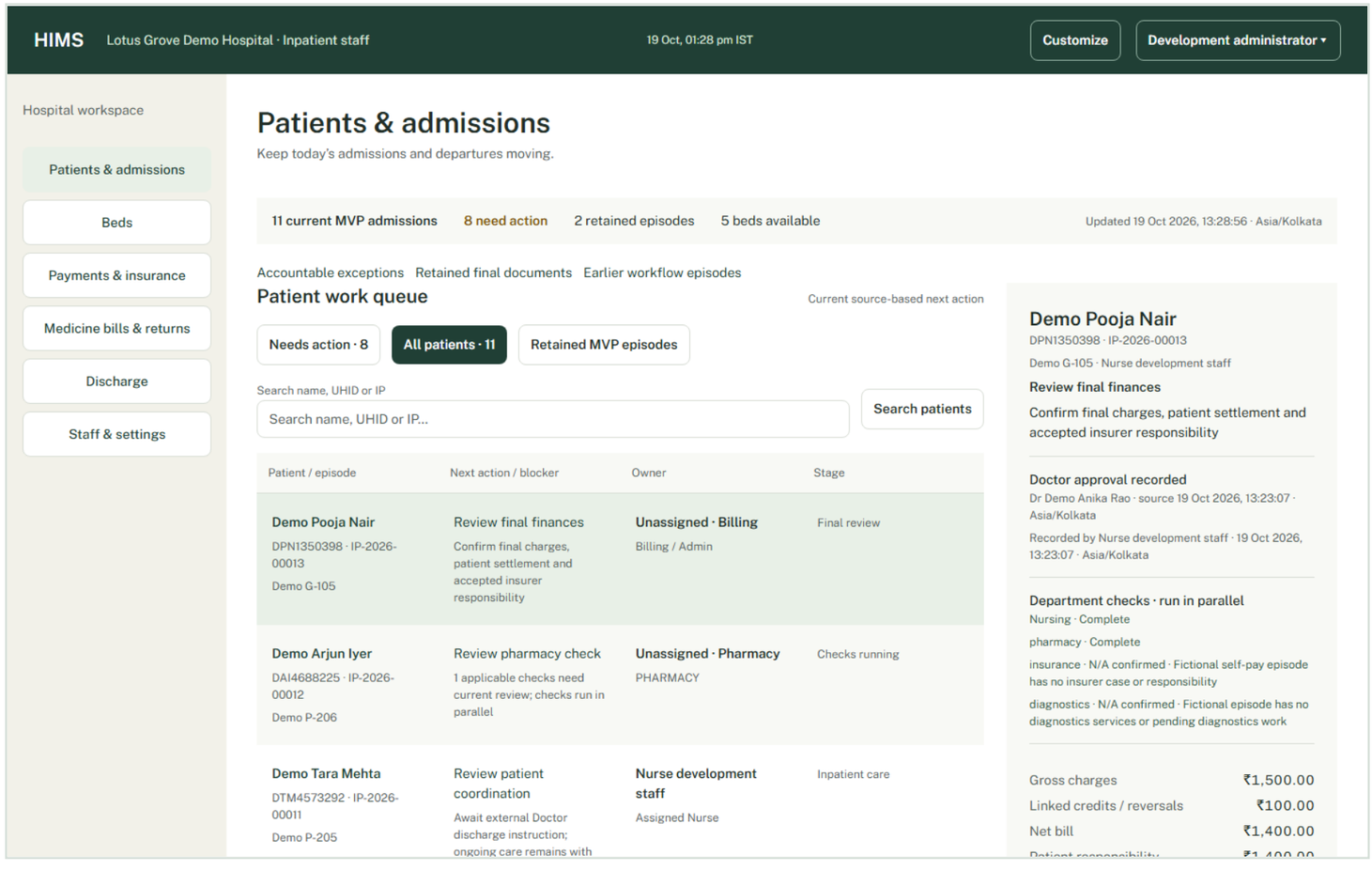


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Patients & admissions - coordinate the next desk

Patients & admissions

1. All patients shows current episodes; Needs action narrows the queue to flagged work. Search patients finds a patient by name, UHID or IP. Selecting the name shows its section summary; the task link opens its actual work.
2. Read Next action / blocker, Owner and Stage together. Admin coordinates assignments and blockers; the professional author remains accountable for each check.
3. Accountable exceptions, Retained final documents and Earlier workflow episodes open their corresponding retained sources.
4. Review physical beds opens physical availability. View availability and responsibility opens the separate Doctor availability register.
5. A reserved bed, actual occupancy, physical vacancy, actual exit and cleaning are separate facts. A vacant bed can remain held until exit and turnover.

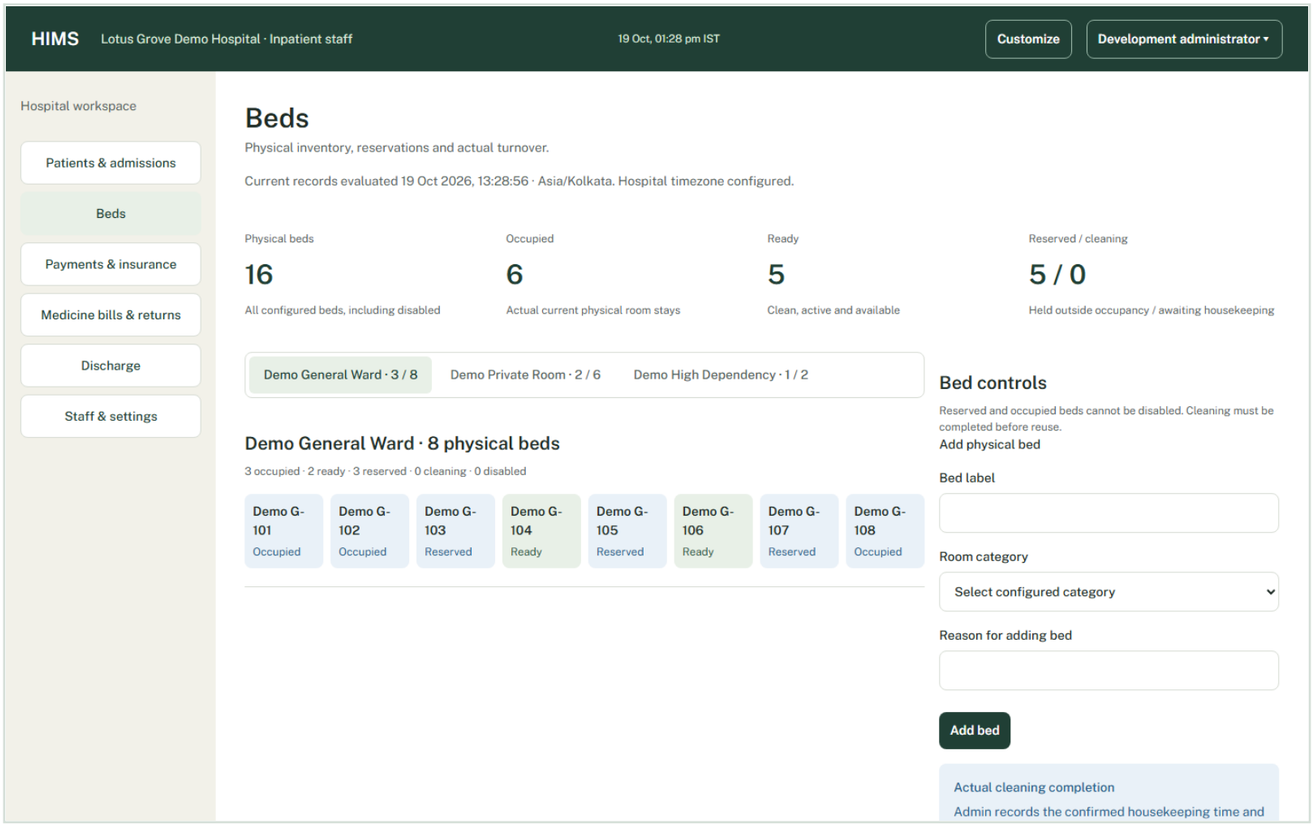


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Beds - maintain physical capacity and turnover

Beds

1. Category tabs group the existing beds. Occupied counts actual physical stays; Ready means clean, active and available. Reserved / cleaning are held outside occupancy and awaiting housekeeping respectively.
2. Add physical bed requires Bed label, Room category and Reason for adding bed, then Add bed. Configure an active category in Hospital settings first.
3. An occupied or reserved bed tile opens its admission record. In Pending reservations, Review deposit and actual arrival opens that patient's arrival work.
4. Review cleaning and turnover opens Discharge. After actual exit, Record cleaning completion records Actual cleaning completed (Asia/Kolkata) and Housekeeping confirmation / evidence before reuse.
5. Physical bed inventory and disable controls exposes Disable available bed. Enter Reason for disabling bed, then Disable bed; reserved, occupied and cleaning beds cannot be disabled through this control.

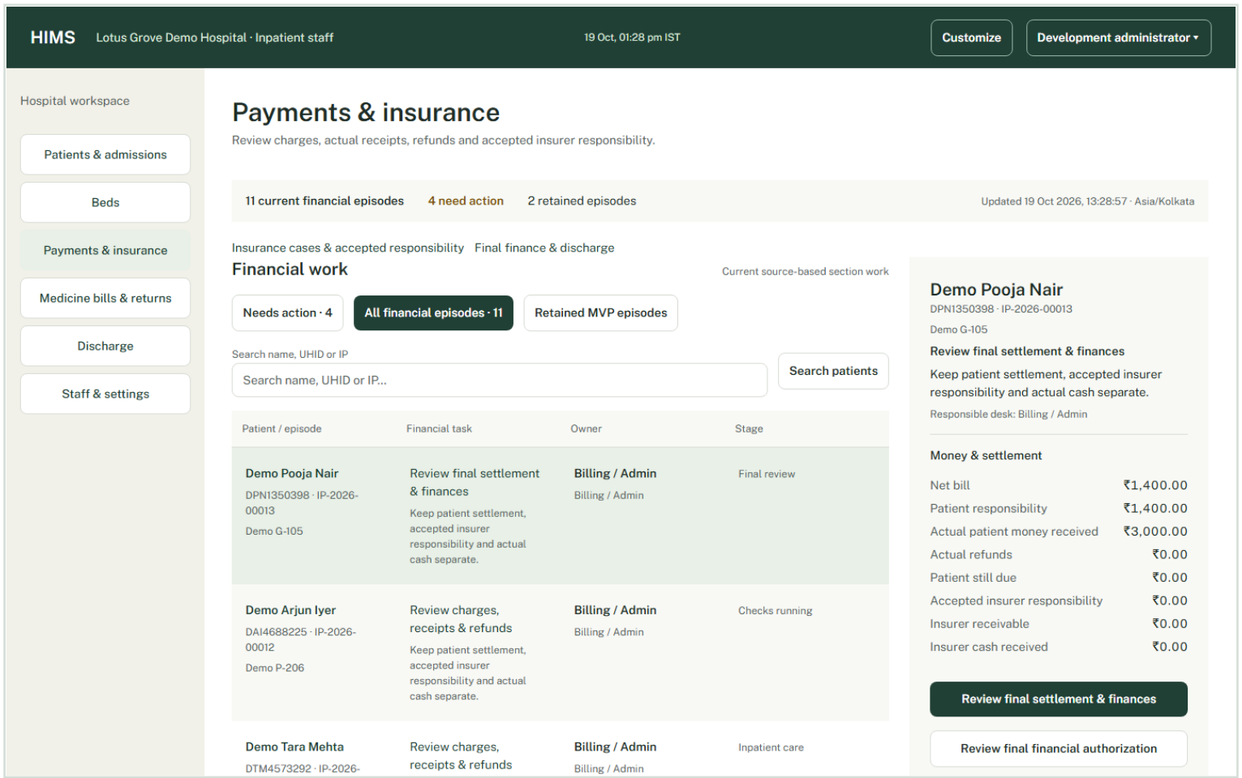


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Payments & insurance - oversee money and responsibility

Payments & insurance

1. All financial episodes includes routine charges, receipts and refunds; Needs action highlights pending arrivals and final reviews. Retained MVP episodes includes later receivable work for closed stays.
2. Review charges, receipts & refunds or Review final settlement & finances opens Money & insurance for that exact patient.
3. Insurance cases & accepted responsibility opens verified case work. Final finance & discharge opens discharge preparation; Review final financial authorization goes to the selected patient’s final gate.
4. Keep patient responsibility, actual patient money, actual refunds, accepted insurer responsibility, insurer receivable and insurer cash separate. Accepted insurer credit may remain receivable at ordinary exit.
5. Admin / Billing can **Record manual receipt**, Allocate deposit and **Record actual refund** using actual source evidence. A medicine return credit or insurance authorization is not cash. The Billing chapter shows these forms.

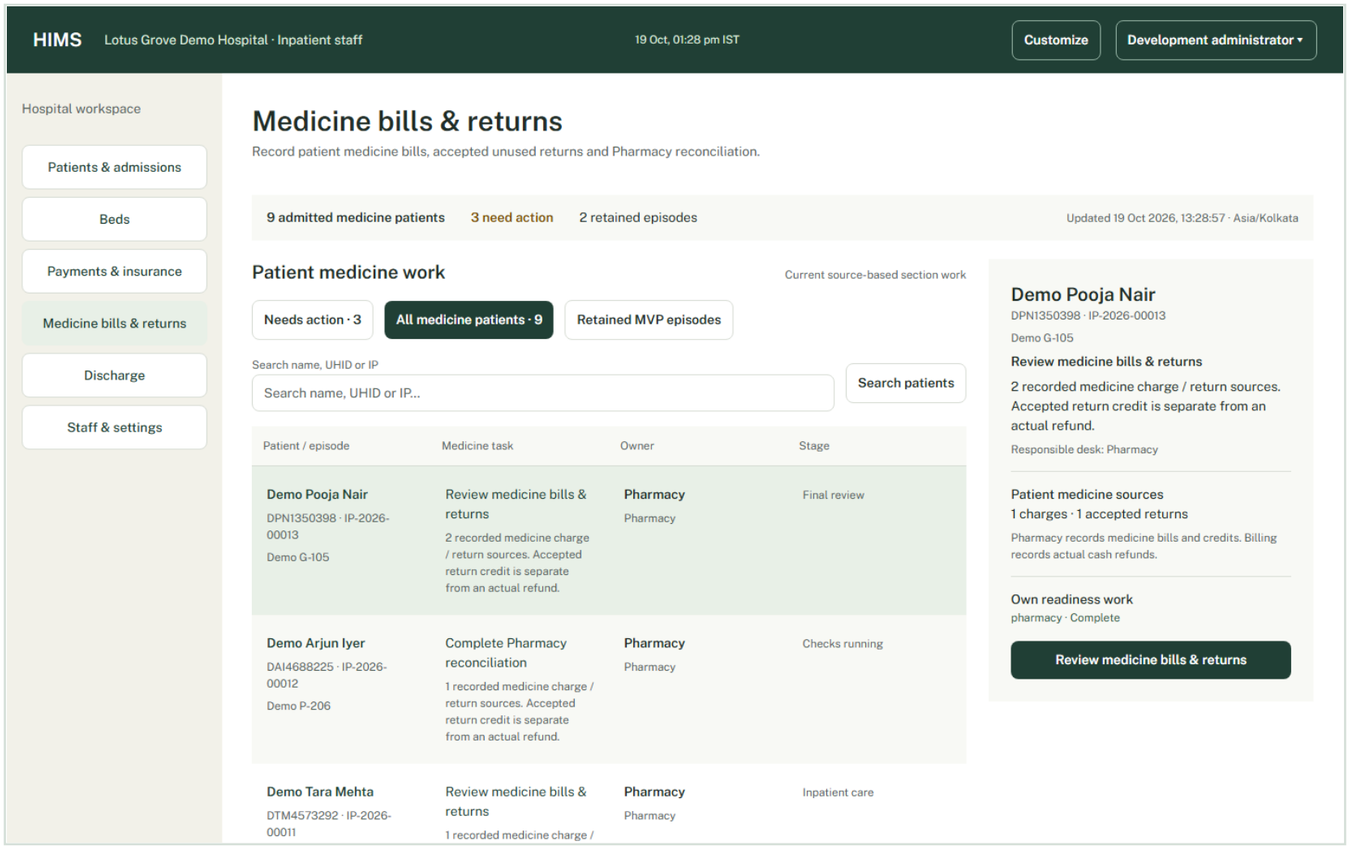


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Medicine bills & returns - review Pharmacy sources

Medicine bills & returns

1. All medicine patients includes admitted patients with routine medicine work, not only those preparing for discharge. Needs action highlights applicable Pharmacy reconciliation.
2. Review medicine bills & returns or Complete Pharmacy reconciliation opens the patient's Money & insurance sources for Admin oversight. Pharmacy staff records its own separate readiness check.
3. Patient medicine sources preserves original charges, accepted returns, original price and remaining returnable quantity. Accepted returns reduce the bill using the original price.
4. Pharmacy records medicine charges and accepted returns. Admin / Billing records actual cash refunds separately. This section is patient billing and reconciliation, without stock inventory or prescribing.

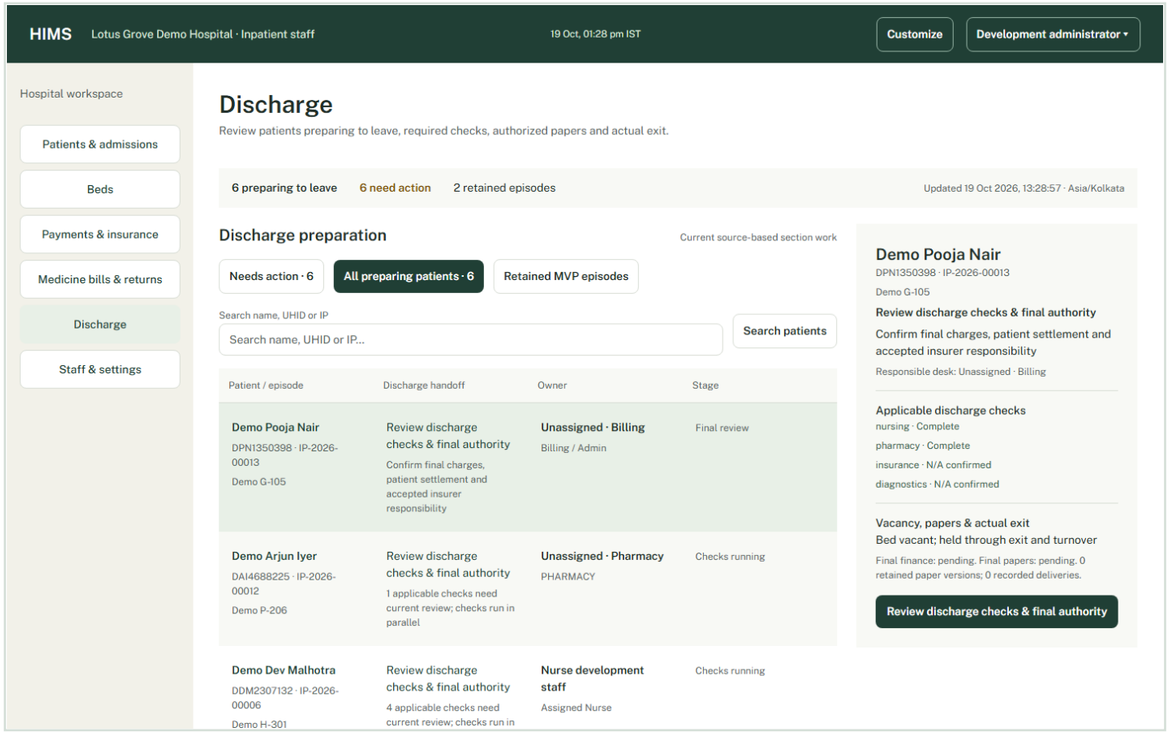


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Discharge - checks, financial authority and release

Discharge > Review discharge checks & final authority

1. All preparing patients contains patients with discharge preparation work. Review discharge checks & final authority opens that patient; Readiness & checks shows the recorded external Doctor instruction and parallel checks.
2. Admin uses Confirm proposed optional N/A > Confirm optional N/A only with No relevant service or pending work · reason. The server rejects N/A when applicable work remains. Doctor authority, Nursing and final finance stay mandatory.
3. Vacancy, papers & exit lists current blockers. Admin / Billing uses Final financial review and acceptance reason > **Authorize final finances** after current charges, refunds, patient settlement and accepted insurer responsibility are checked.
4. Generate an authorized paper version > Generate authorized paper creates an exact protected version. Deliver authorized patient papers > Record authorized paper delivery records its exact version, recipient, time and acknowledgment. Final papers require final finance.
5. The assigned Nurse records actual vacancy and hospital exit. Admin then records cleaning completion in Discharge before bed reuse. Financial authorization or paper generation alone does not prove that the patient left.

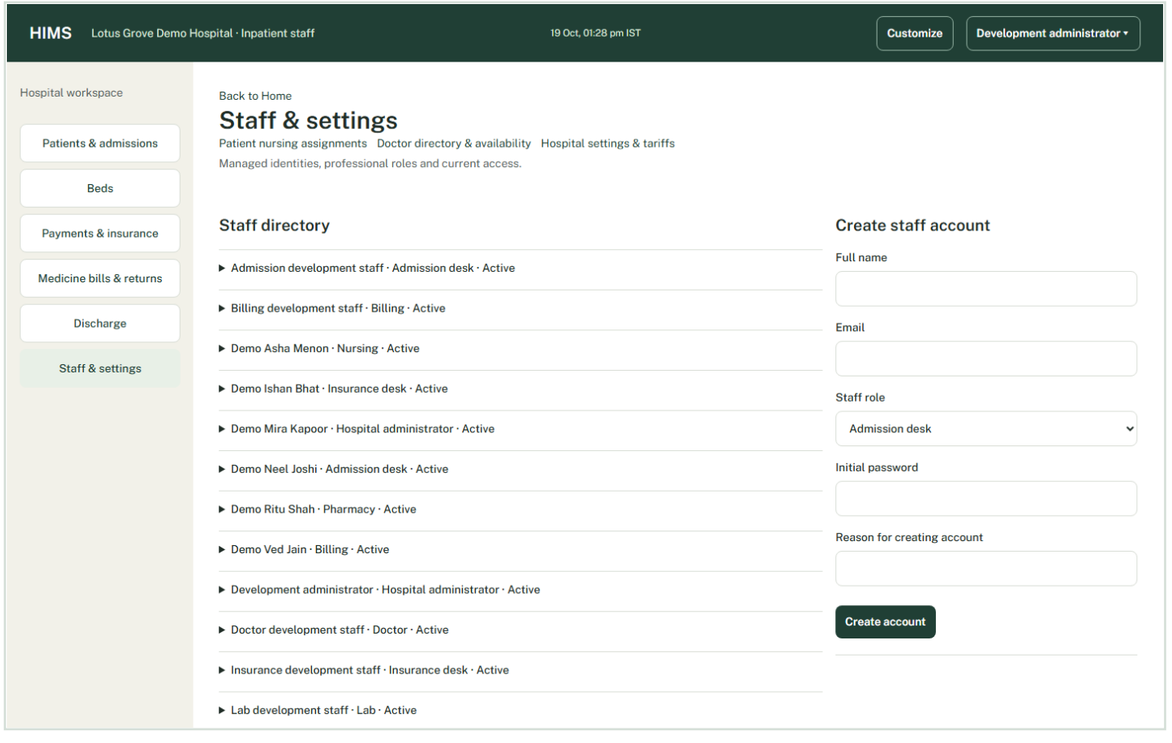


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Staff & settings - accounts and operational responsibility

Staff & settings

1. Create staff account requires Full name, Email, Staff role, Initial password and Reason for creating account, then **Create account**. Account creation alone assigns no patient or Insurance case.
2. Expand a Staff directory entry to edit Full name, Staff role, Account active or Reset password (optional). Enter Reason for change, then **Save staff changes**. Disabling, role changes and resets revoke sessions; the last active Admin is protected.
3. **Patient nursing assignments** opens Assignments. Select Admission and Nurse, enter Scheduling authority and Assignment reason, then **Record nursing assignment**. **Revoke nursing responsibility** requires authority and reason, then **Revoke recorded assignment**; an open shift needs signed handover first.
4. **Doctor directory & availability** opens the operations register. **Assign availability shift owner & backup** > **Assign availability responsibility** records its owner, optional backup and period; patient assignment is a separate responsibility.
5. **Add a treating Doctor to the directory** > **Add directory Doctor** creates an external identity without login. **Manage retained Doctor directory versions** > **Save directory version** retains changes. **Hospital settings & tariffs** opens the configuration tabs on the next page.



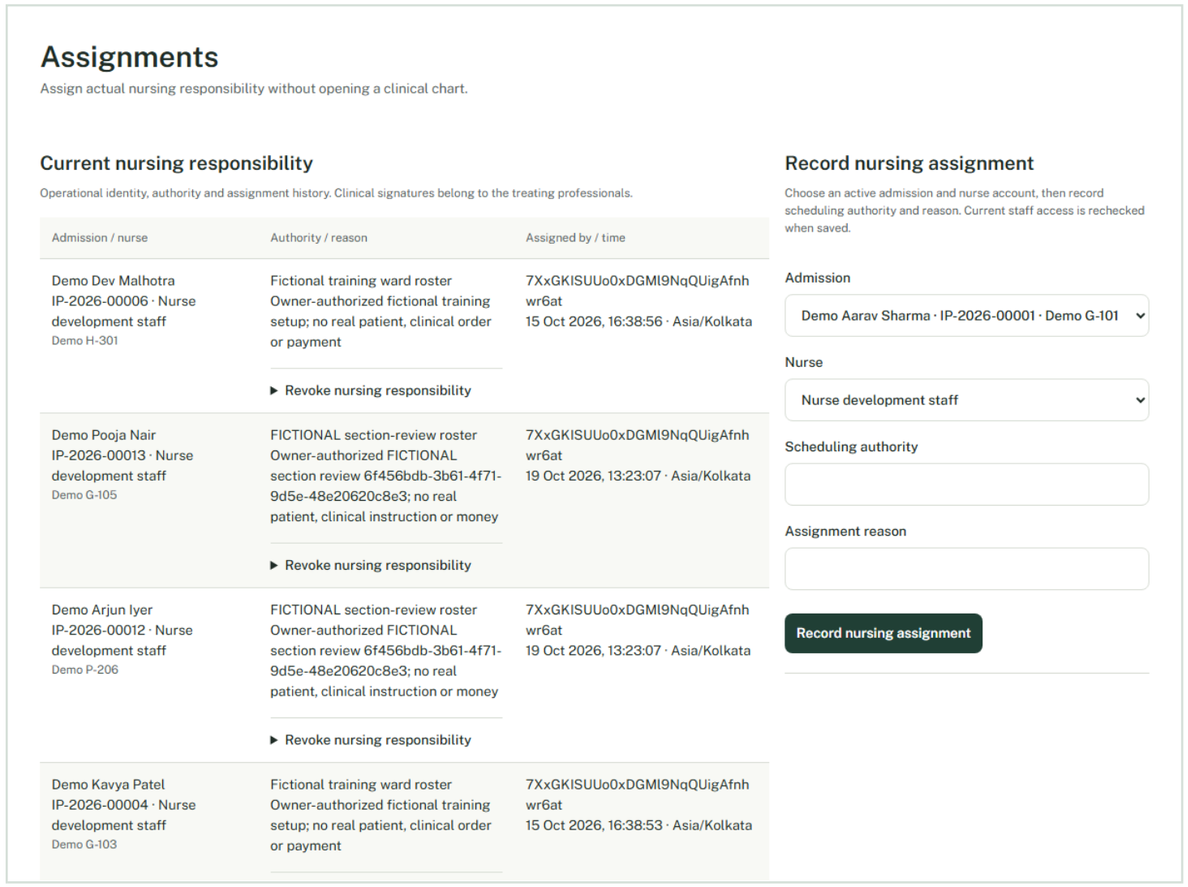
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Patient assignments and Doctor responsibility

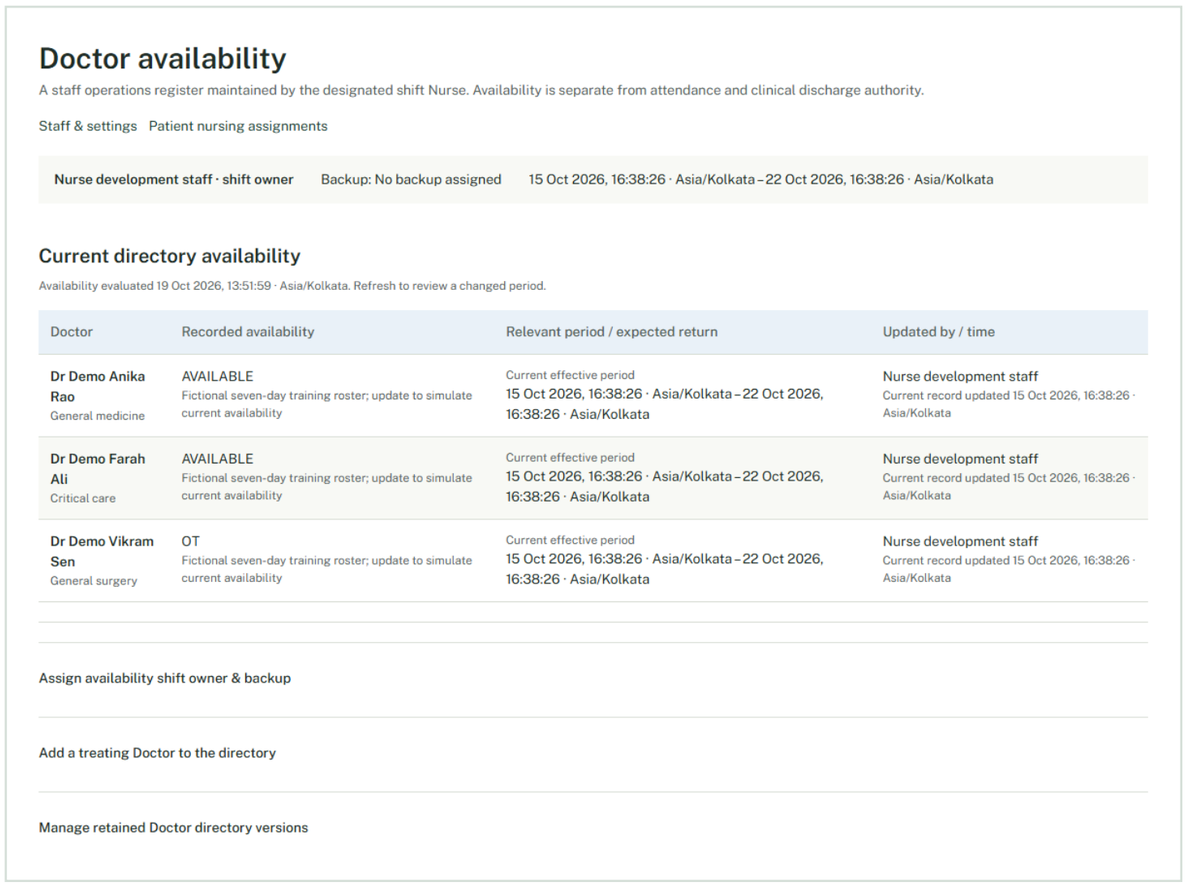
Staff & settings > Patient nursing assignments / Doctor directory & availability

1. **Patient nursing assignments** opens the care-assignment screen. Select the admission and responsible Nurse, scheduling authority and reason; **Record nursing assignment** establishes patient responsibility.
2. **Doctor directory & availability** maintains external Doctor identities and assigns who updates operational availability. A Doctor directory record does not create a routine Doctor login.
3. New named Nurse and Insurance accounts are unassigned. Existing role accounts own the prepared patients; deliberate assignment is needed before a new staff member can access them.

Note: Reference forms are shown without submitting any new assignment or Doctor configuration.



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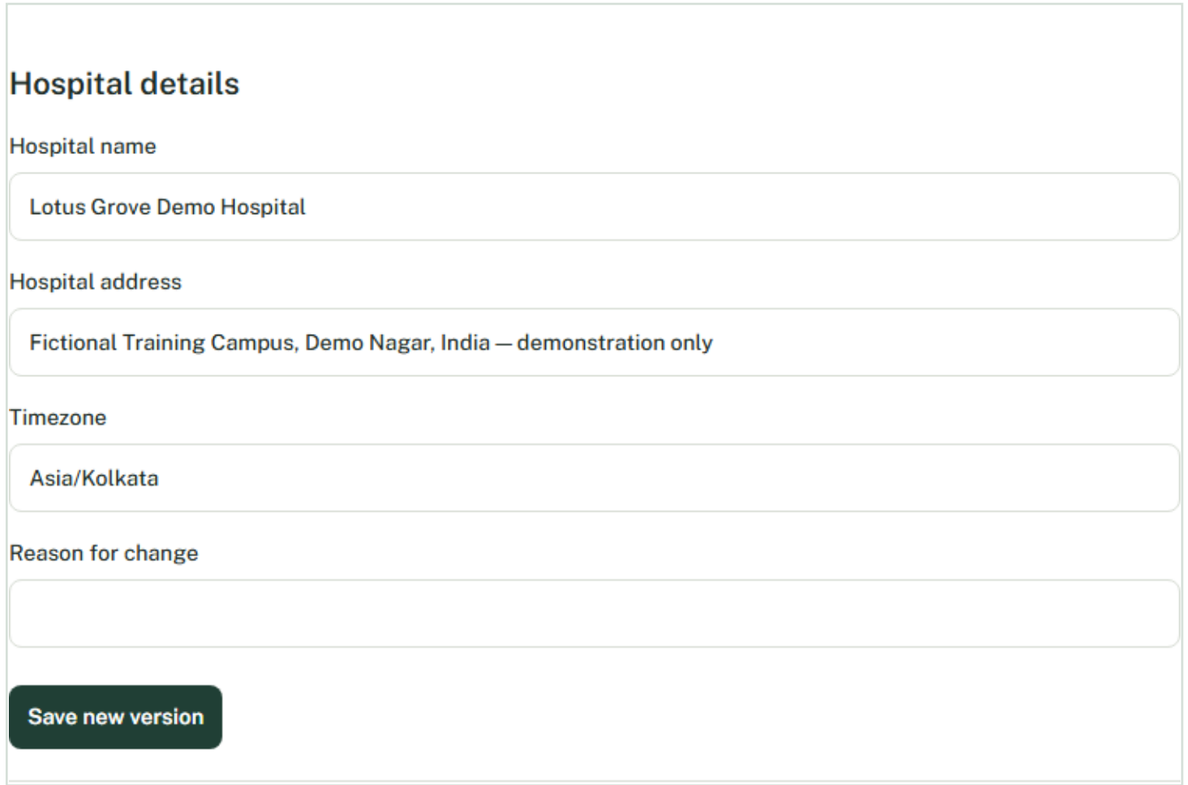
Current section / form reference

Hospital identity and bed categories

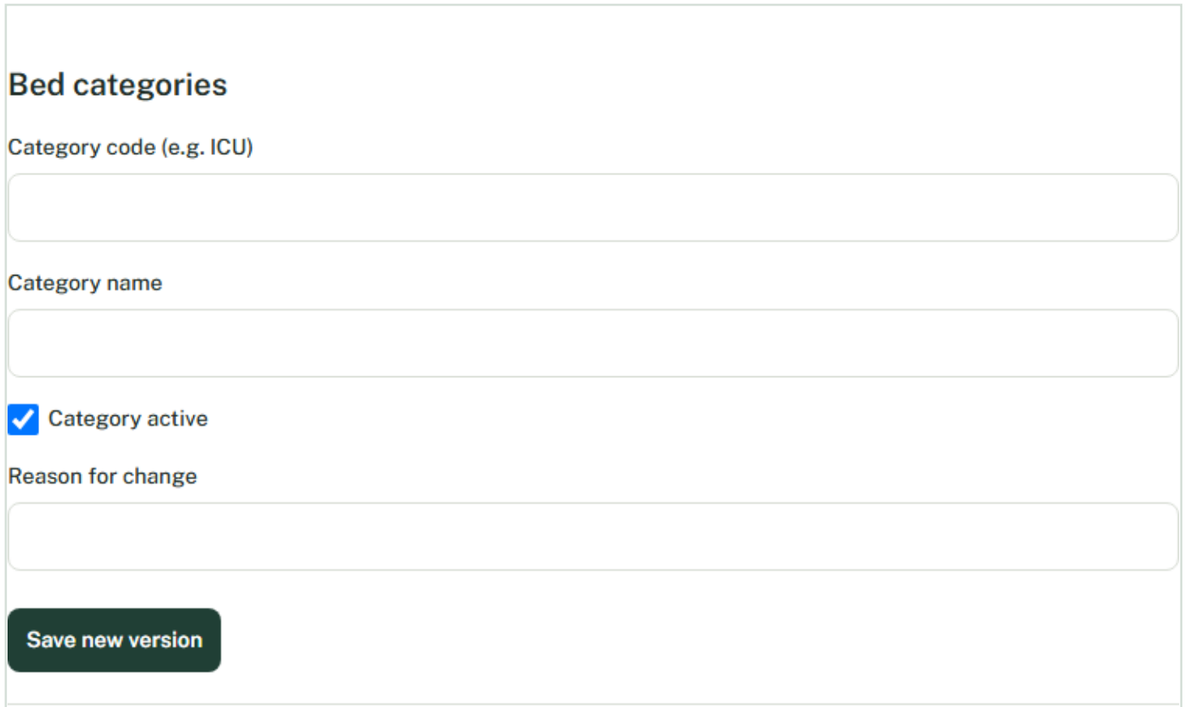
Staff & settings > Hospital settings & tariffs

1. Hospital: enter Hospital name, Hospital address and Timezone. Bed categories: define the Category code and Category name used by configured beds.
2. Each form requires Reason for change. **Save new version** retains earlier versions; it does not replace historical policy evidence.
3. Use existing configured codes consistently. Current beds are grouped by category and label; structured ward / room storage is still deferred.

Note: Configuration forms inspected without saving. Existing hospital configuration remains unchanged.



Current section / form reference



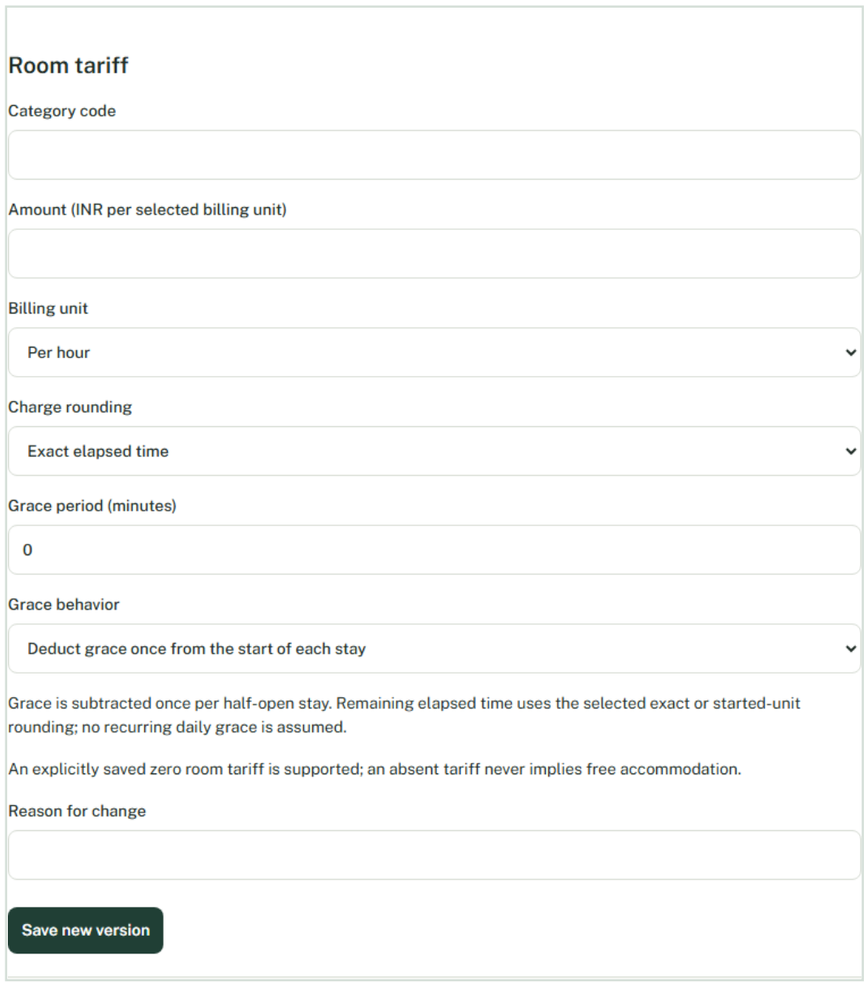
Current section / form reference

Room tariffs and billable items

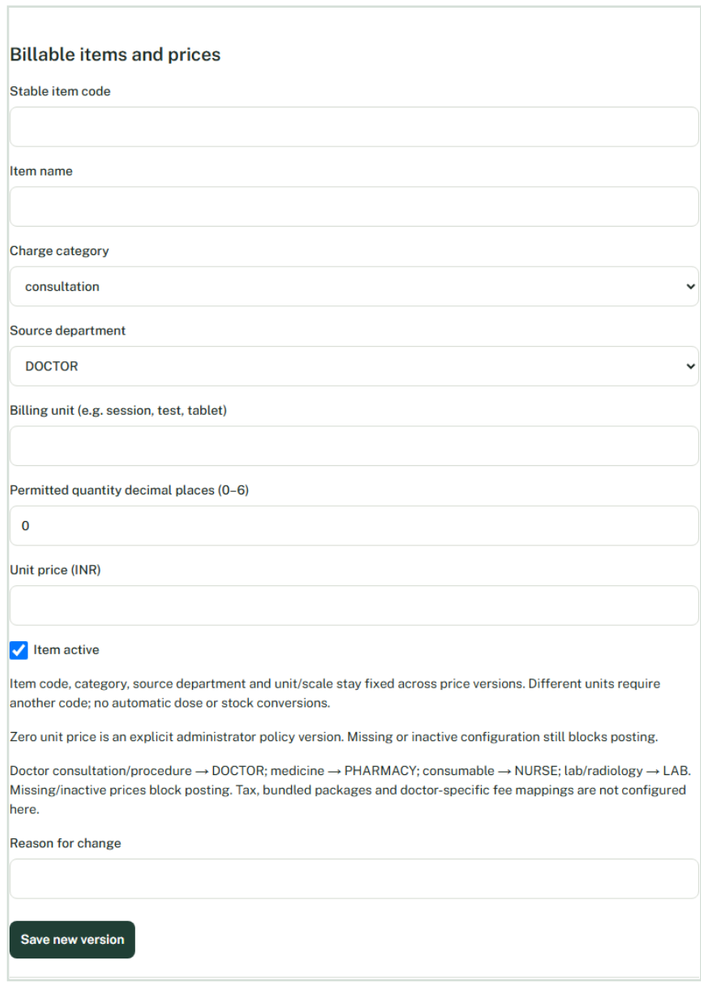
Staff & settings > Hospital settings & tariffs

1. Room tariffs: select category, amount, billing unit, rounding and grace period. The fictional General room is INR 1,200 per started 24 hours.
2. Billable items: configure stable code, name, charge category, source department, unit, quantity decimal places and price. Existing DEMO_MED is INR 100 per pack.
3. **Save new version** requires Reason for change. Review the intended effective policy carefully; do not edit training configuration merely to follow this guide.

Note: Configuration forms inspected without saving. Existing hospital configuration remains unchanged.



Current section / form reference



Current section / form reference

Deposits and alert thresholds

Staff & settings > Hospital settings & tariffs

1. Deposits: configure category, payment route, whether a deposit is required and fixed initial deposit. The Admission screen uses the configured rule to check activation.
2. Alerts: choose the amount/rule to alert against and Threshold (%). An alert is staff information, not an automatic insurer approval or payment.
3. Enter Reason for change and use **Save new version**. Previous versions remain in history; configuring a deposit does not record any actual money.

Note: Configuration forms inspected without saving. Existing hospital configuration remains unchanged.

Initial deposit rule

Category code

Payment route

Self-pay

Deposit requirement

Deposit required

Fixed initial deposit (INR; zero when not required)

Fixed amount rules only. Formula deposits need an approved daily reference rate before they can be configured.

Reason for change

Save new version

Current section / form reference

Staff follow-up alerts

Alert against

Approved insurer limit

Threshold (%)

Alerts request staff follow-up; they do not approve coverage or record payment.

Reason for change

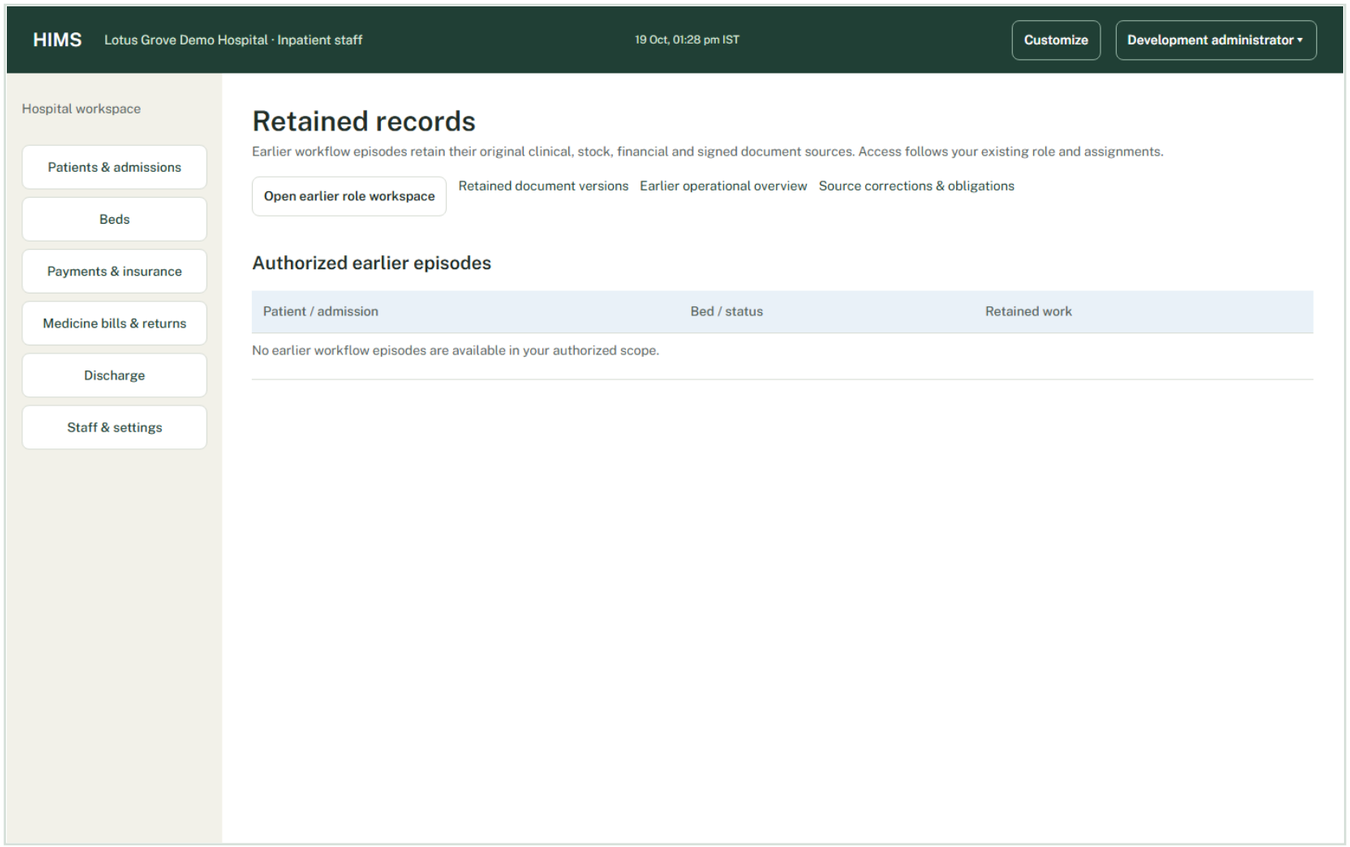
Save new version

Current section / form reference

Retained records - keep the original episode discoverable

Retained records / a section’s Retained MVP episodes filter

1. Retained records lists authorized earlier-workflow episodes and preserves their original sources. Open earlier role workspace, Retained document versions and Earlier operational overview open those retained views.
2. Source corrections & obligations opens attributable correction work. History is retained rather than overwritten; changed financial sources require linked adjustments and authority.
3. For recently closed MVP admissions such as Demo Rohan Desai, use Retained MVP episodes in Patients & admissions, Payments & insurance, Medicine bills & returns or Discharge. The earlier-workflow page can correctly be empty.
4. Later insurer cash settles the retained receivable without rewriting the original hospital exit or final paper bytes. The current retained MVP queues cover the latest 50 closed episodes.

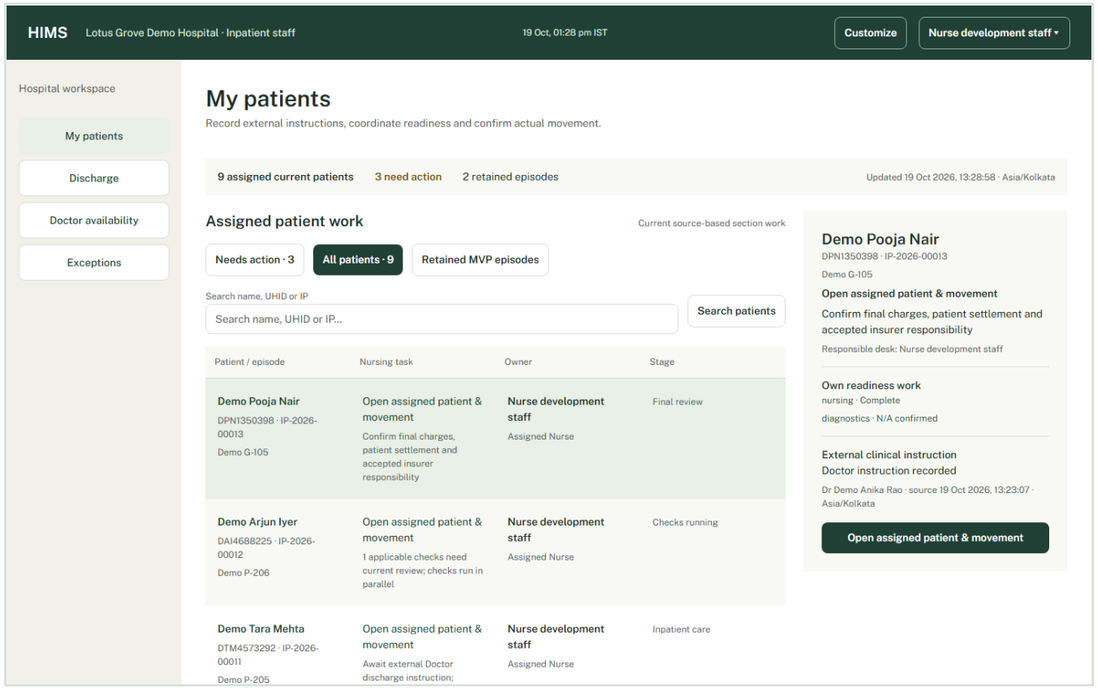


Current section / form reference | upper workspace; scroll for more

My patients - assigned care and the next desk

Sidebar > My patients (/nursing)

1. Use All patients for routine assigned care, Needs action for flagged Nursing work, and Retained MVP episodes for closed current-workflow stays. Enter a name, UHID or IP and select Search patients.
2. Select the patient name to change the summary; select Open assigned patient & movement, Review Nursing readiness & instructions or Deliver authorized final papers to open the named task.
3. In Readiness & checks, expand Start assigned care shift; enter Actual shift start and Incoming handover and care responsibility, then select **Start care shift**. Assignment must already exist.
4. Start discharge opens Readiness & checks; it does not itself approve or close a stay. Shift patient opens Bed transfer. **Record external transfer authority** takes Doctor identity, named authorizer, source time/evidence, reason and destination category; select **Record transfer authority**. Record actual transfer & handover then takes the matching available bed, care shift, move time and handover; select Record actual transfer.
5. **Sign handover and end care shift** records Actual shift end and Outgoing handover. It ends care attendance; the separate **Confirm actual hospital exit** records departure. After exit, the shift end cannot be later than the actual exit.



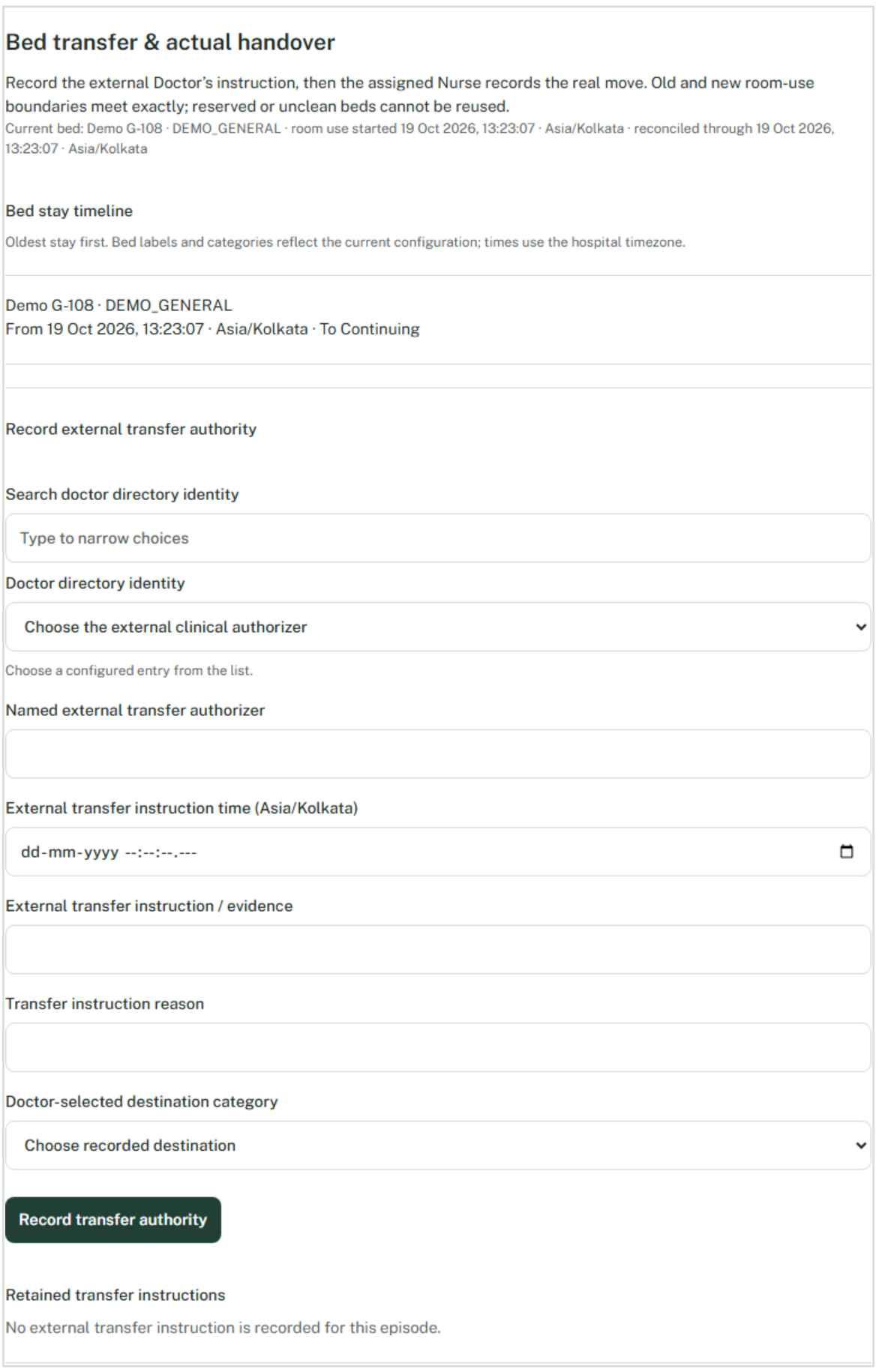
Current section / form reference | upper workspace; scroll for more

Bed transfer - authority before physical movement

My patients > patient > Shift patient / Bed transfer

1. Shift patient opens Bed transfer; it does not move the patient immediately.
2. Expand **Record external transfer authority**. Record the external Doctor, named authorizer, source time, evidence, reason and destination category, then **Record transfer authority**.
3. After a current authority exists, Record actual transfer & handover offers a matching available bed. Record actual transfer needs your care shift, actual movement time and destination handover.
4. Keep the move time exact: the previous bed stay ends and the next begins at that boundary. Read the retained transfer timeline.

Note: Transfer form inspected only; no transfer was submitted for this document. The screenshot shows Kabir, who remains in G-108.

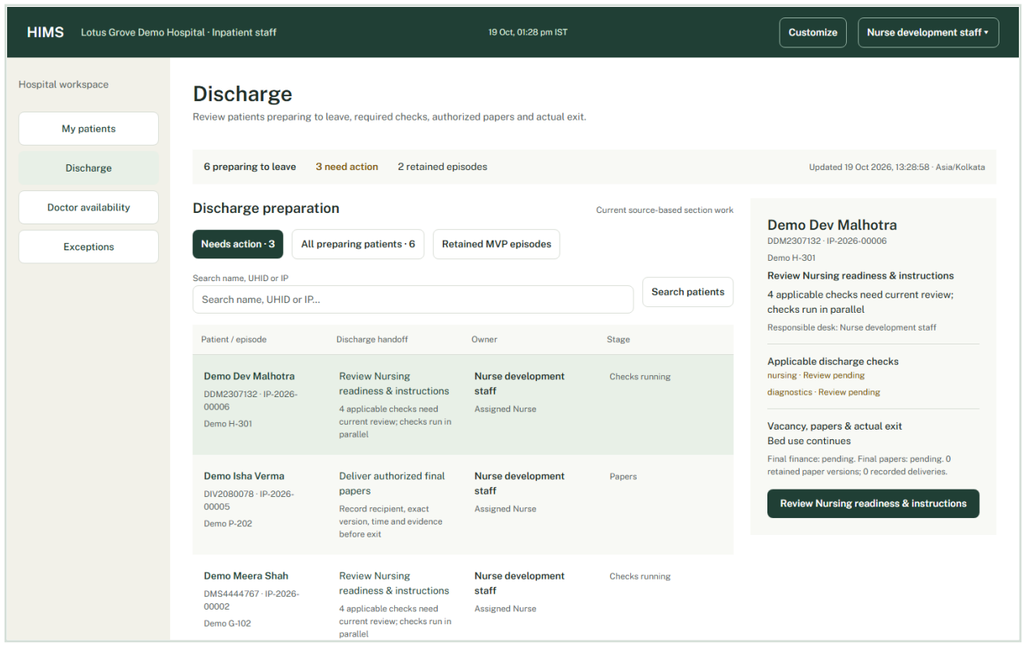


Current section / form reference

Discharge - Nursing checks, papers and actual exit

Sidebar > Discharge (/discharge); patient > Readiness & checks / Vacancy, papers & exit

1. Needs action shows work owned by Nursing; All preparing patients includes other desks' discharge blockers. Open the named action. A Pharmacy or financial blocker remains with that desk.
2. In Readiness & checks, Record external Doctor approval or Record updated Doctor instruction opens the source form. Record the named authorizer, source time, evidence, supplied summary, prescription or explicit none, and follow-up; select **Record Doctor approval**. The Nurse remains the separate recorder.
3. Publish physical patient checklist takes one readiness item per line and applicability remarks; select **Publish patient checklist**. Optional N/A proposals need Admin confirmation. Record your readiness check takes Responsible check, Check outcome and remarks; select **Record readiness check**. DIAGNOSTICS also needs the external confirmer, time and evidence.
4. In Vacancy, papers & exit, Record actual bed vacancy needs your care shift, actual vacancy time, waiting location and waiting-care owner/handover; select Record bed vacancy. Room use ends, while the bed stays held.
5. Generate an authorized paper version > Generate authorized paper creates a protected version. Deliver authorized patient papers > Record authorized paper delivery records exact version, recipient, actual time and acknowledgment. After final finance and required final-paper delivery, **Confirm actual hospital exit** records your shift and actual exit time; Admin then owns turnover.

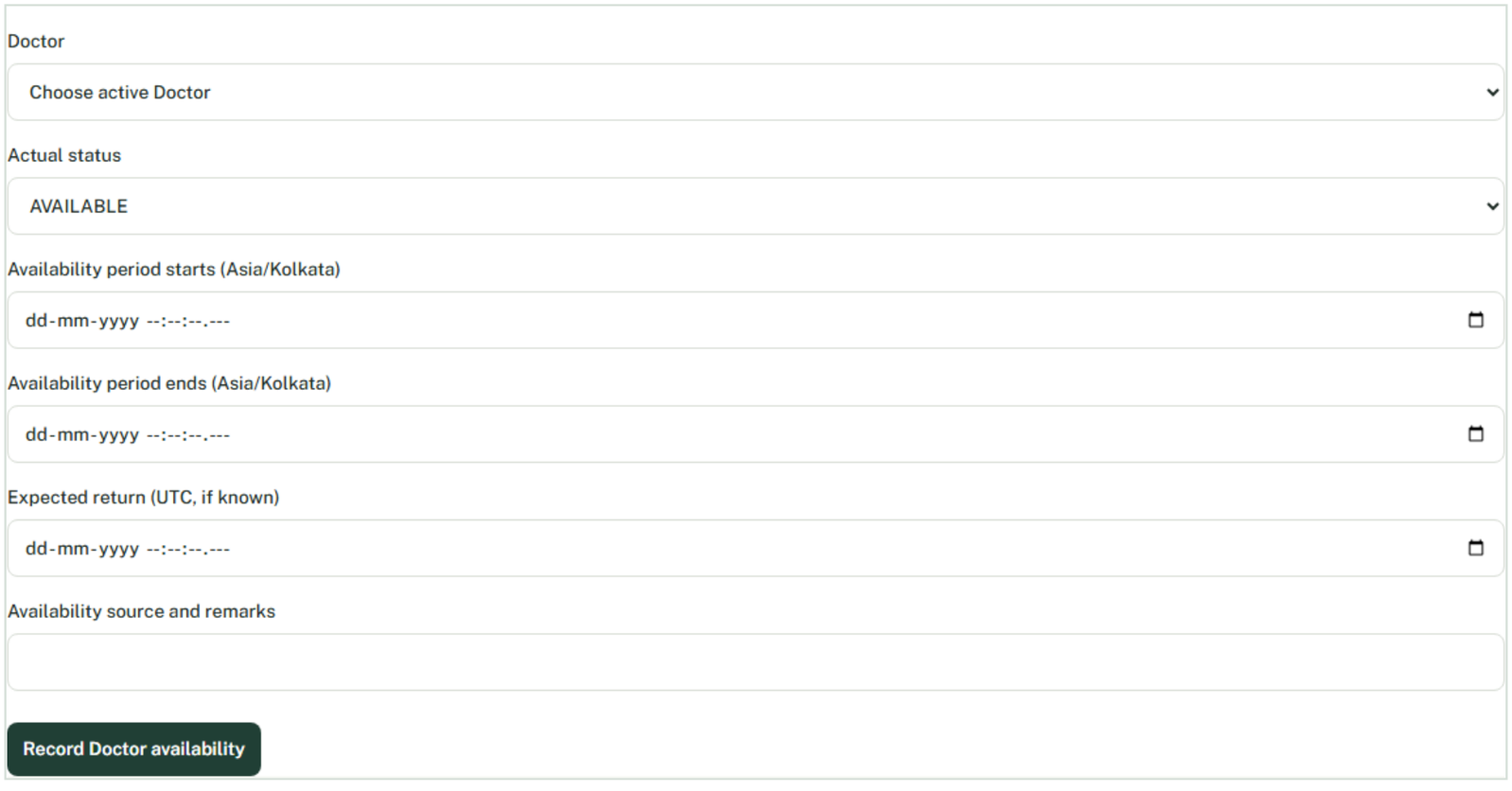


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Doctor availability - the designated shift register

Sidebar > Doctor availability (/availability)

1. Read the current shift owner, backup and responsibility period above the Doctor directory. Patient assignment and care attendance do not confer this separate register responsibility.
2. The current designated Nurse or backup uses Record actual availability: choose Doctor and Actual status - AVAILABLE, OT, LEAVE or OFF_DUTY.
3. Enter Availability period starts, Availability period ends, optional Expected return, and Availability source and remarks. Follow the timezone displayed beside each date field; select Record Doctor availability.
4. The table preserves named recorder and update time. A scheduled or expired entry is labelled as such; refresh before relying on a changed period.
5. Availability does not authorize discharge. Ask Admin to renew the availability responsibility period when it ends; the captured training assignment ends on 22 October 2026.

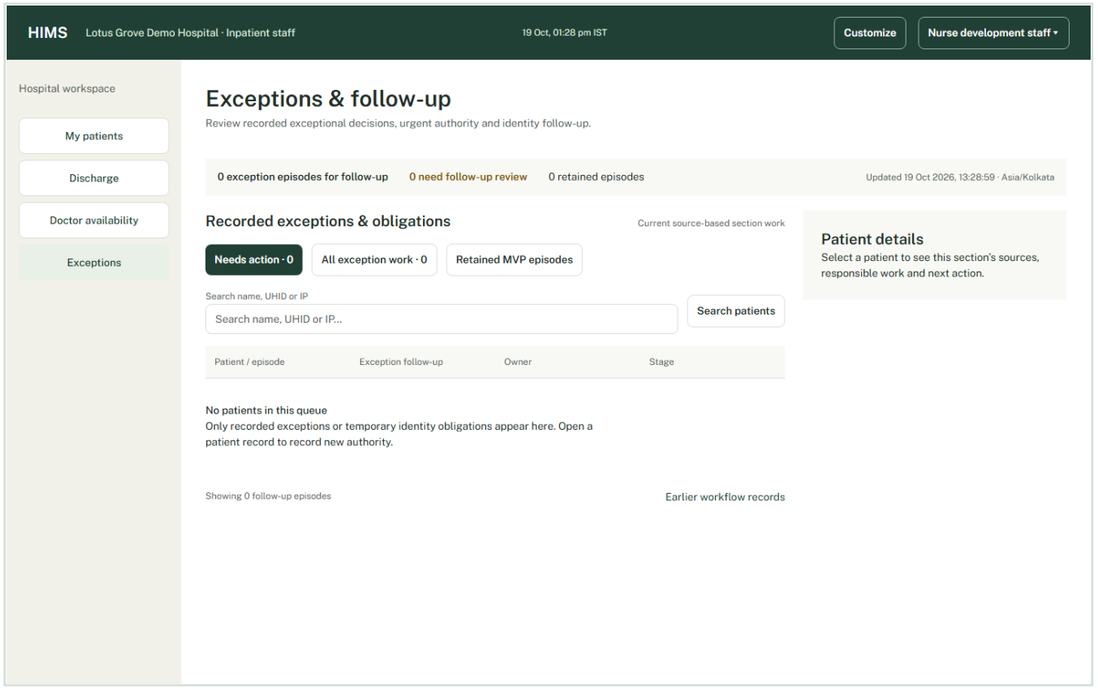


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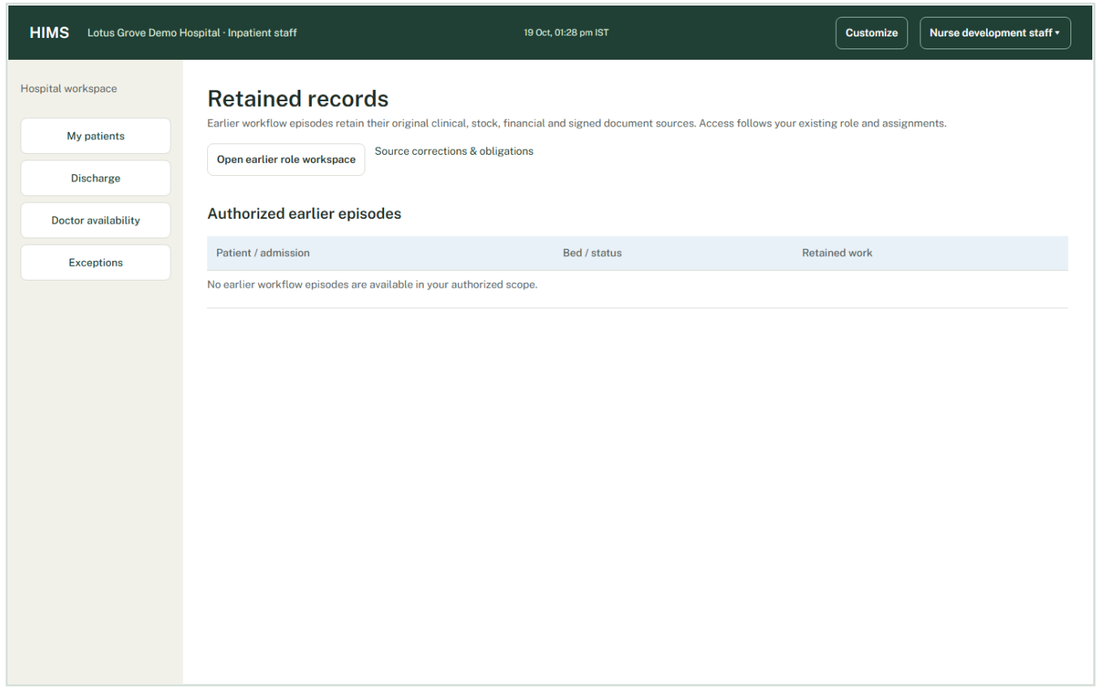
Exceptions and retained records - use the correct history

Sidebar > Exceptions (/exceptions) or Retained records (/history)

1. Exceptions shows relevant recorded obligations. Needs action, All exception work and Search patients filter this queue; an empty queue means no matching recorded exception, not permission to skip an ordinary check.
2. Within a patient, Exceptions & return > **Record external exceptional instruction** records the actual disposition, named external authorizer, source time, evidence and circumstances. Select **Record exceptional clinical instruction**; accountable Admin authority is a separate action.
3. Conditional forms cover exceptional factual vacancy, exact-paper delivery, exceptional hospital exit and authorized return to bed. Use only the recorded real situation; obligations and unpaid amounts remain. A return needs the original vacancy/instruction, Admin authority, completed turnover, your shift, return time and handover.
4. Retained records opens the earlier-workflow archive. Open earlier role workspace and Source corrections & obligations lead to its retained sources; no earlier records are visible in this captured authorized scope.
5. For completed current-workflow patients such as Rohan, use Retained MVP episodes from My patients or Discharge. This is separate from the earlier-workflow archive; current queues show the latest 50 closed episodes.



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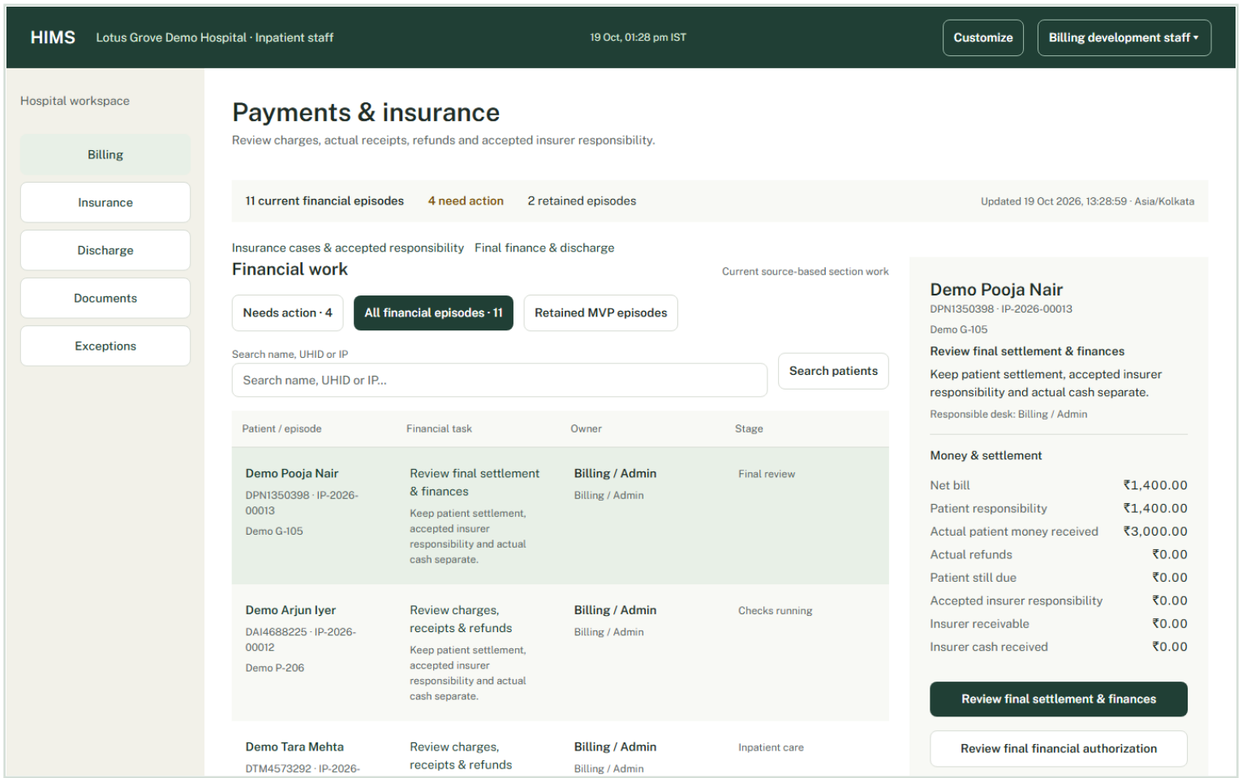


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Billing - the financial queue

Sidebar > Billing (/billing); page heading Payments & insurance

1. Use All financial episodes for everyday charges and receipts; Needs action narrows the flagged queue. Search name, UHID or IP, then select Search patients. Retained MVP episodes opens completed current-workflow stays.
2. Selecting a name changes the financial summary. Review charges, receipts & refunds opens Money & insurance; Review final settlement & finances opens that patient's settlement work.
3. Review final financial authorization opens Vacancy, papers & exit. Insurance cases & accepted responsibility opens the Insurance section; Final finance & discharge opens the Discharge queue.
4. Read patient responsibility, actual received money, actual refunds, patient still due, accepted insurer responsibility, insurer receivable and insurer cash separately.
5. Pooja's captured example has a frozen ₹1,400 net bill, ₹3,000 received and no actual refund yet. ₹1,400 is already allocated; the next settlement step is the ₹1,600 actual refund, followed by final review. Do not enter the original deposit again.

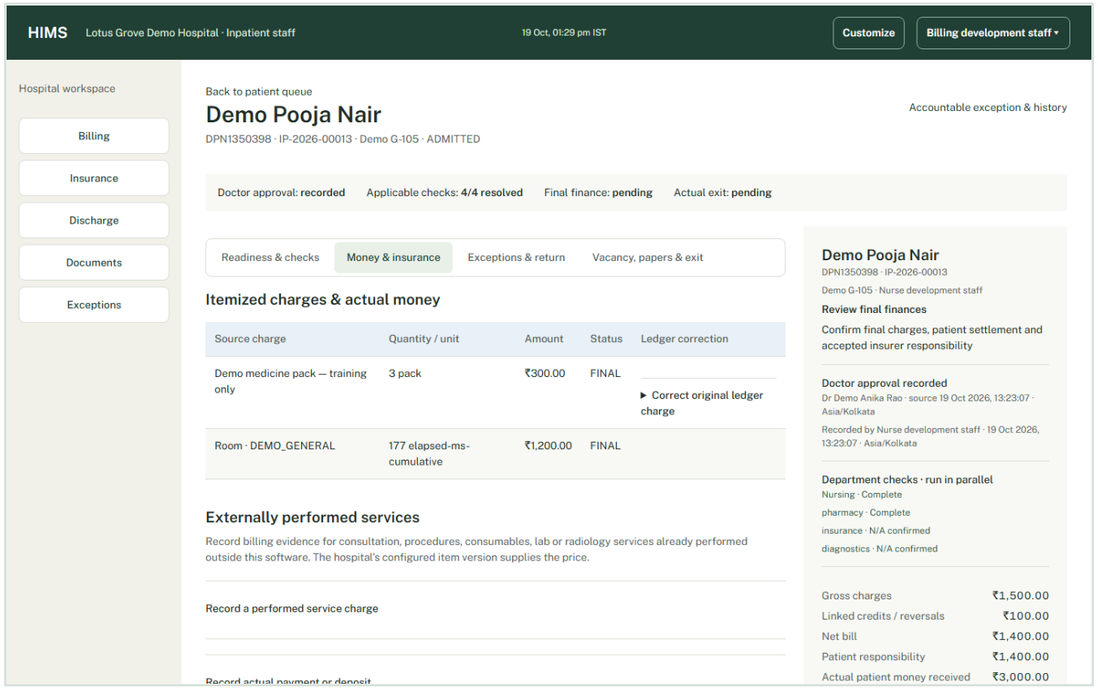


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Money & insurance - charges, receipts and corrections

Billing > patient action > Money & insurance

1. Record a performed service charge records a configured service, actual quantity/time, performing issuer, external bill and unique line references, and evidence; select **Record performed service charge**. Configured prices determine money. Reconcile current room charges > Post room charges through captured cutoff applies while bed use continues.
2. Record actual payment or deposit takes Receipt party, Receipt kind, amount, mode, cash book/channel, transaction identity and staff reference; select **Record manual receipt**. Record only money actually received through hospital channels; reuse the same transaction identity on retries.
3. Expand Allocate original deposit · [receipt reference], enter Allocate to bill (INR) and Allocation reason, then select Allocate deposit. This uses the existing receipt; it does not collect new money.
4. Record an actual linked refund needs the original receipt, actual refund amount, mode, channel, transaction reference, evidence, actual time and reason; select **Record actual refund**. Pharmacy's accepted return credit reduces a charge but does not prove a cash refund.
5. Correct original ledger charge > Record linked reversal retains the original non-room charge with a reason. Correct an original deposit allocation > Cancel deposit allocation restores held deposit cash. Neither action creates a new payment or refund.

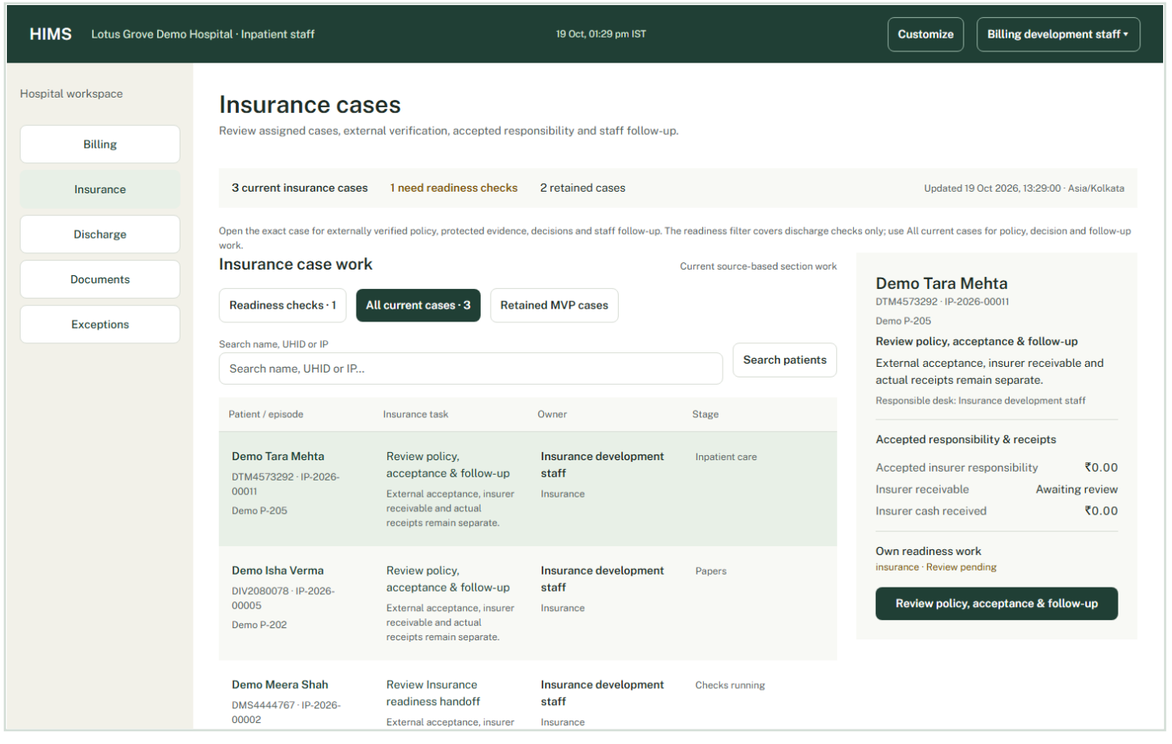


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Insurance - responsibility and the exact case

Sidebar > Insurance (/insurance)

1. All current cases includes routine policy, payer-decision and follow-up work. Readiness checks narrows discharge checks only; it is not the full insurance workload. Self-pay episodes without cases are excluded.
2. Use Search patients or select the patient's name to inspect accepted responsibility, receivable, actual insurer cash and readiness. Select Review policy, acceptance & follow-up to open that exact case.
3. Manage verified cases, assignments & protected evidence opens the case-management area. Policy details, payer decisions and evidence come from external verification; the app does not contact or approve an insurer.
4. Review Insurance readiness handoff opens the exact case for that handoff. Insurance staff records its own separate readiness check; Billing owns settlement and final finance.
5. Accepted insurer responsibility may remain receivable after an ordinary exit once patient settlement and required checks are satisfied. An acceptance is not an actual cash receipt.

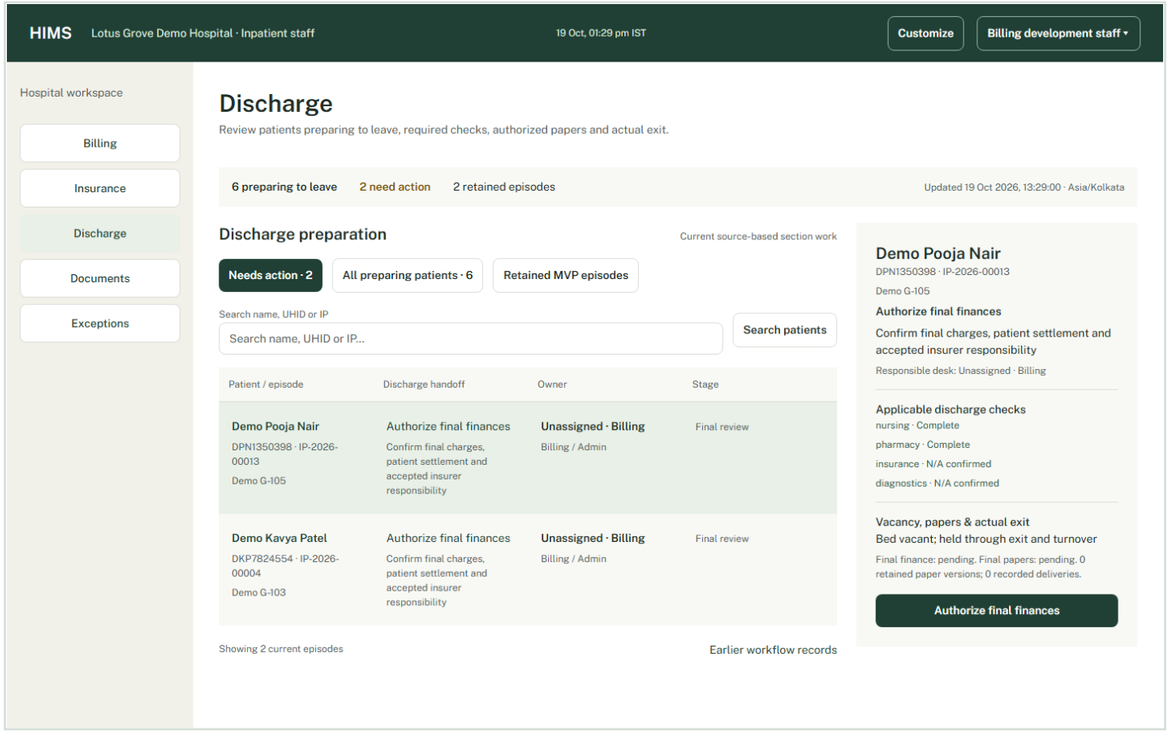


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Discharge - final financial authorization

Sidebar > Discharge (/discharge); patient > Vacancy, papers & exit

1. Needs action shows financial discharge handoffs; All preparing patients includes other discharge stages. Select **Authorize final finances** to open the patient's final-review tab.
2. Review the applicable department checks, actual vacancy, charges, linked credits, receipts, deposit allocations, actual refunds and payer responsibility. The form lists current blockers; a visible button does not override them.
3. Enter Final financial review and acceptance reason, then select **Authorize final finances**. The server rereads current sources and rejects unresolved checks or settlement.
4. Pooja is still pending in the captured screen because excess cash needs disposition. Record the remaining ₹1,600 actual refund first, then review current totals; no final authorization is claimed for her.
5. After finance, Nurse/Admin generates and delivers the exact authorized final papers; the assigned Nurse records actual exit. Admin then records cleaning/turnover. Accepted insurer credit can remain receivable; it is not fabricated cash.

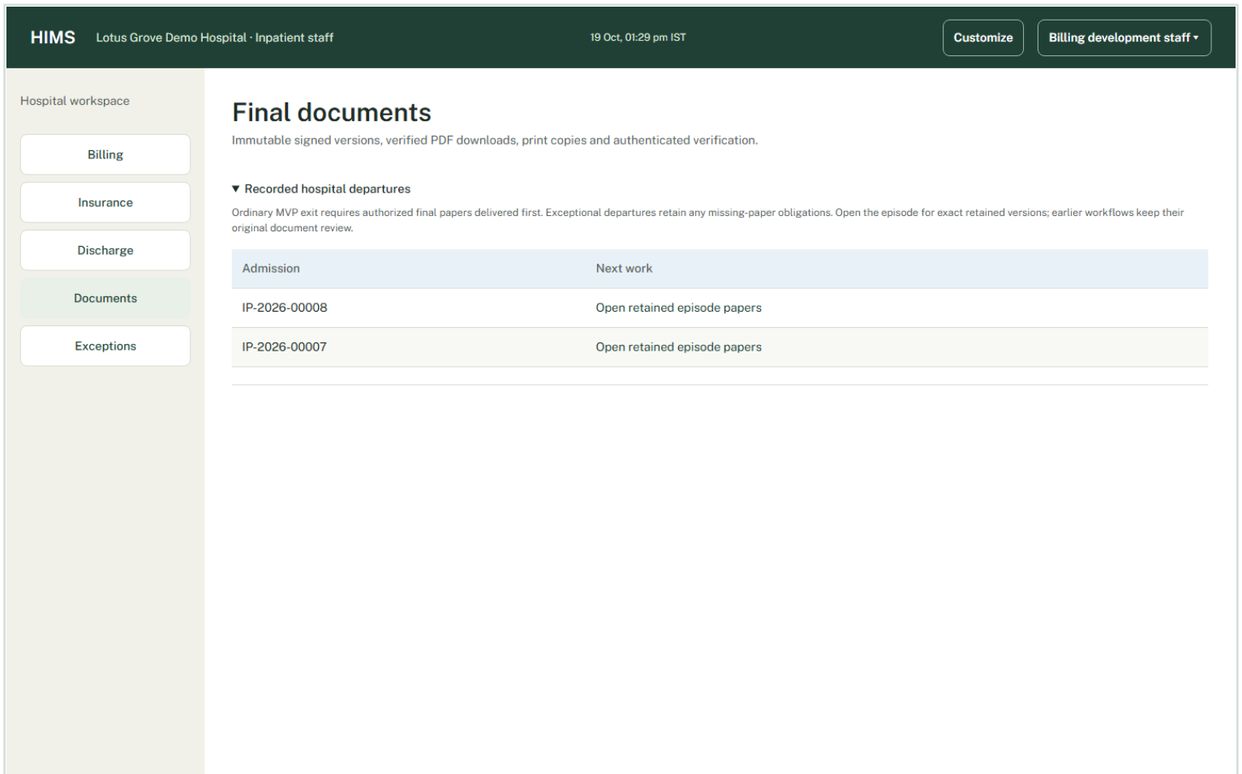


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Documents - retrieve the exact retained paper

Sidebar > Documents (/documents)

1. Recorded hospital departures lists admission IP codes. Select Open retained episode papers for a current MVP episode; it opens the patient's Vacancy, papers & exit tab.
2. Under Available protected paper versions, select CLINICAL · version [number] or FINAL · version [number] to open that exact immutable version. Retained delivery records show the recipient, time and linked document.
3. On the document, Print / save exact paper opens the print copy; Download verified PDF downloads the retained verified file; Verify version and integrity opens authenticated verification.
4. Back to paper delivery & exit returns to the episode. Billing reviews and retrieves papers; Nurse/Admin owns generation and actual delivery in the current MVP.
5. Later receipts settle a receivable without rewriting historical exit or paper bytes. Rohan's later ₹2,000 insurer receipt left the final PDF unchanged; the journey shows that completed example.

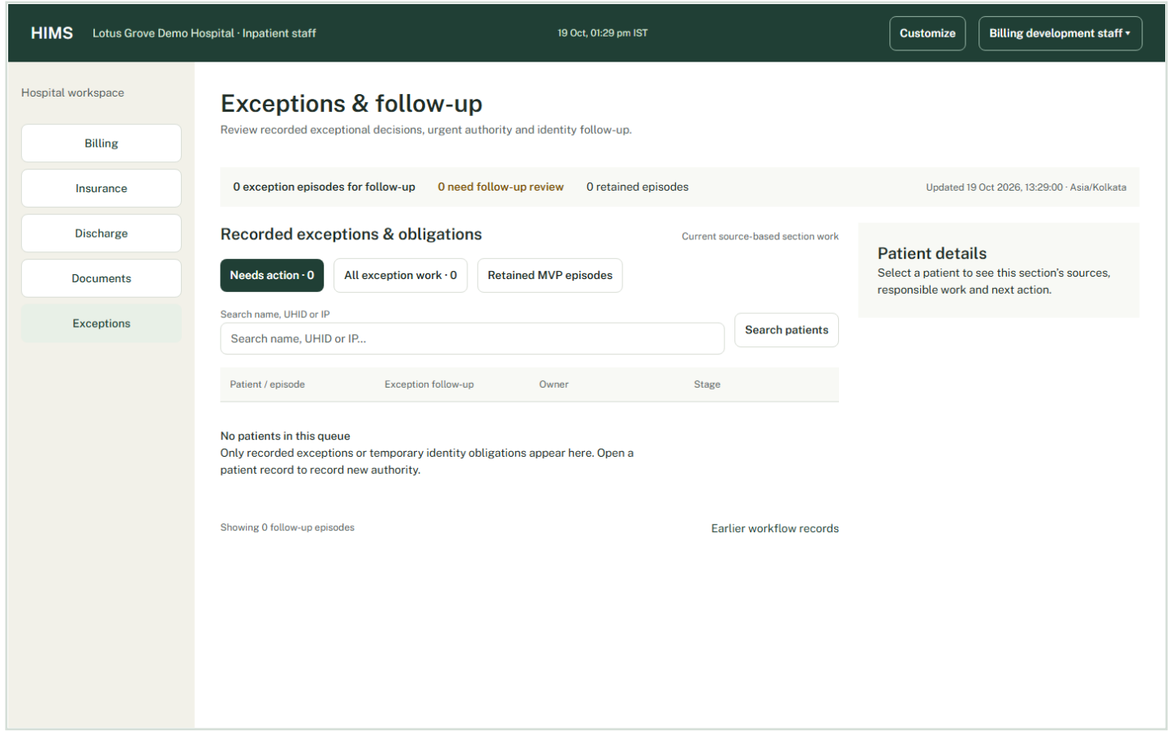


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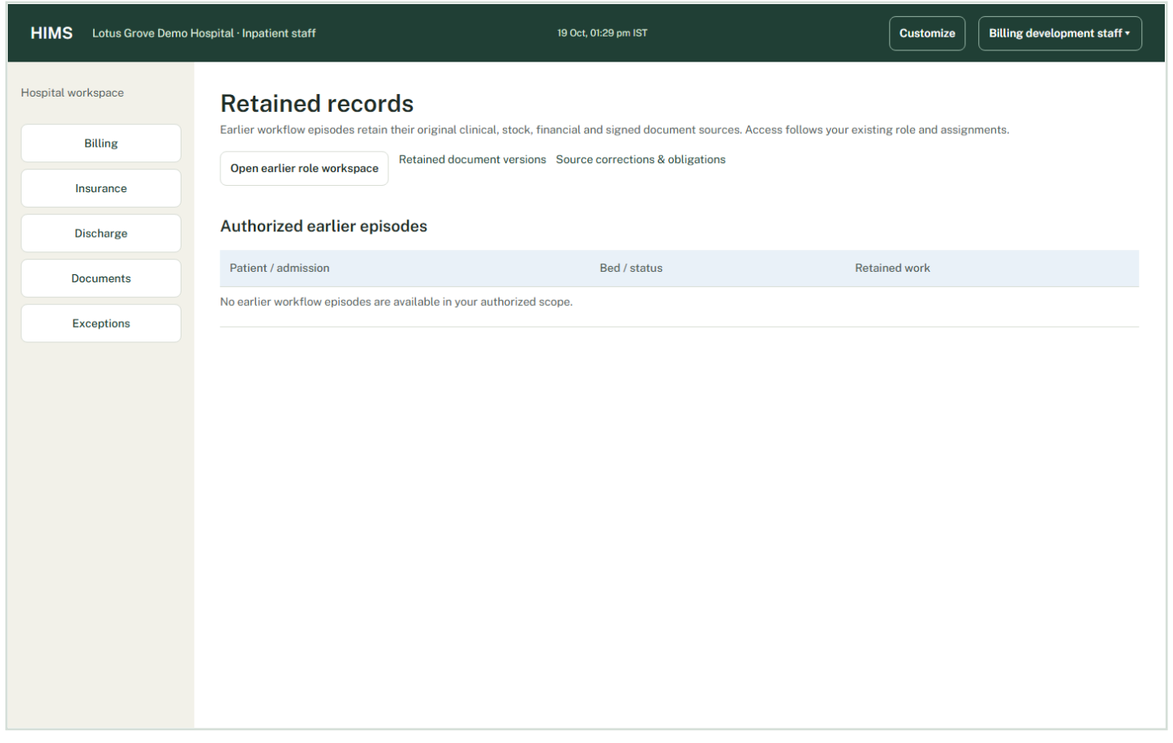
Exceptions, retained records and later receipts

Sidebar > Exceptions (/exceptions), Retained records (/history); Billing > Retained MVP episodes

1. Exceptions filters recorded financial/exceptional obligations with Needs action, All exception work and Search patients. The captured empty queue contains no recorded matching exception work.
2. Retained records is the earlier-workflow archive. Open earlier role workspace, Retained document versions and Source corrections & obligations lead to those retained sources; the captured authorized table is empty.
3. Use Retained MVP episodes on Billing for completed current-workflow stays such as Rohan. Open that patient's Money & insurance tab for receivables and original financial history.
4. Record actual insurer receipt against receivable takes the actual amount, mode, bank/channel, transaction/book reference, receipt record reference and evidence; select Record insurer receivable receipt. It settles existing accepted responsibility without reopening exit or rewriting papers.
5. For post-exit financial corrections, Admin first authorizes the exact original source. The permitted desk then uses the selected correction form and Record linked financial amendment with evidence and reason. Original sources and obligations remain; do not replace an old receipt or invent a new approval.



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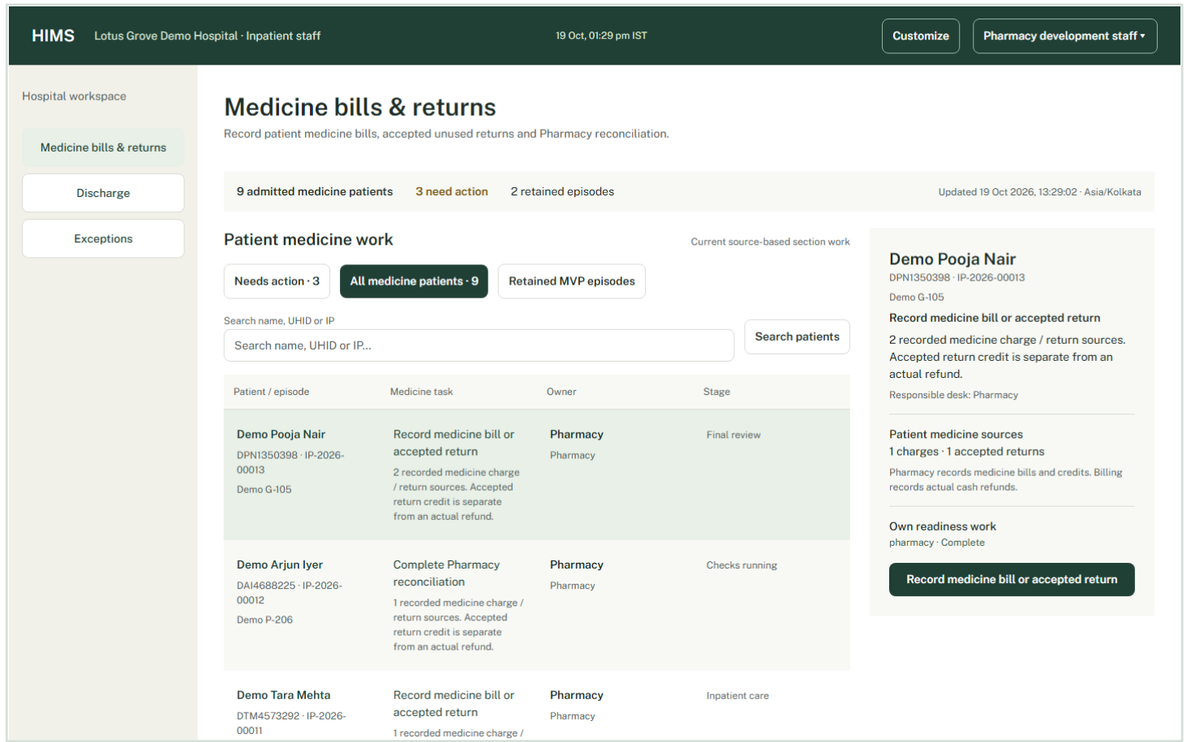


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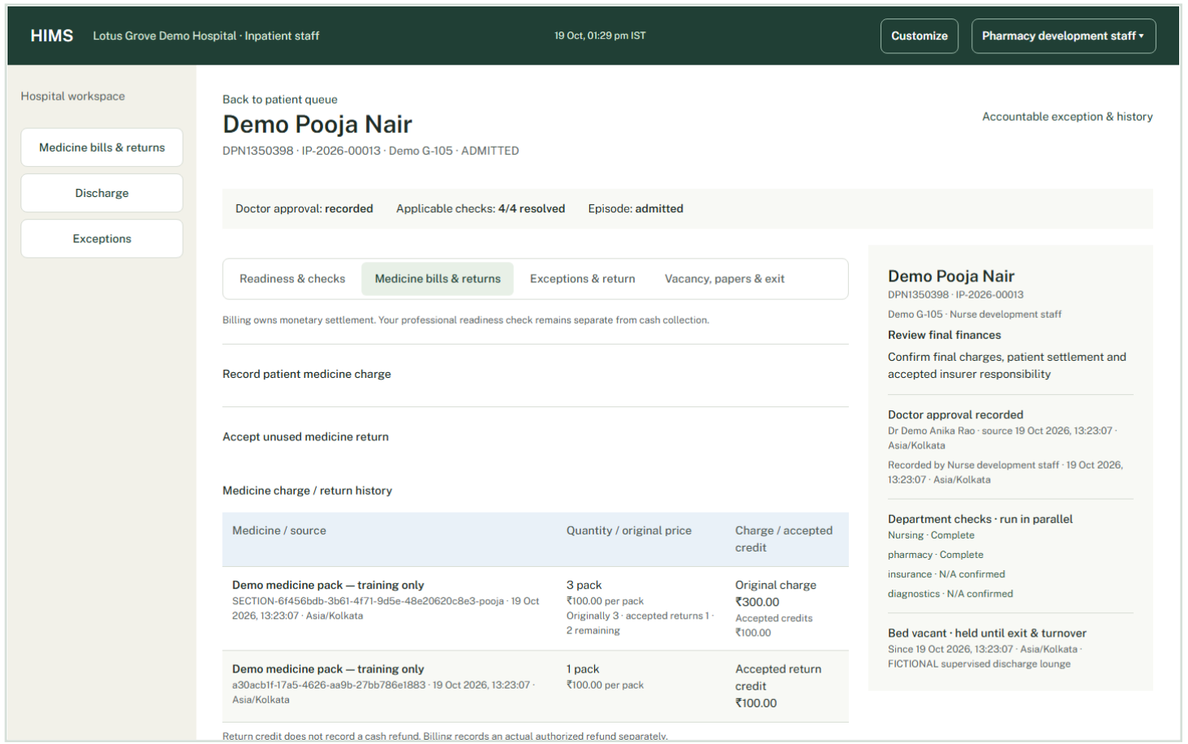
Pharmacy - Medicine bills & returns

Sidebar > Medicine bills & returns · /pharmacy

1. All medicine patients includes ordinary admitted patients. Needs action narrows to current Pharmacy readiness work; Search patients accepts name, UHID or IP.
2. Select a patient to inspect the summary; click Record medicine bill or accepted return to open Medicine bills & returns.
3. Record patient medicine charge > select configured medicine/price, quantity, original reference and evidence > **Record medicine charge**.
4. Accept unused medicine return > select original charge, quantity, reason and acceptance evidence > **Accept medicine return**. History retains original price, returns and remaining quantity.
5. Accepted returns reduce the bill. Billing records actual refunds; this section does not manage stock or prescribe medicines.



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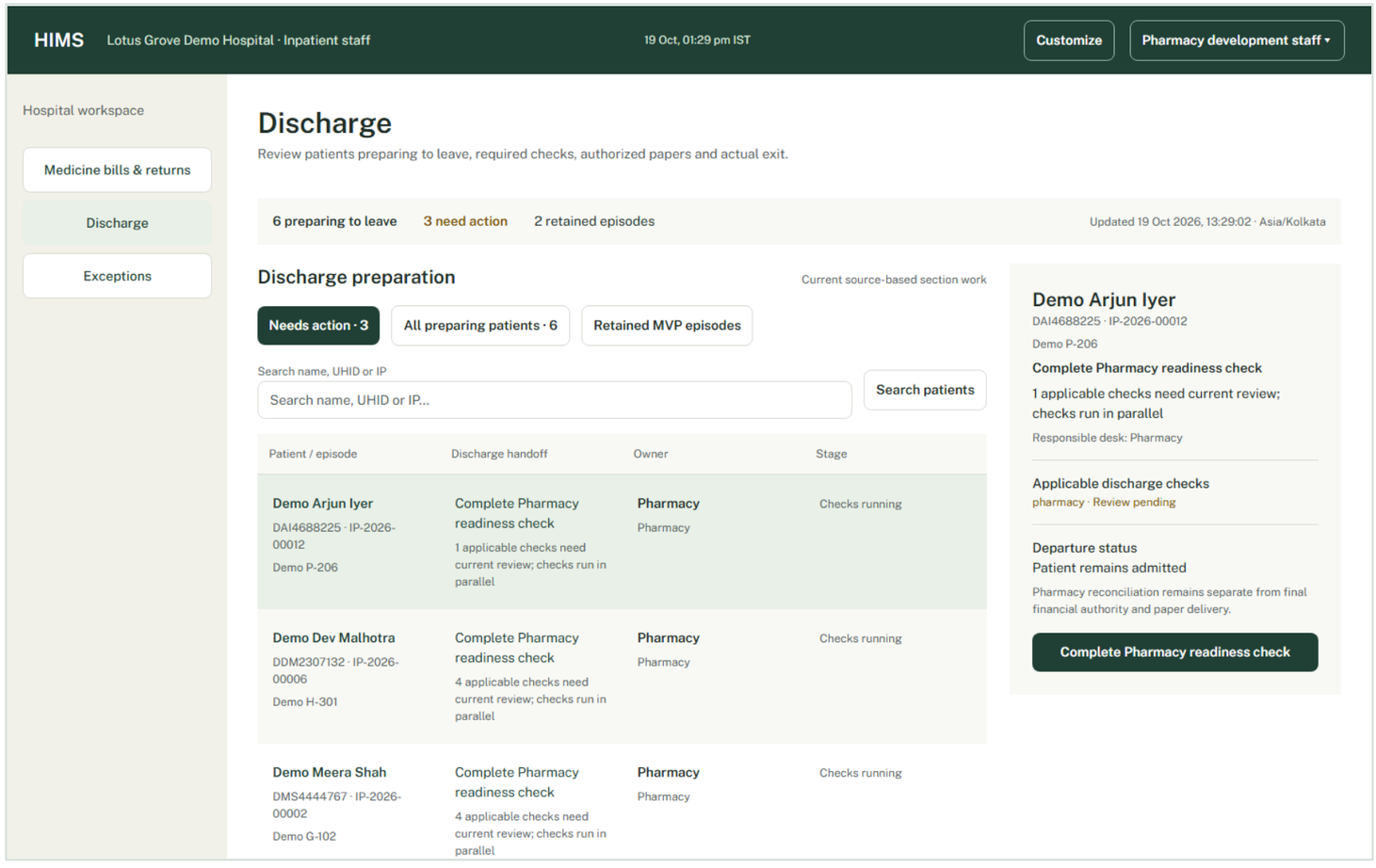


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Pharmacy - Discharge reconciliation

Sidebar > Discharge · /discharge

1. Needs action lists Pharmacy checks awaiting review. All preparing patients shows the wider discharge queue; Retained MVP episodes shows completed episodes.
2. Search patients by name, UHID or IP. Select a patient, inspect Applicable discharge checks and Departure status, then click Complete Pharmacy readiness check.
3. In Readiness & checks, expand Record your readiness check. Select PHARMACY, choose Complete or Pending / further work, and enter Review outcome and remarks.
4. Click **Record readiness check**. Demo Arjun Iyer is the prepared example still awaiting Pharmacy reconciliation.
5. Pharmacy's check is separate from final finance and paper delivery. Pharmacy cannot open the final discharge PDF.



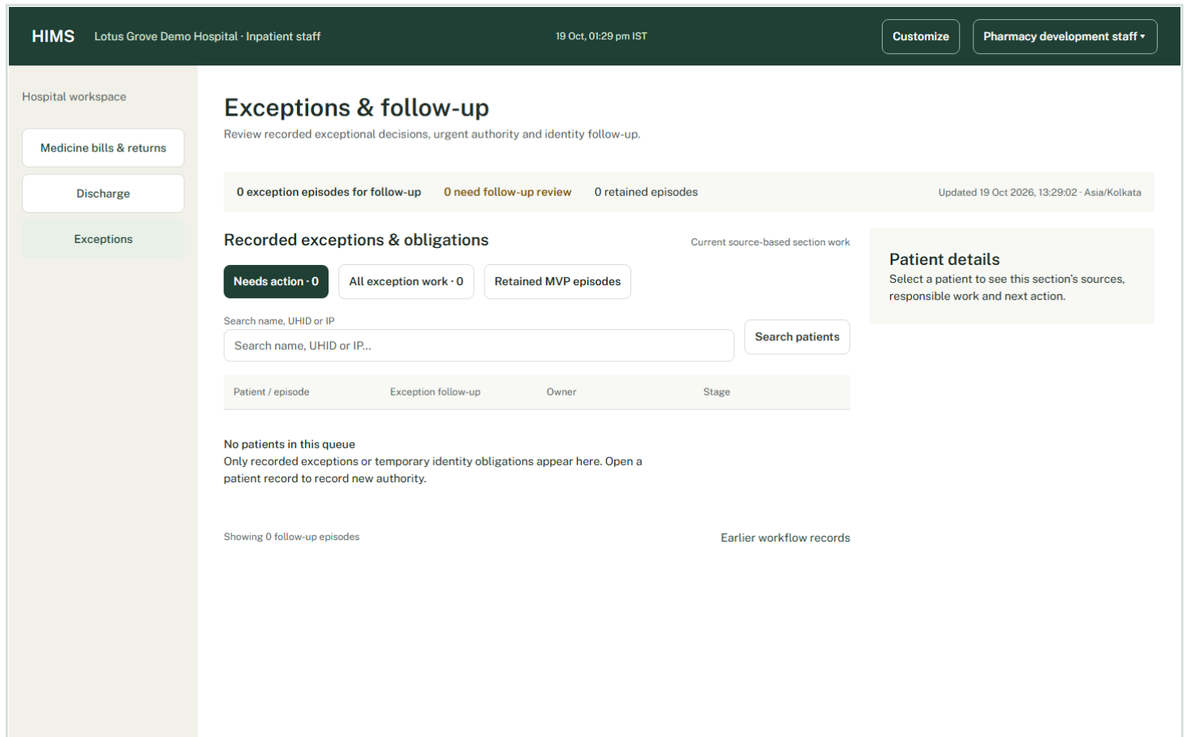
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Pharmacy - Exceptions and retained records

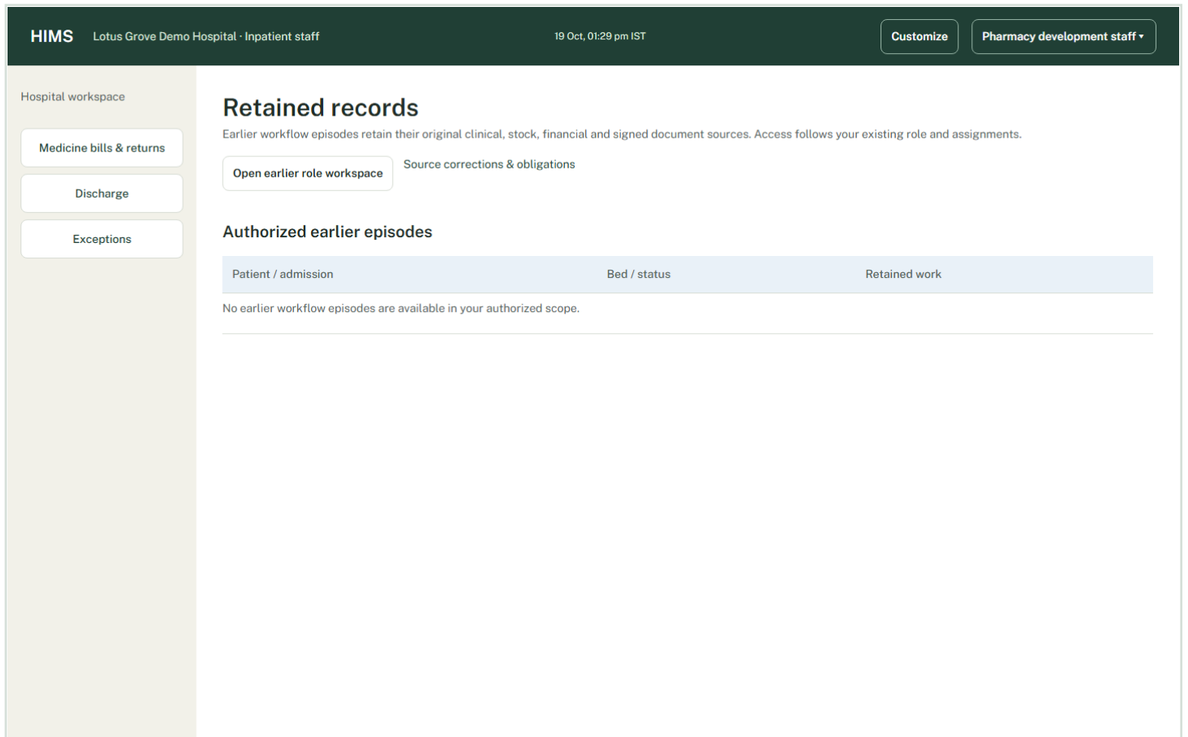
Sidebar > Exceptions · /exceptions; Retained records · /history

1. Exceptions offers Needs action, All exception work, Retained MVP episodes and Search patients. The verified Pharmacy queue is empty; no unrelated clinical or financial exception work is copied here.
2. Select an available patient to inspect its responsible action. Admin authority cannot supply Pharmacy approval or remove unpaid amounts.
3. Retained records opens Authorized earlier episodes. Open earlier role workspace leads to retained Pharmacy workflow; Source corrections & obligations returns to Exceptions.
4. For completed current-MVP medicine work, use Retained MVP episodes within Medicine bills & returns. Retained records is the separate earlier-workflow route.
5. Historical access still follows the staff role and assignment. Original medicine sources remain traceable; final discharge PDF access remains restricted.

Note: Navigation and empty states were verified. No Pharmacy exception mutation was exercised; current MVP source deliberately exposes no generic Pharmacy exception queue.



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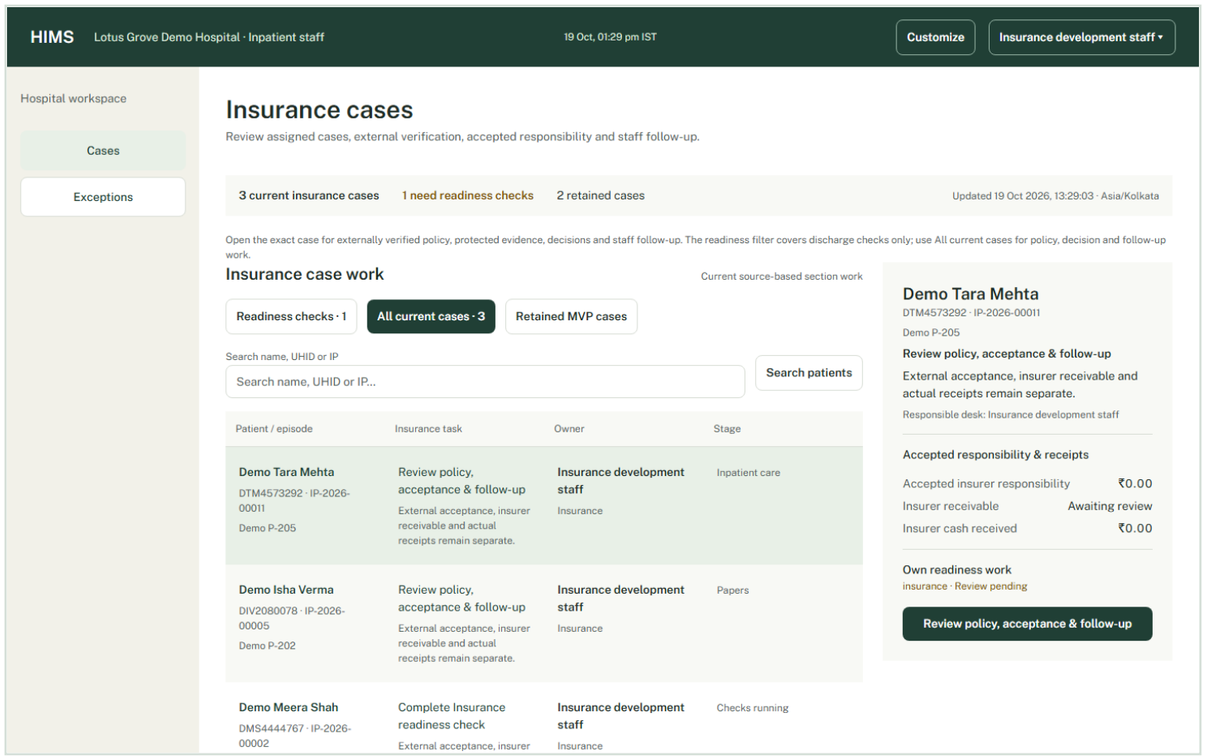
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Insurance - Cases, exact case and staff follow-up

Sidebar > Cases · /insurance; selected case > Case

1. All current cases covers assigned policy/decision/follow-up work. Readiness checks filters discharge checks only; Retained MVP cases shows closed cases. Use Search patients to find the exact admission.
2. Review policy, acceptance & follow-up opens that case. Tabs: Case, Decision, Documents and Paperwork; Choose another case switches patient.
3. **Open current policy and decision** opens Decision. Review Insurance discharge check opens the separate patient readiness form after Doctor instruction/checklist.
4. In Case, expand **Record manual pre-auth, enhancement or appeal work**. Choose Request type, enter Follow-up reference and Staff next action; click **Record staff follow-up**.
5. For open follow-up work, enter outcome reference/outcome and click Record follow-up completed. Existing email/portal contact remains staff work outside the app.

Note: Case navigation was verified. Follow-up controls are source-verified instructions; this guide did not submit new follow-up work. Transfer-specific follow-up appears only when a pending transfer task exists.



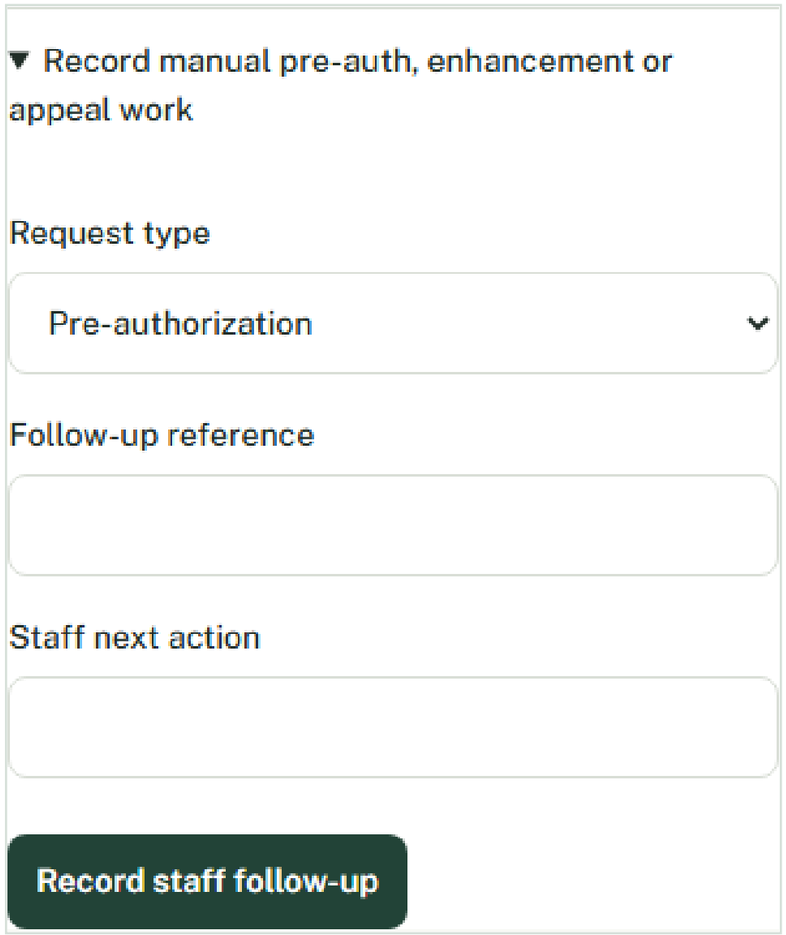
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Inside a case - manual work and external follow-up

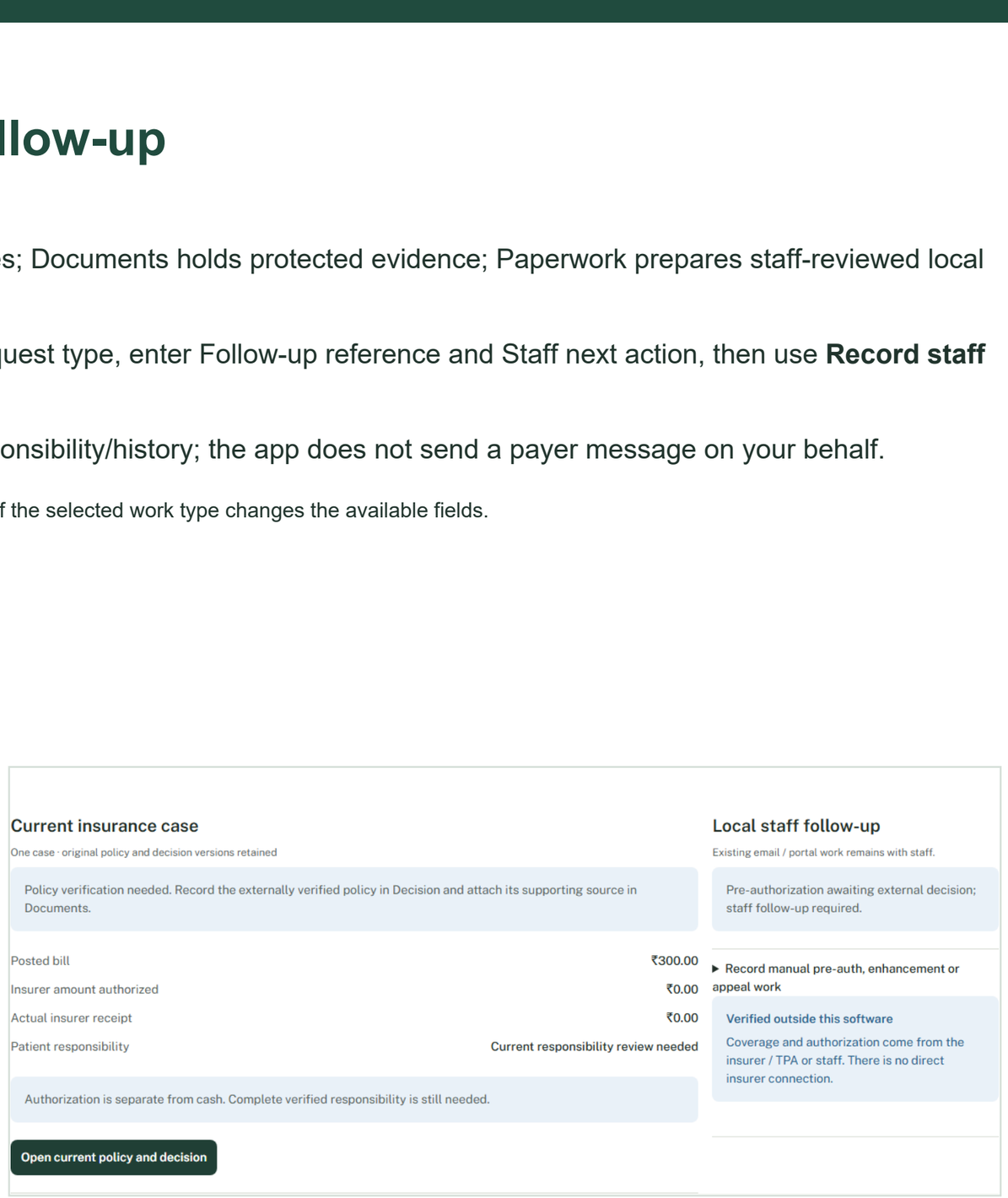
Cases > exact assigned patient > Case

1. Case is the coordination tab. Decision records verified policy and payer outcomes; Documents holds protected evidence; Paperwork prepares staff-reviewed local drafts.
2. Expand **Record manual pre-auth, enhancement or appeal work**. Choose Request type, enter Follow-up reference and Staff next action, then use **Record staff follow-up**.
3. Use the follow-up controls to record staff work and outcome. They preserve responsibility/history; the app does not send a payer message on your behalf.

Note: Case/follow-up forms were inspected without submitting new work. Refer to the exact visible save label if the selected work type changes the available fields.



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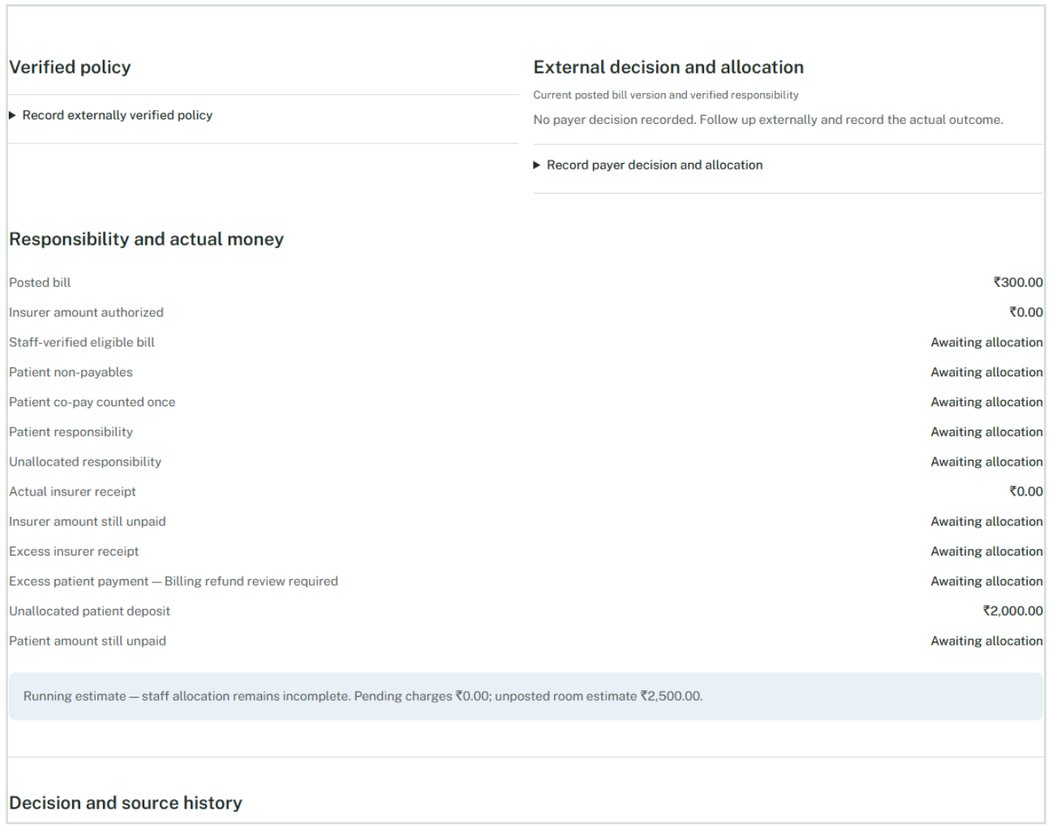


Current section / form reference

Insurance - Decision, policy and responsibility

Selected case > Decision

1. **Record externally verified policy**: insurer, optional TPA, policy/member numbers, validity, verifier, time, external reference and supporting evidence > **Save verified policy**.
2. **Record payer decision and allocation**: request type, external status, verifier/time/reference, exclusions, authorized amount, eligible bill, non-payables, co-pay, patient liability and allocation reference > **Save external decision**.
3. External status choices are Pending, Approved, Partial and Rejected. Request types are Pre-authorization, Enhancement and Appeal.
4. Responsibility and actual money keeps authorized credit, patient liability, unpaid insurer amount, actual receipts and excess/refunds separate. Renew a stale allocation after bill/policy changes.
5. Expand Previous decision history and Retained policy, assignment and follow-up events to inspect earlier actors, versions and references. Counts vary by case.



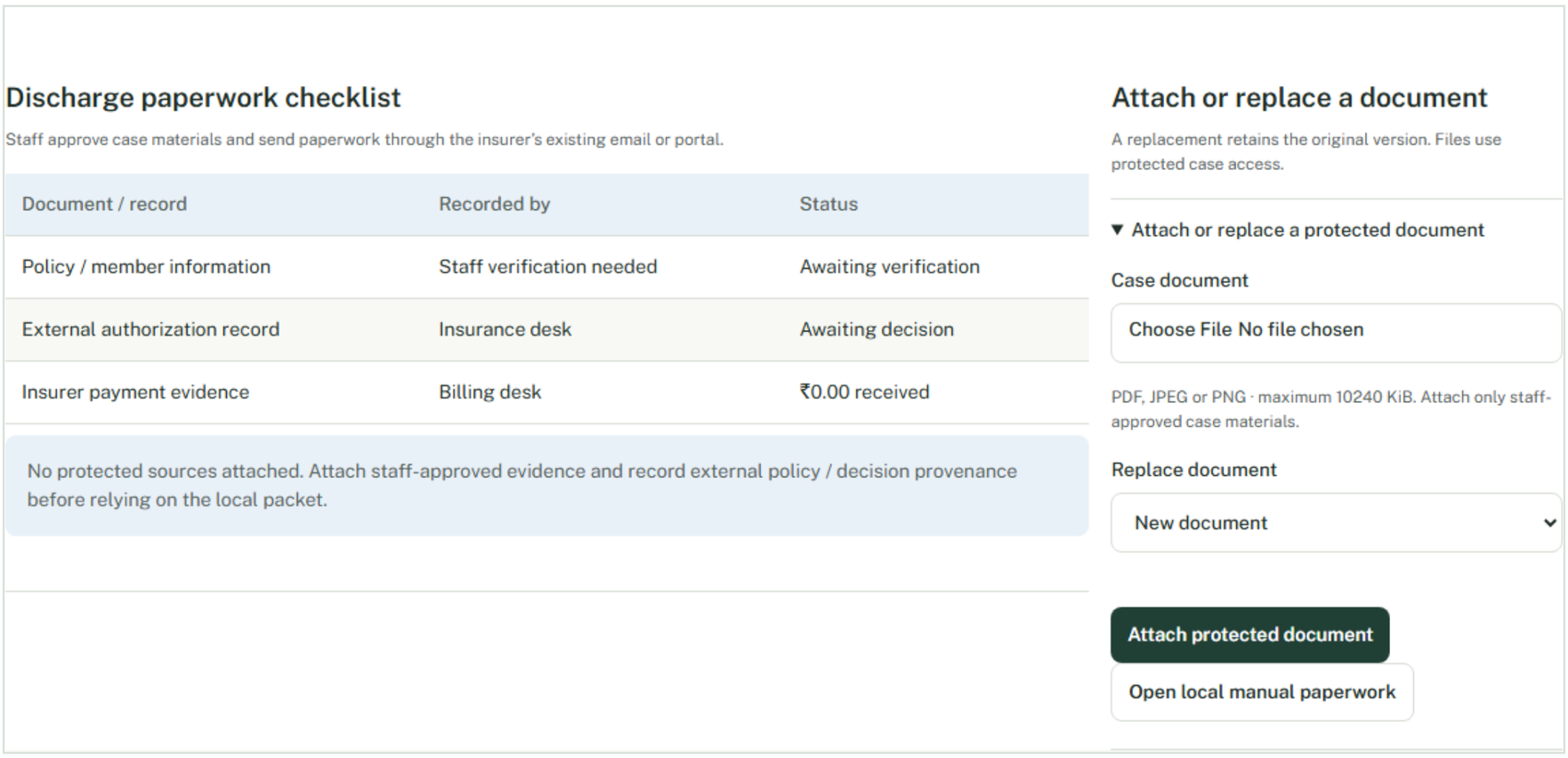
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Insurance - Documents and protected evidence

Selected case > Documents

1. Discharge paperwork checklist shows staff policy verification, external authorization, insurer payment evidence and attached protected files.
2. In **Attach or replace a protected document**, choose a staff-approved PDF, JPEG or PNG using Case document. Follow the maximum size displayed by the app.
3. Choose New document or the existing item under Replace document, then click **Attach protected document**. A replacement retains the original version.
4. Click an authorized document filename to open it. File integrity and retained source shows MIME type, byte size and SHA-256; original replaced sources remain listed.
5. **Open local manual paperwork** moves to Paperwork. Attachments alone do not verify payer completeness, approve coverage or send a packet.

Note: Controls are verified from current source. This guide did not upload a new file, replace evidence or contact any payer.



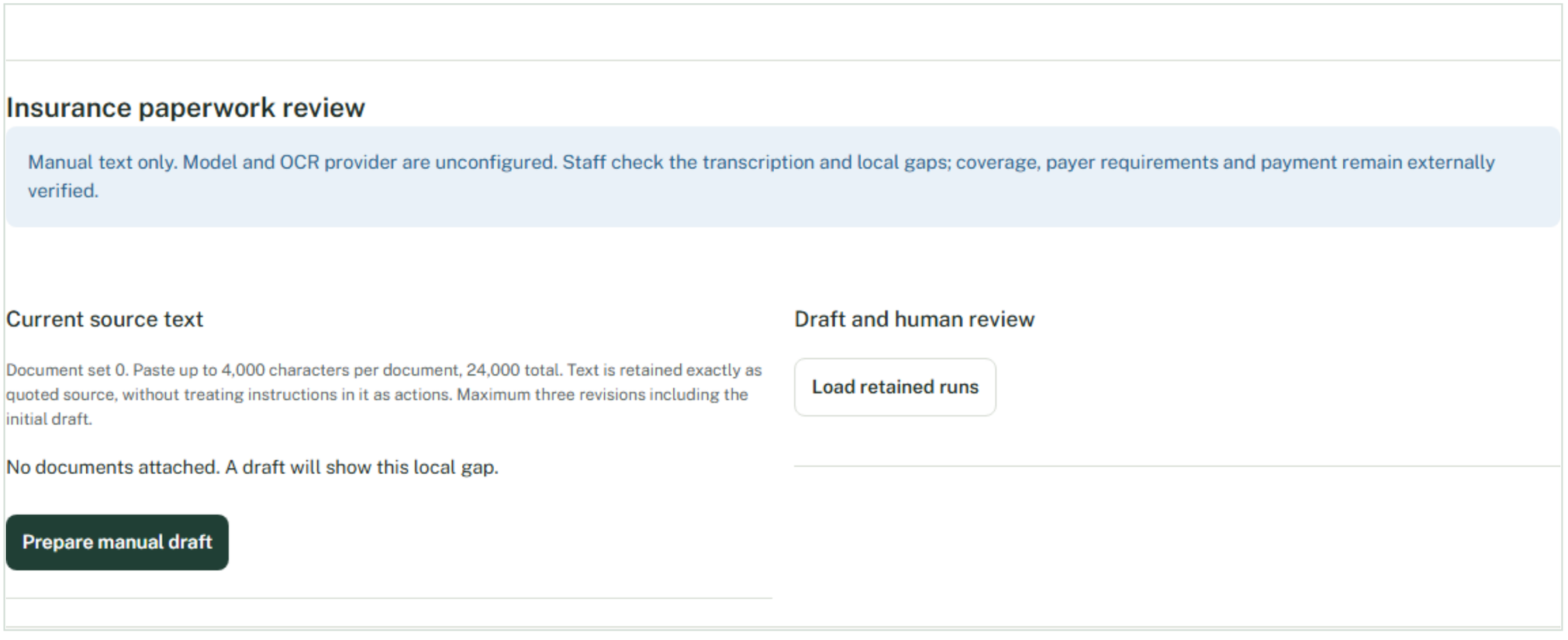
Current section / form reference

Insurance - Manual paperwork and human review

Selected case > Paperwork

1. Model and OCR providers are unconfigured. In Current source text, paste checked Manual text for each attached document, then click **Prepare manual draft**.
2. Review local missing records, source citations and exact quoted text. A local gap check does not prove completeness against payer requirements.
3. Choose Staff decision: Accept local draft, Request changes or Reject run. Enter Review comment and click Save staff decision. Changes/rejection require a reason; at most three revisions are allowed.
4. **Load retained runs** retrieves saved drafts. Open a run for Retained proposal and human review history.
5. A reviewed packet and local follow-up do not approve insurance/discharge or send anything. Staff use the existing payer email or portal separately.

Note: Paperwork instructions are source-verified only; no new draft/review was submitted. Retry same request / Leave pending request appear only after an uncertain request; recorded retries require the original staff actor. Preparation/material review closes after departure or self-pay conversion.



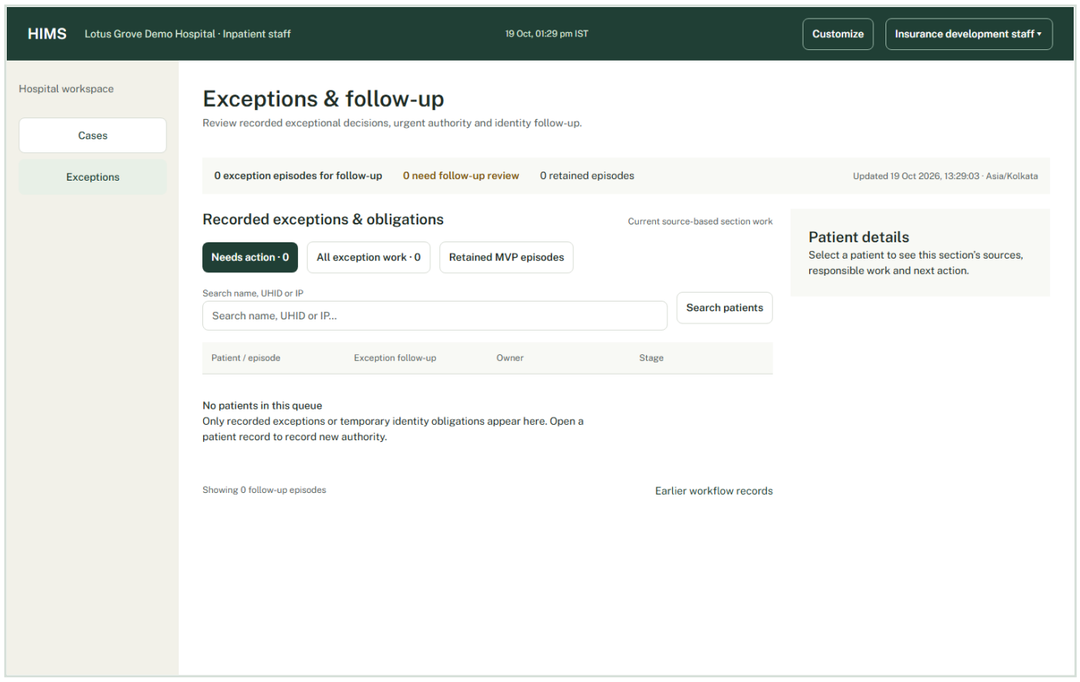
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Insurance - Exceptions and retained case history

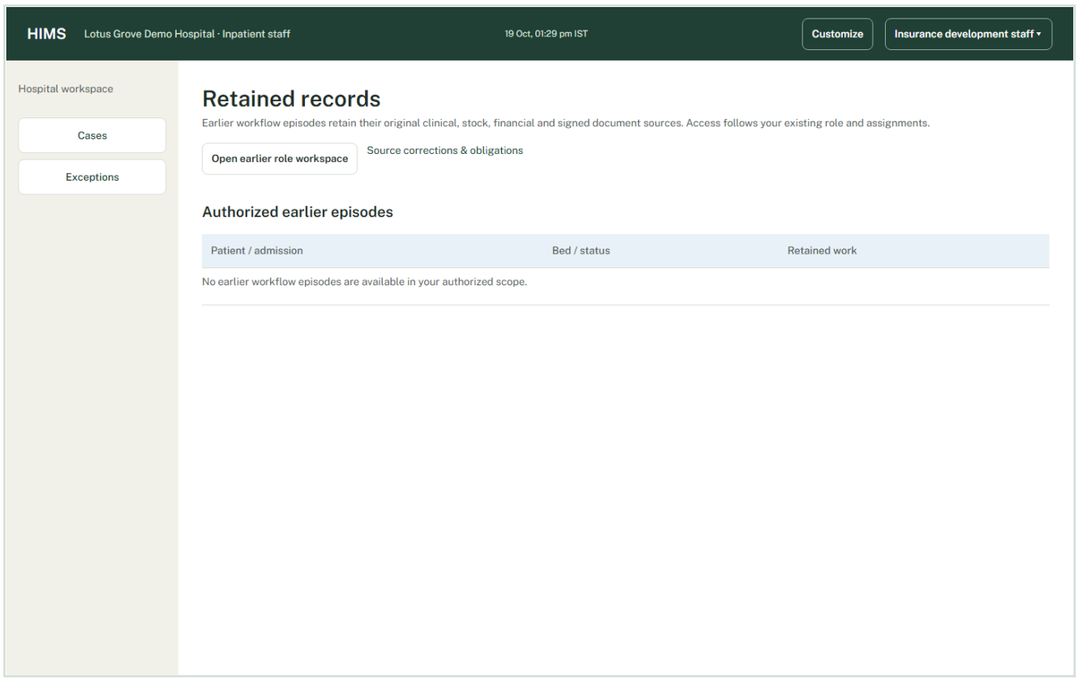
Sidebar > Exceptions · /exceptions; Retained records · /history

1. Exceptions offers Needs action, All exception work, Retained MVP episodes and Search patients. The verified snapshot has no recorded Insurance exception work.
2. When Admin has authorized an unapplied later Insurance correction, Review authorized insurance correction opens the closed patient's Authorized insurance corrections tab.
3. Apply selected externally verified insurance decision shows the bound authority/source. Enter external decision/allocation and provenance, then Record linked financial amendment; original exit remains retained.
4. Retained records opens earlier workflow history through Open earlier role workspace. Source corrections & obligations returns to Exceptions.
5. For current-MVP closed cases, use Cases > Retained MVP cases. An individual closed case also offers Open authorized source review; ordinary policy/material changes are closed.

Note: Navigation and empty queues were verified. The later-correction form is conditional source-verified behavior, not an exercised exceptional path. Admin must grant source-bound authority; Insurance cannot create that authority or supply another profession's approval.



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Current section / form reference | upper workspace; scroll for more

Choose an unfinished fictional patient

Use the existing role accounts for these assignments

1. Admission - Ananya Rao, IP-2026-00009 / G-107: pending reservation. Record the configured INR 3,000 deposit and actual arrival; Admin assigns a Nurse afterward.
2. Nurse - Kabir Sethi, IP-2026-00010 / G-108: ongoing care and an open shift. Medicine posted INR 400 after one accepted return; no external Doctor discharge instruction yet.
3. Insurance - Tara Mehta, IP-2026-00011 / P-205: assigned insured case awaiting external policy verification/decision. Pharmacy - Arjun Iyer, IP-2026-00012 / P-206: Pharmacy readiness still pending.
4. Billing - Pooja Nair, IP-2026-00013 / G-105: frozen INR 1,400 bill, INR 3,000 deposit, INR 1,400 allocated. Record the remaining INR 1,600 actual refund, then review final finance.
5. Rohan is completed. Review him through Retained MVP episodes; do not recreate his deposit, refund, exit or insurer receipt. His INR 1,100 refund is different from Pooja's pending INR 1,600.
6. The hospital has 16 beds, 13 admission episodes and 14 staff accounts. New named Nurse/Insurance staff are intentionally unassigned; the existing role accounts own the staged cases.

What to remember and what is still deferred

Current app, current local evidence

1. Same patient identity and essential handoff status appear across desks; section-specific queues and tasks differ. Select the patient row to inspect its summary, then the named action to enter that task. Buttons appear only when role/state permits.
2. Retained MVP episodes in a current queue shows completed current-workflow stays. Sidebar Retained records opens the earlier-workflow archive. Current closed-episode lists are limited to the latest 50.
3. Screenshots/forms explain current UI. Rohan's ordinary insured cycle was exercised in 28 workflow steps, followed by signed shift-end verification. Transfer, exceptional exit, new uploads and paperwork-review variants were inspected; this guide does not claim those were newly executed.
4. Admission reason / optional-Doctor changes and structured ward > room > bed storage remain deferred with database migrations. Current intake requires a configured Doctor and uses existing bed labels/categories. Vercel deployment remains on hold.
5. Read the current bill for active occupied stays: room units keep accruing. Renew the designated Doctor-availability period after 22 October 2026, 4:38 pm IST. Existing sign-in throttling permits five attempts per minute; wait a minute if prompted.
6. Sources: current web/src app/components/server; docs/agent/DASHBOARD_SECTION_REVIEW.md; docs/FICTIONAL_HOSPITAL_TESTING.md; local screenshots and Rohan journey evidence captured 19 October 2026. Source checkpoint 0b99b43 with reviewed local changes; preview task21-618fdbe.

Note: No cloud deployment, database migration, password change or real-patient data was used. This training guide is not a production-readiness certification.